

VACS AI Patient & Grading Issue Report

Patient A: Michael Thompson

Final Clinical and Operational Audit

Purpose: This report lists only confirmed system and data problems found by comparing the AI patient interview, the EMR screen shown to the student, the official Patient A answer key, and the AI grader. SOAP writing suggestions and personal review notes are excluded. Repeated problems are listed under each visit when they can affect grading independently.

Quick Guide for Non-Clinical Readers

Term	Plain language meaning
AI patient	The simulated patient the student interviews.
EMR	The electronic patient chart shown to the student.
Grader / rubric	The AI scoring system and the facts it expects.
Answer key	The official source document describing the intended patient state for that visit.
HFrEF	Heart failure with reduced ejection fraction.
HFimpEF	Heart failure with improved ejection fraction after treatment.
GDMT	Guideline directed medical therapy for heart failure.
LVEF	Left ventricular ejection fraction, a measure of heart pumping function.
BNP / NT proBNP	Related but different blood tests used to assess heart failure related cardiac strain.
HCTZ	Hydrochlorothiazide, a diuretic used for blood pressure or fluid management.

Final Review Summary

This Patient A review contains 26 grouped findings across all 3 Week 4 visits. Only confirmed conflicts or missing information problems are included. Repeated issues are kept under each visit when they can affect grading independently. The overall pattern is a longitudinal synchronization failure: Tuesday and Thursday graders largely follow the official answer key, while Wednesday largely follows an alternate live case.

Encounter	Grouped issues
Patient A – Michael Thompson – Tuesday Initial Visit	10
Patient A – Michael Thompson – Wednesday 3-Month Follow-Up	8
Patient A – Michael Thompson – Thursday 6-Month Follow-Up	8

Patient A – Michael Thompson – Tuesday Initial Visit

10 confirmed system/data issues are included for this visit. Items are grouped so one underlying problem is not counted multiple times just because it caused several grader deductions.

#	Issue	Problem type
1	Symptom duration, fatigue, and exercise tolerance returned by the AI patient do not match the grader case.	AI patient and grader disagree
2	Hydrochlorothiazide restroom access adherence barrier required by the grader is directly denied by the AI patient.	AI patient and grader disagree
3	Appendectomy required by the grader contradicts the AI patient surgical history.	AI patient and grader disagree
4	Family history and paternal myocardial infarction age differ from the grader case.	AI patient and grader disagree
5	Smoking exposure and alcohol use required by the grader are not returned by the AI patient.	AI patient and grader disagree
6	Two Tuesday EMR screens show incompatible vital signs and anthropometric data for Michael.	Internal EMR contradiction
7	The grader requires a third vital sign set that does not match the primary Tuesday vitals table.	EMR and grader disagree
8	Potassium and cardiac biomarker data are not synchronized across the live EMR and grader.	EMR and grader disagree
9	Exact laboratory, lipid, and echocardiogram data required by the grader are not available in the live EMR.	Required information is unavailable
10	Preventive care and referral details required by the grader are not fully exposed to the student.	Required information is unavailable

Issue 1. Symptom duration, fatigue, and exercise tolerance returned by the AI patient do not match the grader case

What the student saw or heard: When directly asked when symptoms began and how they changed, Michael says he cannot give an exact start date and describes the symptoms as something that gradually crept up on him. He reports that his energy has been about the same and that he can perform normal work and household activities, with windedness mainly when he climbs stairs quickly.

What the grader expected: The grader and official Tuesday answer key require approximately 6 months of gradually worsening symptoms, fatigue as a positive symptom, reduced exercise tolerance, decreased physical activity, and an initial belief that the symptoms were due to aging, weight gain, or reduced fitness.

Why this is a problem: The student asked the relevant symptom questions, but the AI patient returned a milder and less specific symptom history than the facts used for grading. Documenting the hidden 6-month course, fatigue, or reduced exercise tolerance would require replacing the fresh patient response with information the patient did not provide.

Suggested fix: Synchronize the Tuesday symptom history across the AI patient, answer key, and grader. If the 6-month course, fatigue, reduced exercise tolerance, and initial symptom attribution are graded, the AI patient should return those facts when the student asks the matching questions.

Evidence: Tuesday AI patient interview.

Michael Thompson

Standardized patient - Initial Ambulatory Care Visit

What brings you in today, and what has changed since your last visit or since this problem started?

Well, I was recently diagnosed with this heart condition - they said my heart isn't pumping as well as it should. I'm here to see about adjusting my medications for it. I've noticed my ankles get a bit puffy by the end of the day, and I get short of breath if I climb stairs too fast.

🌟 new info uncovered

How have your heart-failure symptoms changed, and when did they first start or begin getting worse?

Honestly, I couldn't tell you exactly when it started - it kind of crept up on me. The ankle swelling and getting winded on stairs is really all I've noticed, and it's mostly just at the end of the day or when I'm pushing myself a bit.

🌟 new info uncovered

How has your energy been? When do you feel fatigued, and what brings it on?

My energy's been about the same, I'd say - not really any worse. I just notice I get a little more tired or winded if I'm doing something active, like the stairs, but otherwise I feel pretty normal day to day.

🌟 new info uncovered

Have you had swelling in your ankles, legs, or abdomen? When is it worst?

Yeah, my ankles get a little puffy, mostly by the end of the day. I haven't noticed anything in my legs higher up or my stomach, just the ankles.

🌟 new info uncovered

Do you get short of breath when lying flat, need extra pillows, or wake up at night short of breath?

No, I don't think so - I sleep fine and haven't noticed needing extra pillows or waking up gasping or anything like that.

🌟 new info uncovered

Have you had chest pain, a racing or irregular heartbeat, dizziness, or fainting?

Michael describes the live symptom history and cannot provide the intended 6-month timeline.

Grader evidence: Tuesday grading feedback.

be quantified precisely (1-2 drinks/weekend, 20 pack-years) to match expected documentation detail. Ensure vital signs (RR, temperature, SpO2 with room air qualifier, BP) are transcribed precisely from source data. Include all available labs (Na, BUN, AST, ALT, glucose, lipid panel components) even if only to note they are unavailable, and explicitly identify missing-but-expected data (TSH, NT-proBNP vs BNP distinction, no prior LVEF for trending) as gaps in workup. Add BMI classification (obesity) and echocardiogram valvular findings. Strengthen the assessment by explicitly stating ACC/AHA Stage C classification, explicitly naming absence of ARNI, beta blocker, and MRA therapies rather than referring to them collectively as 'other foundational therapies,' and explicitly citing BNP value within the diagnostic reasoning. Contextualize lipid and glucose values against current statin therapy and diabetes risk when data becomes available. To strengthen the note: (1) explicitly cite the 2022 AHA/ACC/HFSA Heart Failure Guideline as the evidentiary basis for GDMT recommendations; (2) explicitly state all four goals of GDMT (mortality reduction, hospitalization reduction, symptom improvement, and slowed progression) within the assessment; (3) add a distinct preventive care assessment addressing pneumococcal, Shingrix, and Tdap vaccination gaps with a statement that preventive care optimization remains appropriate; (4) deepen the knowledge deficit assessment to explicitly note limited understanding of ejection fraction, heart failure pathophysiology, and long-term management, and clarify whether the patient exhibits anxiety versus general concern; (5) explicitly educate on the heart failure diagnosis and ejection fraction meaning, and directly state the importance of medication adherence as part of education; (6) specify that the beta blocker (not just 'HFrEF therapies' collectively) may contribute additional BP lowering. Add explicit vaccination review items: recommend reviewing vaccination history at future visits, recommend pneumococcal vaccination per current guidelines, and assess need for Shingrix. Correct the BMP recheck timeframe to align with the guideline-recommended 1-2 weeks after initiating spironolactone/ARNI, and extend the follow-up interval to 4-6 weeks with explicit mention that future visits will focus on progressive implementation of all four foundational HFrEF medication classes. Reorder the assessment to match model priority (HFrEF, HTN, Knowledge Deficit, Preventive Care) and include a distinct preventive care problem. Cite the guideline source (2022 AHA/ACC/HFSA) supporting GDMT recommendations. Explicitly include exercise tolerance as a monitored parameter to capture all nine expected monitoring domains.

- ☐ [1A.3] -0.1 pts: States 'ambulatory care pharmacist visit' but does not specify cardiology clinic qualifier.
- ☐ [1A.5] -0.3 pts: Referral source (primary care provider) is not mentioned anywhere in the note.
- ☐ [1A.6] -0.3 pts: Note does not state the referral reason (progressive dyspnea and reduced exercise tolerance) as triggers for referral.
- ☐ [1B.1] -0.5 pts: Symptom duration of approximately 6 months is not documented.
- ☐ [1B.2] -0.3 pts: Gradual onset character is not documented.
- ☐ [1B.3] -0.3 pts: Patient's initial attribution of symptoms to aging, weight gain, or reduced fitness is not documented.
- ☐ [1B.6] -0.3 pts: Note explicitly denies increased fatigue rather than documenting fatigue as present.
- ☐ [1B.7] -0.3 pts: Note states patient remains able to perform usual activities, contradicting reduced exercise tolerance.
- ☐ [1B.18] -0.1 pts: States general concern about long-term progression but does not specify fear that the heart will stop working.
- ☐ [1B.20] -0.3 pts: Note states patient denies difficulty with hydrochlorothiazide due to urination, contradicting the expected adherence barrier.
- ☐ [1C.4] -0.5 pts: Obesity is not listed under PMH despite elevated BMI.
- ☐ [1C.5] -0.5 pts: Note states patient denies prior surgeries, omitting the appendectomy from PSH.
- ☐ [1C.6] -0.3 pts: Father's hypertension history is not mentioned; only CAD, HF, and MI are listed.
- ☐ [1C.7] -0.4 pts: States father had MI but at incorrect age (64 instead of 62).
- ☐ [1C.8] -0.5 pts: Mother's history is listed as hypertension only; type 2 diabetes mellitus is omitted.

The grader requires the hidden symptom duration, fatigue, reduced exercise tolerance, and attribution details.

Issue 2. Hydrochlorothiazide restroom access adherence barrier required by the grader is directly denied by the AI patient

What the student saw or heard: Michael is specifically asked whether he ever skips or delays the water pill because of urination, work, travel, or restroom access. He states that he takes it every day as prescribed and that restroom access has not been a problem.

What the grader expected: The grader requires Michael to occasionally skip hydrochlorothiazide when he expects limited restroom access and deducts because the SOAP note documents the patient denial instead.

Why this is a problem: This is a direct contradiction after the exact adherence barrier was queried. The student should not create an adherence problem that the AI patient explicitly denies.

Suggested fix: Align hydrochlorothiazide adherence behavior across the AI patient, answer key, and grader. If restroom access is an intended barrier, the AI patient should disclose it when asked directly.

Evidence: Tuesday AI patient interview.

No, I haven't had anything like that - no chest pain, no racing heart or anything irregular that I've noticed, and no dizziness or fainting.

❖ new info uncovered

Do you weigh yourself at home? How often, and have your weights been stable or changed recently?

No, I don't check my blood pressure or weigh myself at home. Should I be doing that?

❖ new info uncovered

Do you check your blood pressure or pulse/heart rate at home? How often, and what have the readings been like?

No, I don't check my blood pressure or weigh myself at home. Should I be doing that?

❖ new info uncovered

Since the last visit, have you been to the emergency department or been hospitalized for your heart condition or any other reason?

No, I haven't been to the ER or hospitalized for anything since I saw you last.

❖ new info uncovered

Do you ever skip or delay a water pill because of urination, work, travel, or trouble getting to a restroom?

No, I take it every day like I'm supposed to - I haven't skipped it or anything. It hasn't really been an issue for me.

❖ new info uncovered

What have you been told about heart failure and your ejection fraction, and what questions do you still have?

They told me my heart's pumping percentage - I think they called it ejection fraction - is around 35%, which is lower than it should be, and that my heart is a little enlarged. Honestly, I'm still not totally clear on what that means for me long-term or why it happened, since I don't have blocked arteries or anything like that.

How long have you had each of your major medical conditions, and when were they diagnosed?

Michael states that he takes the water pill every day and restroom access has not been an issue.

Grader evidence: Tuesday grading feedback.

be quantified precisely (1-2 drinks/weekend, 20 pack-years) to match expected documentation detail. Ensure vital signs (RR, temperature, SpO2 with room air qualifier, BP) are transcribed precisely from source data. Include all available labs (Na, BUN, AST, ALT, glucose, lipid panel components) even if only to note they are unavailable, and explicitly identify missing-but-expected data (TSH, NT-proBNP vs BNP distinction, no prior LVEF for trending) as gaps in workup. Add BMI classification (obesity) and echocardiogram valvular findings. Strengthen the assessment by explicitly stating ACC/AHA Stage C classification, explicitly naming absence of ARNI, beta blocker, and MRA therapies rather than referring to them collectively as 'other foundational therapies,' and explicitly citing BNP value within the diagnostic reasoning. Contextualize lipid and glucose values against current statin therapy and diabetes risk when data becomes available. To strengthen the note: (1) explicitly cite the 2022 AHA/ACC/HFSA Heart Failure Guideline as the evidentiary basis for GDMT recommendations; (2) explicitly state all four goals of GDMT (mortality reduction, hospitalization reduction, symptom improvement, and slowed progression) within the assessment; (3) add a distinct preventive care assessment addressing pneumococcal, Shingrix, and Tdap vaccination gaps with a statement that preventive care optimization remains appropriate; (4) deepen the knowledge deficit assessment to explicitly note limited understanding of ejection fraction, heart failure pathophysiology, and long-term management, and clarify whether the patient exhibits anxiety versus general concern; (5) explicitly educate on the heart failure diagnosis and ejection fraction meaning, and directly state the importance of medication adherence as part of education; (6) specify that the beta blocker (not just 'HFrEF therapies' collectively) may contribute additional BP lowering. Add explicit vaccination review items: recommend reviewing vaccination history at future visits, recommend pneumococcal vaccination per current guidelines, and assess need for Shingrix. Correct the BMP recheck timeframe to align with the guideline-recommended 1-2 weeks after initiating spironolactone/ARNI, and extend the follow-up interval to 4-6 weeks with explicit mention that future visits will focus on progressive implementation of all four foundational HFrEF medication classes. Reorder the assessment to match model priority (HFrEF, HTN, Knowledge Deficit, Preventive Care) and include a distinct preventive care problem. Cite the guideline source (2022 AHA/ACC/HFSA) supporting GDMT recommendations. Explicitly include exercise tolerance as a monitored parameter to capture all nine expected monitoring domains.

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The grader requires the opposite hydrochlorothiazide adherence history.

Issue 3. Appendectomy required by the grader contradicts the AI patient surgical history

What the student saw or heard: When asked whether he has had any surgeries or procedures, Michael states that he cannot think of any prior surgery or procedure.

What the grader expected: The official Master Profile and grader require an appendectomy at age 24 and deduct when prior surgery is denied.

Why this is a problem: A surgical history is a patient specific fact. The student should not document an appendectomy after the simulated patient denies prior surgery.

Suggested fix: Persist the intended appendectomy history in the AI patient memory and visible chart. If appendectomy is graded, the patient should disclose it when asked about past surgery or procedures.

Evidence: Tuesday AI patient interview.

The heart failure thing is pretty recent - just found out about it after some testing. As for my blood pressure, I've been on medication for it for a while, but I honestly don't remember exactly how many years it's been.

new info uncovered

What lifestyle changes have you made since the last visit, such as cooking at home, eating out less, exercise, or other habits?

Honestly, I haven't really changed much since finding out about this. I still eat pretty normally and haven't started any specific exercise routine or anything like that.

new info uncovered

What medical conditions have you been diagnosed with?

I've got high blood pressure, high cholesterol, and now this heart failure thing they just found - the low pumping percentage and enlarged heart.

new info uncovered

Have you ever had any surgeries or procedures? If so, what were they and about when?

No, I haven't had any surgeries or procedures that I can think of.

Tell me about your family medical history. Has anyone had the same conditions, heart disease, stroke, kidney disease, diabetes, or other major illnesses?

My father had coronary artery disease and heart failure, and he actually had a heart attack at age 64. My mother has high blood pressure too.

new info uncovered

Please tell me every medication you are taking right now, including the dose, how you take it, and how often.

Sure - I take Lisinopril 20 mg once a day, hydrochlorothiazide 25 mg once a day, and atorvastatin 40 mg once a day. That's it for my prescriptions.

new info uncovered

How consistently are you taking your medications? About how many doses do you miss, and what usually causes you to miss them?

I'm pretty consistent - I don't think I really miss doses. I just take them as part of my daily routine.

Michael denies prior surgeries or procedures.

Answer key evidence: Patient A Master Profile.

Patient A Master Profile

Patient Name

Michael Thompson

Age

58 years old

Sex

Male

Visit Setting

Ambulatory Care Cardiology Clinic

Referral from Primary Care Provider

Main Medical Conditions

- Newly diagnosed Heart Failure with Reduced Ejection Fraction (HFrEF)
- Hypertension
- Hyperlipidemia
- Obesity

Educational Purpose

Patient A serves as the foundational patient for Week 4.

Students will focus on:

- Identifying and classifying heart failure
- Applying heart failure guidelines
- Initiating guideline-directed medical therapy (GDMT)

The official Master Profile includes appendectomy at age 24.

Issue 4. Family history and paternal myocardial infarction age differ from the grader case

What the student saw or heard: Michael reports that his father had coronary artery disease, heart failure, and a heart attack at age 64. He reports that his mother has hypertension.

What the grader expected: The grader and Master Profile require paternal hypertension with myocardial infarction at age 62 and maternal type 2 diabetes mellitus.

Why this is a problem: The student asked for family history and documented the response provided. Substituting a different myocardial infarction age and different parental diagnoses would make the SOAP note inconsistent with the interview.

Suggested fix: Use one persistent family history across the Master Profile, AI patient, EMR, and grader. The same family history should be returned on every encounter when the student asks for it.

Evidence: Tuesday AI patient interview.

The heart failure thing is pretty recent - just found out about it after some testing. As for my blood pressure, I've been on medication for it for a while, but I honestly don't remember exactly how many years it's been.

❖ new info uncovered

What lifestyle changes have you made since the last visit, such as cooking at home, eating out less, exercise, or other habits?

Honestly, I haven't really changed much since finding out about this. I still eat pretty normally and haven't started any specific exercise routine or anything like that.

❖ new info uncovered

What medical conditions have you been diagnosed with?

I've got high blood pressure, high cholesterol, and now this heart failure thing they just found - the low pumping percentage and enlarged heart.

❖ new info uncovered

Have you ever had any surgeries or procedures? If so, what were they and about when?

No, I haven't had any surgeries or procedures that I can think of.

Tell me about your family medical history. Has anyone had the same conditions, heart disease, stroke, kidney disease, diabetes, or other major illnesses?

My father had coronary artery disease and heart failure, and he actually had a heart attack at age 64. My mother has high blood pressure too.

❖ new info uncovered

Please tell me every medication you are taking right now, including the dose, how you take it, and how often.

Sure - I take Lisinopril 20 mg once a day, hydrochlorothiazide 25 mg once a day, and atorvastatin 40 mg once a day. That's it for my prescriptions.

❖ new info uncovered

How consistently are you taking your medications? About how many doses do you miss, and what usually causes you to miss them?

I'm pretty consistent - I don't think I really miss doses. I just take them as part of my daily routine.

Michael gives paternal MI at age 64 and maternal hypertension.

Answer key evidence: Tuesday official answer key.

11. Family History

Father

Hypertension

Myocardial infarction at age 62

Mother

Type 2 diabetes mellitus

12. Problem List

Active Problems

Newly diagnosed HFrEF (LVEF 35%)

Hypertension

Hyperlipidemia

Obesity

Dyspnea on exertion

Suboptimal cardiovascular risk factor control

No guideline-directed medical therapy for HFrEF

Limited heart failure self-management knowledge

Need for heart failure education

Need for home monitoring plan

The intended family history is paternal hypertension and MI at age 62 with maternal type 2 diabetes.

Issue 5. Smoking exposure and alcohol use required by the grader are not returned by the AI patient

What the student saw or heard: Michael reports that he is a former smoker who quit about 5 years ago but cannot remember the amount he previously smoked. He describes alcohol use as rare, usually a glass of wine or beer on special occasions.

What the grader expected: The grader and answer key require a 20-pack year smoking history and 1 to 2 alcoholic beverages on weekends.

Why this is a problem: The student can document former smoking and occasional alcohol use, but the exact pack year exposure and weekend drinking pattern are not available from the live interview. Adding them would require inventing patient history.

Suggested fix: Synchronize tobacco exposure and alcohol quantity/frequency across the AI patient and grader. If exact pack years and weekend drinking are graded, the patient should return those details when directly asked.

Evidence: Tuesday AI patient interview.

Have cost, insurance, refills, transportation, or access made it harder to take your medications or keep appointments?

No, none of that has been a problem for me. I haven't had any trouble getting my medications or making it to appointments.

Do you have any medication or food allergies? What reaction do you have?

No, I don't have any known drug or food allergies.

Do you know whether you are up to date on the vaccines recommended for you? Which vaccines do you remember receiving recently?

Hmm, I'm honestly not sure about that. I don't really keep track of which vaccines I've had recently.

Who do you live with, and what do you do for work or school?

I'm married and live with my wife. I work full-time as an accountant.

Do you currently use tobacco or nicotine, or have you in the past? If yes, how much, for how long, and when did you quit if you stopped?

I'm a former smoker. I quit about 5 years ago.

How much alcohol do you drink, and do you use any recreational or illicit drugs?

I rarely drink alcohol—maybe a glass of wine or beer on special occasions. And no, I don't use any recreational or illicit drugs.

What does your usual physical activity look like, and has your condition changed what you are able to do?

Michael cannot quantify prior smoking exposure and describes rare special occasion alcohol use.

Answer key evidence: Tuesday official answer key.

Pneumococcal

Not documented

Shingrix

Not documented

Tdap

Last documented 9 years ago

10. Social History

Tobacco

Former smoker

20 pack-year history

Quit 5 years ago

Alcohol

1–2 alcoholic beverages on weekends

Exercise

Minimal structured exercise

Reports decreasing activity due to fatigue and stiffness

Diet

Frequently eats restaurant meals

Consumes several processed foods weekly

Limited attention to sodium intake

The answer key requires 20 pack years and 1 to 2 alcoholic beverages on weekends.

Issue 6. Two Tuesday EMR screens show incompatible vital signs and anthropometric data for Michael

What the student saw or heard: One Tuesday Vitals table shows BP 138/86 mmHg, repeat BP 136/84 mmHg, HR 82 bpm, weight 102 kg, height 5 feet 10 inches, and BMI 32.2 kg/m². A separate Tuesday Key vitals screen supplied from the same Patient A encounter shows BP 128/78 mmHg, repeat BP 126/76 mmHg, HR 88 bpm, weight 84 kg, height 5 feet 5 inches, and BMI 30.8 kg/m².

What the grader expected: The grader expects yet another Tuesday objective state based on the official answer key. This issue is present even before comparing the student data with the grader because the student facing Tuesday evidence itself is internally inconsistent.

Why this is a problem: Two different heights, weights, heart rates, and blood pressures cannot represent the same patient at the same encounter timestamp. The difference is too large to be explained by rounding or repeat measurement.

Suggested fix: Identify which Tuesday objective dataset is intended and remove the other one from the Patient A encounter. All Tuesday widgets should read from the same vital sign and anthropometric source.

Evidence: Tuesday detailed Vitals table.

Vitals				
Vital signs recorded at this visit				
VITAL	VALUE	REFERENCE	TREND	FLAG
Office BP 06/23/2026 09:14	138/86	<130/80 mmHg		
Repeat BP 06/23/2026 09:14	136/84	<130/80 mmHg	—	
Heart rate 06/23/2026 09:14	82	60–100 bpm	—	
Respiratory rate 06/23/2026 09:14	16	12–20 /min	—	
Temperature 06/23/2026 09:14	98.6°F	97.0–99.5 °F	—	
SpO ₂ 06/23/2026 09:14	96%	95–100 %	—	
Weight 06/23/2026 09:14	102 kg	—		
Height 06/23/2026 09:14	5'10"	—	—	
BMI 06/23/2026 09:14	32.2	18.5–24.9 kg/m ²	—	
BSA (Mosteller) 06/23/2026 09:14	2.04 m ²	—	—	

One Tuesday screen shows BP 138/86, HR 82, weight 102 kg, height 5 feet 10 inches, and BMI 32.2.

Evidence: Tuesday Key vitals panel.

 Key vitals		
Office BP		128/78
06/23/2026 09:14		
Repeat BP		126/76
06/23/2026 09:14		
HR		88 bpm
06/23/2026 09:14		
Temp		98.2°F
06/23/2026 09:14		
RR		16
06/23/2026 09:14		
SpO ₂		98%
06/23/2026 09:14		
Weight		84 kg
06/23/2026 09:14		
Height		5'5"
06/23/2026 09:14		
Pain		0/10
06/23/2026 09:14		
BMI		30.8
06/23/2026 09:14		

A second Tuesday screen shows BP 128/78, HR 88, weight 84 kg, height 5 feet 5 inches, and BMI 30.8.

Issue 7. The grader requires a third vital sign set that does not match the primary Tuesday vitals table

What the student saw or heard: The primary Tuesday Vitals table shows office BP 138/86 mmHg, repeat BP 136/84 mmHg, respiratory rate 16/min, temperature 98.6 F, and SpO2 96%.

What the grader expected: The grader and official answer key expect office BP 138/84 mmHg, respiratory rate 18/min, temperature 98.4 F, and SpO2 98% on room air.

Why this is a problem: The student is penalized for transcribing objective values exactly as displayed in the main Tuesday Vitals table. Combined with the second conflicting Key vitals panel, the grader effectively introduces a third objective state.

Suggested fix: Choose one Tuesday vital sign dataset and update every EMR widget and the grader together. The grader should compare the SOAP note against the exact values displayed to the student.

Evidence: Tuesday primary Vitals table.

Vitals				
Vital signs recorded at this visit				
VITAL	VALUE	REFERENCE	TREND	FLAG
Office BP 06/23/2026 09:14	138/86	<130/80 mmHg		
Repeat BP 06/23/2026 09:14	136/84	<130/80 mmHg	—	
Heart rate 06/23/2026 09:14	82	60–100 bpm	—	
Respiratory rate 06/23/2026 09:14	16	12–20 /min	—	
Temperature 06/23/2026 09:14	98.6°F	97.0–99.5 °F	—	
SpO ₂ 06/23/2026 09:14	96%	95–100 %	—	
Weight 06/23/2026 09:14	102 kg	—		
Height 06/23/2026 09:14	5'10"	—	—	
BMI 06/23/2026 09:14	32.2	18.5–24.9 kg/m ³	—	
BSA (Mosteller) 06/23/2026 09:14	2.04 m ²	—	—	

The primary Tuesday Vitals screen displays BP 138/86, RR 16, temperature 98.6 F, and SpO2 96%.

Answer key evidence: Tuesday intended vital signs.

4. Past Medical History

Heart Failure with Reduced Ejection Fraction (New Diagnosis)

LVEF 35%

Hypertension

Diagnosed 10 years ago

Hyperlipidemia

Diagnosed 8 years ago

Obesity

5. Medication List

Lisinopril 20 mg daily

Hydrochlorothiazide 25 mg daily

Atorvastatin 40 mg daily

6. Allergies

Drug Allergies

NKDA

Food Allergies

None

7. Vitals

Today's Vitals

BP: 138/84 mmHg

The official answer key begins the intended Tuesday vital sign set.

Grader evidence: Tuesday grading feedback.

- ☐ [1D.5] -0.5 pts: Pack-year smoking history (20 pack-years) is not quantified.
- ☐ [1D.7] -0.8 pts: States 'rare alcohol use on special occasions' rather than the expected 1-2 drinks on weekends.
- ☐ [1D.9] -0.5 pts: Note states no major lifestyle changes and ability to perform usual activities, omitting reduced physical activity secondary to symptoms.
- ☐ [1E.9] -0.3 pts: COVID vaccination status is not documented anywhere in the note.
- ☐ [1E.10] -0.5 pts: Pneumococcal vaccination status is not documented.
- ☐ [1E.11] -0.5 pts: Shingrix vaccination status is not documented.
- ☐ [1E.12] -0.8 pts: Tdap vaccination status and timing are not documented.
- ☐ [1F.2] -0.3 pts: Fatigue is documented as denied rather than as a positive ROS finding.
- ☐ [1G.1] -0.5 pts: Note states patient is 'concerned about long term progression' but does not specify the fear that his heart will stop working.
- ☐ [1G.2] -0.3 pts: States 'limited prior heart failure education' but does not explicitly state patient never received formal HF education.
- ☐ [1G.4] -0.5 pts: Note states patient denies difficulty with HCTZ due to urination, rather than identifying it as an adherence barrier.
- ☐ [2A.1] -0.3 pts: BP documented as 138/86 mmHg, a single-digit transcription discrepancy from expected 138/84 mmHg.
- ☐ [2A.3] -0.3 pts: RR documented as 16/min rather than the expected 18 breaths/min.
- ☐ [2A.4] -0.3 pts: Temperature documented as 98.6°F rather than the expected 98.4°F.
- ☐ [2A.5] -0.5 pts: SpO2 documented as 96% without room air qualifier and differing from expected 98%.
- ☐ [2A.9] -0.8 pts: BMI value stated but not classified as obesity/class I obesity.
- ☐ [2B.1] -0.3 pts: Sodium value not documented anywhere in the note.
- ☐ [2B.2] -0.1 pts: Potassium documented as 4.2 mEq/L, a minor transcription discrepancy from expected 4.3 mEq/L.
- ☐ [2B.3] -0.3 pts: BUN value not documented.
- ☐ [2B.6] -0.3 pts: AST value not documented.
- ☐ [2B.7] -0.3 pts: ALT value not documented.
- ☐ [2B.8] -0.3 pts: Glucose value not documented.
- ☐ [2B.11] -0.3 pts: Total cholesterol value not documented.
- ☐ [2B.12] -0.5 pts: LDL-C value not documented.
- ☐ [2B.13] -0.3 pts: HDL-C value not documented.
- ☐ [2B.14] -0.3 pts: Triglyceride value not documented.
- ☐ [2B.17] -0.3 pts: Echo finding regarding valvular status not documented.
- ☐ [2B.20] -0.3 pts: LFT values and normal-limits interpretation not documented.
- ☐ [2C.1] -0.5 pts: Missing TSH/thyroid studies not noted as absent.
- ☐ [2C.2] -0.5 pts: No distinction made regarding BNP vs NT-proBNP testing gap.
- ☐ [2C.3] -0.5 pts: Does not note this is the first echocardiogram or lack of prior LVEF for trending.
- ☐ [2C.5] -0.5 pts: LDL-C value not available so no contextualization vs statin therapy provided.
- ☐ [2C.6] -0.5 pts: Glucose value not documented, so no borderline/elevated interpretation provided.
- ☐ [2C.7] -0.5 pts: LFT values not documented, so no clinical relevance to statin/GDMT safety noted.
- ☐ [3A.4] -0.3 pts: Fatigue not cited as supporting evidence (patient denies fatigue).
- ☐ [3A.5] -0.3 pts: Reduced exercise tolerance not explicitly cited as supporting evidence.
- ☐ [3A.6] -0.3 pts: Edema cited as evidence but without the 'intermittent' qualifier.
- ☐ [3A.7] -0.5 pts: Elevated BNP value not cited within the assessment as supporting diagnostic evidence.
- ☐ [3A.9] -1.0 pts: ACC/AHA Stage C classification not stated anywhere in the assessment.

The grader expects the answer key values rather than the primary live Vitals table.

Issue 8. Potassium and cardiac biomarker data are not synchronized across the live EMR and grader

What the student saw or heard: The live laboratory panel reports potassium 4.2 mEq/L and NT proBNP 320 pg/mL.

What the grader expected: The grader and official answer key expect potassium 4.3 mEq/L and BNP 320 pg/mL.

Why this is a problem: Potassium can affect medication safety decisions, and BNP and NT proBNP are related but distinct laboratory tests. The student should not be penalized for using the displayed potassium value or exact biomarker name.

Suggested fix: Synchronize potassium and the natriuretic peptide assay across the answer key, live laboratory panel, educational prompts, trends, and grader. Use one exact test name and one exact value.

Evidence: Tuesday live laboratory panel.

All Labs		Collected: 06/09/2026 07:50	
TEST	RESULT	REFERENCE RANGE	FLAG
NT-proBNP	320 pg/mL	—	• High
K	4.2 mEq/L	—	
SCr	1.0 mg/dL	—	
eGFR	88 mL/min/1.73m ²	—	

The live EMR displays potassium 4.2 mEq/L and NT proBNP 320 pg/mL.

Answer key evidence: Tuesday laboratory data.

AST: 24 U/L

ALT: 26 U/L

BNP

BNP: 320 pg/mL

Elevated

Lipid Panel

TC: 192 mg/dL

LDL-C: 101 mg/dL

HDL-C: 46 mg/dL

TG: 178 mg/dL

Echocardiogram

LVEF: 35%

Mild left ventricular dilation

No significant valvular abnormalities

9. Immunizations

Influenza

Received this season

COVID-19

Completed primary series

The official answer key uses BNP 320 pg/mL and a different potassium value from the live panel.

Issue 9. Exact laboratory, lipid, and echocardiogram data required by the grader are not available in the live EMR

What the student saw or heard: The live CMP panel exposes only potassium, serum creatinine, and eGFR. The Lipid Panel screen says that no results are available for the visit, and the Missing / Needed Labs screen also shows no results. The visible echocardiogram information shows LVEF 35% and mild LV dilation but does not display the complete intended valvular statement.

What the grader expected: The grader and answer key require sodium, BUN, glucose, AST, ALT, full lipid values, and no significant valvular abnormalities, among other objective facts.

Why this is a problem: These are exact objective values and findings. A student cannot safely infer or invent them when the student facing panels do not display them.

Suggested fix: Expose every objective fact that is used for grading or remove hidden values from the rubric. If a test is intentionally unavailable, the grader should accept documentation that the result is unavailable.

Evidence: Tuesday live objective panels.

The screenshot displays four separate EMR panels arranged in a 2x2 grid. The top-left panel, titled 'Live CMP panel', shows a 'Comprehensive Metabolic Panel (CMP)' with a collection time of 06/09/2026 07:50. It contains a table with three rows of data: Potassium (K) at 4.2 mEq/L, Serum Creatinine (SCr) at 1.0 mg/dL, and Estimated Glomerular Filtration Rate (eGFR) at 88 mL/min/1.73m². The top-right panel, titled 'Live lipid panel', shows a 'Labs' section with a dropdown menu set to 'Lipid Panel' and a message stating 'No results in this panel for this visit.' The bottom-left panel, titled 'Live Missing / Needed Labs panel', also shows a 'Labs' section with a dropdown menu set to 'Missing / Needed Labs' and a message stating 'No results in this panel for this visit.' The bottom-right panel, titled 'Live immunization panel', shows an 'Immunizations' section with a status for 'Influenza' marked as 'Up to date'.

TEST	RESULT	REFERENCE RANGE	FLAG
K	4.2 mEq/L	—	
SCr	1.0 mg/dL	—	
eGFR	88 mL/min/1.73m²	—	

The live CMP is incomplete and the lipid and Missing / Needed Labs panels show no results.

Answer key evidence: Tuesday intended laboratory and echocardiogram data.

AST: 24 U/L

ALT: 26 U/L

BNP

BNP: 320 pg/mL

Elevated

Lipid Panel

TC: 192 mg/dL

LDL-C: 101 mg/dL

HDL-C: 46 mg/dL

TG: 178 mg/dL

Echocardiogram

LVEF: 35%

Mild left ventricular dilation

No significant valvular abnormalities

9. Immunizations

Influenza

Received this season

COVID-19

Completed primary series

The official answer key contains BNP, full lipid values, and the intended echocardiogram interpretation.

Issue 10. Preventive care and referral details required by the grader are not fully exposed to the student

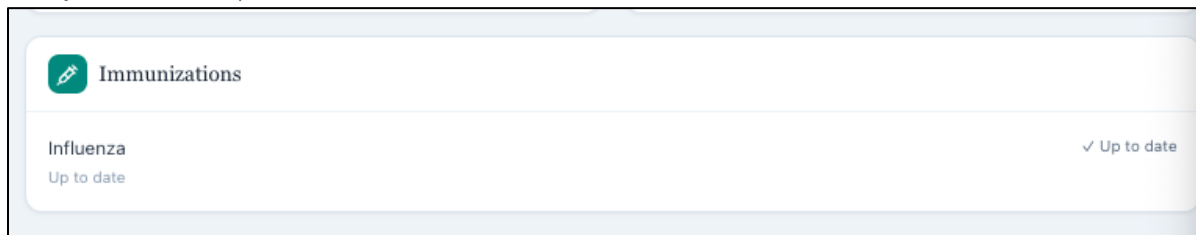
What the student saw or heard: The live immunization panel displays influenza as up to date but does not display the full COVID, pneumococcal, Shingrix, and Tdap history used by the grader. The visible encounter identifies an ambulatory care clinic and says the patient was referred for GDMT optimization after a new nonischemic cardiomyopathy diagnosis, but it does not clearly identify the exact primary care referral pathway and cardiology clinic qualifier required by the grader.

What the grader expected: The grader expects COVID primary series status, pneumococcal status, Shingrix status, a Tdap interval, PCP referral, progressive dyspnea and reduced exercise tolerance as referral triggers, and an ambulatory care cardiology clinic setting.

Why this is a problem: These are chart facts rather than clinical interpretations. The student should not have to infer a referral source, clinic type, or vaccine history that is not visible.

Suggested fix: Display the complete graded immunization record and the exact referral source, referral reason, and clinic setting in the student facing encounter. Otherwise remove those exact facts from grading.

Evidence: Tuesday live immunization panel.



Only influenza is visible in the live Tuesday immunization section.

Answer key evidence: Tuesday intended referral and preventive care information.

SECTION 9: PATIENT A – TUESDAY INITIAL VISIT

Encounter Overview

Encounter Type

Initial Ambulatory Care Pharmacist Visit

Visit Length

30-minute collaborative practice visit

Week

Week 4 – Tuesday

Difficulty Level

Foundational APPE Student

Primary Learning Objectives

Students should be able to:

- Classify heart failure appropriately
- Assess symptom burden and functional status
- Identify HFrEF guideline-directed medical therapy (GDMT)
- Evaluate current medication appropriateness
- Identify missing mortality-reducing therapies
- Develop an evidence-based treatment plan
- Construct a complete SOAP note
- Create an appropriate monitoring and follow-up strategy
- Provide patient-centered heart failure education

1. Patient Snapshot

Michael Thompson is a 58-year-old male presenting to an ambulatory care cardiology clinic following referral from his primary care provider for newly diagnosed heart failure with reduced ejection fraction (HFrEF).

The official answer key defines the referral and clinic context.

Answer key evidence: Tuesday immunization and social history.

Pneumococcal
Not documented
Shingrix
Not documented
Tdap
Last documented 9 years ago

10. Social History

Tobacco
Former smoker
20 pack-year history
Quit 5 years ago

Alcohol
1–2 alcoholic beverages on weekends

Exercise
Minimal structured exercise
Reports decreasing activity due to fatigue and shortness of breath

Diet
Frequently eats restaurant meals
Consumes several processed foods weekly
Limited attention to sodium intake

The official answer key contains the additional preventive care information.

Patient A – Michael Thompson – Wednesday 3-Month Follow-Up

8 confirmed system/data issues are included for this visit. Items are grouped so one underlying problem is not counted multiple times just because it caused several grader deductions.

#	Issue	Problem type
1	The live medication and GDMT state does not match the official Wednesday case.	Answer key and live case disagree
2	Symptom response and functional status returned by the AI patient do not match the official follow up history.	AI patient and answer key disagree
3	Home blood pressure and weight monitoring expected by the answer key are not present in the live AI patient.	AI patient and answer key disagree
4	Adherence and lifestyle progress in the answer key are not returned by the live AI patient.	AI patient and answer key disagree
5	Visible Wednesday vital signs and weight differ from the official answer key while the grader accepts the live values.	Answer key and grader disagree
6	The grader requires a 2 kg weight decrease that is supported by neither the live baseline nor the answer key.	Grader trend calculation mismatch
7	BNP and NT proBNP naming is inconsistent across the Wednesday sources.	Terminology / data mismatch
8	Three month follow up vitals and laboratory results reuse the baseline timestamps.	Longitudinal data integrity issue

Issue 1. The live medication and GDMT state does not match the official Wednesday case

What the student saw or heard: The live Wednesday EMR and AI patient show sacubitril/valsartan 24/26 mg twice daily, carvedilol 3.125 mg twice daily, spironolactone 25 mg daily, dapagliflozin 10 mg daily, hydrochlorothiazide 25 mg daily, and atorvastatin 40 mg daily. The live clinical problem is symptomatic orthostasis while the patient is already receiving all four foundational HFrEF classes.

What the grader expected: The official Wednesday answer key expects sacubitril/valsartan 49/51 mg twice daily, metoprolol succinate 50 mg daily, empagliflozin 10 mg daily, atorvastatin 40 mg daily, and specifically states that the patient is not yet receiving an MRA. The intended learning task is to evaluate spironolactone initiation. The Wednesday grader, however, praises the alternate live regimen and hydrochlorothiazide discontinuation.

Why this is a problem: The official case and the live/grader case represent different stages of treatment and lead to different clinical decisions. A student cannot simultaneously initiate an MRA and discontinue nonessential blood pressure therapy because the MRA is already present in the live case.

Suggested fix: Choose one Wednesday longitudinal state and update the answer key, live EMR, AI patient, and grader together. The medication list and learning objective should be identical across components.

Evidence: Wednesday live medication list.

Medications

Documented medication list (as recorded in the chart)



Medications

Current / Home meds (6)

Discontinued (0)

✓ Allergy/interaction check active · NKDA — no conflicts

PRESCRIBED / ACTIVE MEDICATIONS

MEDICATION	STATUS	DOSE	ROUTE	FREQ	INDICATION	START	PRESCRIBER	LAST FILLED	REFILLS	PHA
Sacubitril/Valsartan (Entresto) ✓ Sig:	Active	24/26 mg	by mouth	BID	HFrEF			Qty		
Carvedilol ✓ Sig:	Active	3.125 mg	by mouth	BID	HFrEF			Qty		
Spironolactone ✓ Sig:	Active	25 mg	by mouth	daily	HFrEF			Qty		
Dapagliflozin ✓ Sig:	Active	10 mg	by mouth	daily	HFrEF			Qty		
Hydrochlorothiazide ✓ Sig:	Active	25 mg	by mouth	daily	HTN			Qty		
Atorvastatin ✓ Sig:	Active	40 mg	by mouth	daily	HLD			Qty		



Medication reconciliation prompts

- Ask about OTC medications (e.g., pain relievers, supplements)
- Ask about any side effects or new symptoms
- Ask about missed doses and the reasons behind them
- Ask about cost or refill barriers

Missing → The visible list may be incomplete — patients often omit OTC products. Confirm in the interview.

The live Wednesday EMR shows all four foundational HFrEF classes plus hydrochlorothiazide.

Answer key evidence: Wednesday intended medication list.

4. Past Medical History

Heart Failure with Reduced Ejection Fraction

Hypertension

Hyperlipidemia

Obesity

5. Medication List

Sacubitril/Valsartan 49/51 mg PO BID

Metoprolol Succinate 50 mg PO daily

Empagliflozin 10 mg PO daily

Atorvastatin 40 mg PO daily

6. Allergies

Drug Allergies

NKDA

Food Allergies

None

7. Vitals

Today's Vitals

BP: 118/72 mmHg

HR: 68 bpm

RR: 16

Temp: 98.2°F

The official Wednesday answer key uses a different regimen and no MRA.

Grader evidence: Wednesday AI feedback.

AI Grading Feedback

Week 4 · Michael Thompson · Wednesday · Generated 9/8/2026, 12:03:42 PM

87.5%

✓ STRENGTHS

The student produced a thorough, well-organized SOAP note that captured nearly all subjective details regarding the dizziness (onset, triggers, duration, absence of falls/syncope), accurately documented the full GDMT regimen and vital signs, and correctly identified orthostatic hypotension as the primary medical therapy problem. The assessment appropriately linked HCTZ discontinuation as the key intervention while explicitly preserving all four GDMT pillars, and the plan included appropriate monitoring for renal function/electrolytes, home BP/weight monitoring with clear escalation thresholds, and patient counseling on positional changes. Family history, allergies, social history, and immunization status were all accurately captured.

○ OPPORTUNITIES TO IMPROVE

To strengthen the assessment, explicitly compare current NT-proBNP (165) and weight (101 kg) to baseline values (320 pg/mL and +2 kg) to demonstrate objective evidence of treatment response, and use this trend to support the assessment that HFrEF is improving on GDMT. When discussing the etiology of orthostatic hypotension, acknowledge that carvedilol and sacubitril/valsartan also contribute to the patient's BP lowering in addition to HCTZ, even though the plan correctly prioritizes HCTZ discontinuation. Finally, align the follow-up interval with the expected ~1 month timeframe for reassessment of GDMT titration, though the 1-2 week recheck for BMP/orthostasis is reasonable and could be described as an interim visit prior to the 1-month follow-up.

▮ GOLD STANDARD COMPARISON

—

This is AI-generated formative feedback — a study aid. Use your preceptor's judgment as the final word.

The grader accepts the live orthostasis and hydrochlorothiazide discontinuation scenario.

Issue 2. Symptom response and functional status returned by the AI patient do not match the official follow up history

What the student saw or heard: The live AI patient reports that edema is completely gone, he has not noticed shortness of breath lately, stairs are fine, and he can perform usual activities without meaningful fatigue or dyspnea. His primary problem is brief positional dizziness.

What the grader expected: The official answer key expects improvement but persistent mild symptoms: residual shortness of breath with more strenuous exertion, improved but not fully normal exercise tolerance, walking about 20 to 30 minutes most days, improved stair tolerance, and occasional mild dizziness.

Why this is a problem: The follow up visit is supposed to assess change from baseline. The live patient is substantially more improved than the intended case, which changes NYHA interpretation and the rationale for titration.

Suggested fix: Carry the intended longitudinal symptom state into the Wednesday AI patient. The same degree of improvement and residual limitation should appear in the answer key, interview, and grader.

Evidence: Wednesday AI patient interview.

"What brings you in today, and how have things been going since your last heart failure visit?"

Well, I've been feeling pretty dizzy when I stand up, especially first thing in the morning or when I get up from my desk too fast. The good news is that swelling in my legs I used to have is gone now. It's just this lightheaded feeling that's been bothering me since I started the new meds.

✧ new info uncovered

"Compared with when we first met, how are your heart failure symptoms now?"

Honestly, a lot better in some ways - that swelling in my legs is completely gone now, which is great. But now I've got this dizziness when I stand up that I didn't have before, so it's kind of a trade-off I guess.

✧ new info uncovered

"How has your shortness of breath changed since the last visit?"

Actually, my breathing has been pretty good, I haven't really noticed being short of breath lately. It's really just this dizziness thing that's been bugging me the most.

✧ new info uncovered

"How has your energy been since starting the new heart medications?"

My energy's been okay, I'd say. Nothing major to complain about there, just really this dizziness when I get up that's been the main issue for me.

✧ new info uncovered

"What kinds of activities are you able to do now, and are there any activities that still make you tired or short of breath?"

I've been able to keep up with my regular stuff at work and around the house without getting too tired or out of breath. I haven't noticed any activities that really wear me out lately - it's really just that dizzy spell when I stand up too fast that gets me.

✧ new info uncovered

"What does your usual physical activity look like now?"

I mostly just do my regular day-to-day stuff - work at my desk, some things around the house. I haven't really been doing any specific exercise routine, but I can get through my normal activities fine without getting too winded.

✧ new info uncovered

The live patient reports near complete symptom resolution apart from positional dizziness.

Evidence: Wednesday AI patient interview.

"How are you doing with stairs compared with before?"

Stairs have been fine, actually, I don't get winded or anything going up them. It's really just when I stand up quickly that the dizziness hits me, not really related to stairs or activity like that.

✧ new info uncovered

Have you noticed any swelling in your feet, ankles, legs, or abdomen?"

No, that swelling I used to have in my legs is actually gone now. It's cleared up pretty well since my last visit.

Do you have any trouble breathing when you lie flat, need extra pillows to sleep, or wake up at night short of breath?

No, I sleep fine flat without needing extra pillows, and I haven't woken up at night gasping for breath or anything like that. That's not been an issue for me.

✧ new info uncovered

Since the last visit, have you had any chest pain, racing or irregular heartbeat, dizziness, lightheadedness, or fainting

No chest pain or anything like a racing or irregular heartbeat that I've noticed. The dizziness is really the main thing - it happens when I stand up too fast, like getting out of bed or from my desk, but I haven't actually fainted, it just passes after a few seconds.

✧ new info uncovered

Can you tell me more about when that happens, how often it occurs, and how long it lasts?

It happens mostly when I get out of bed in the morning or when I stand up quickly from my desk at work. It only lasts a few seconds and then passes - I haven't had it happen at any other random times that I can think of.

✧ new info uncovered

Have you fallen, nearly fallen, passed out, or injured yourself when that happens?

No, I haven't actually fallen or hurt myself from it. I usually just feel dizzy for a second and then it goes away, so I've been careful but haven't had any close calls or injuries.

✧ new info uncovered

Since your last visit, have you had to go to the emergency department or been hospitalized for your heart condition or for any other reason?

The live patient reports stairs are fine and edema is gone.

Answer key evidence: Wednesday intended symptom history.

2. Chief Concern

"I definitely feel better than I did a few months ago, but I don't think I'm quite back to normal yet."

3. History of Present Illness

Michael is a 58-year-old male with HFrEF, hypertension, hyperlipidemia, and obesity presenting for routine follow-up.

Since the previous visit:

- Shortness of breath has improved
- Fatigue has improved
- Exercise tolerance has improved
- Lower extremity swelling has resolved

He reports:

- Walking approximately 20–30 minutes most days
- Increased energy
- Improved ability to climb stairs

He denies:

- Chest pain
- Palpitations
- Syncope
- Orthopnea
- PND
- Edema
- Hospitalization
- Emergency department visits

He reports occasional mild dizziness when standing quickly but denies falls or injury.

The answer key expects residual exertional symptoms and walking 20 to 30 minutes most days.

Issue 3. Home blood pressure and weight monitoring expected by the answer key are not present in the live AI patient

What the student saw or heard: Michael states that he is not consistently weighing himself and does not have a routine for home blood pressure or pulse monitoring, so he cannot provide typical readings.

What the grader expected: The answer key requires daily stable weights, regular home blood pressure monitoring with an average of 120/74 mmHg, and average home heart rate of 66 bpm.

Why this is a problem: The student directly asked about home monitoring. The required daily routine and numeric averages cannot be documented when the patient says the routine does not exist.

Suggested fix: Synchronize home monitoring behavior across the follow up AI patient, visible chart, and answer key. If averages are graded, make the log or averages available to the student.

Evidence: Wednesday AI patient interview.

Have you been weighing yourself at home? Tell me what your routine looks like and whether you have noticed any changes

Honestly, I haven't been super consistent with weighing myself at home. I don't have a set routine for it, so I can't really say if there's been much change one way or another.

Have you been checking your blood pressure or pulse at home? What have your readings generally looked like?

No, I haven't really been checking my blood pressure or pulse at home. I don't have a routine set up for that, so I couldn't tell you what my numbers usually look like.

What changes, if any, have you made to the way you eat since your last visit?

I haven't really made any specific changes to my eating since the last visit. I'm still eating pretty much the way I normally do.

✦ new info uncovered

Have you made any other lifestyle changes since the last visit, such as changes in exercise or activity?

No, not really - I haven't made any specific changes to my exercise or activity levels since last time. I'm just keeping up with my normal day-to-day stuff like before.

✦ new info uncovered

What is your understanding of heart failure and what your medications are doing for your heart?

From what I understand, my heart isn't pumping as strong as it should, so these medications are supposed to help it work better and keep me from retaining fluid. My doctor's explained it helps take some strain off my heart, but I'll admit I don't know all the technical details behind each one.

✦ new info uncovered

What questions or concerns do you still have about your heart failure or your medications?

Really my biggest concern right now is this dizziness - I just want to make sure it's not something serious and that there's something we can do about it. Otherwise I feel like I understand my condition and medications pretty well at this point.

✦ new info uncovered

What is the most important health goal you want to work toward over the next few months?

Michael reports inconsistent weights and no home BP or pulse routine.

Answer key evidence: Wednesday home monitoring log.

Weight: 99 kg

Height: 70 inches

BMI: 31.3 kg/m²

SpO₂: 98% on room air

Home Monitoring Log

Average BP:

120/74 mmHg

Average HR:

66 bpm

Daily weights stable

No recent weight gain

8. Laboratory Data

CMP

Na: 138 mEq/L

K: 4.5 mEq/L

BUN: 18 mg/dL

SCr: 1.1 mg/dL

eGFR: 80 mL/min/1.73m²

AST: 22 U/L

ALT: 25 U/L

BNP

Current: 165 pg/mL

The answer key requires daily stable weights and specific home BP and HR averages.

Issue 4. Adherence and lifestyle progress in the answer key are not returned by the live AI patient

What the student saw or heard: The live patient reports that he cannot recall missing doses and denies barriers. He says he has made no specific dietary changes and no specific exercise or activity changes since the previous visit.

What the grader expected: The official answer key requires occasional missed evening sacubitril/valsartan doses about once every 1 to 2 weeks, walking 20 to 30 minutes most days, and active efforts to reduce sodium and read food labels.

Why this is a problem: These details are used to grade adherence and self-management progress. The student cannot report improvement or a missed dose pattern that the patient explicitly does not provide.

Suggested fix: Persist the intended adherence and lifestyle history into the Wednesday AI patient. If the grader values a specific missed dose frequency or sodium and exercise progress, those facts should be returned by the patient when asked.

Evidence: Wednesday AI patient interview.

No, I haven't had to go to the ER or been hospitalized for anything since my last visit. Just dealing with this dizziness at home, but nothing serious enough for that.

✧ new info uncovered

Please walk me through every medication you are taking right now, including the name, strength, how often you take it, and anything you take over the counter.

Sure, let me think through this. I take the Entresto - that's 24/26 milligrams, twice a day. Then there's Carvedilol, 3.125 milligrams, also twice a day. I take Spironolactone, 25 milligrams once a day, and Dapagliflozin, 10 milligrams once a day. For blood pressure I also take Hydrochlorothiazide, 25 milligrams once daily, and Atorvastatin 40 milligrams once a day for my cholesterol. As far as over-the-counter stuff, I just take a daily multivitamin, nothing else.

✧ new info uncovered

How have you been feeling on your medications? Have you noticed any side effects or anything different since starting them?

The main thing I've noticed is this dizziness when I stand up, especially getting out of bed or from my desk - that started after I began the new medications. Other than that, I haven't really had any other side effects that I've noticed.

✧ new info uncovered

How consistently are you taking each of your medications?

I take them pretty consistently, I try to stick to the schedule every day. I haven't really missed doses that I can think of.

✧ new info uncovered

Do you ever miss or forget a dose? If so, which medication or time of day tends to be missed, and about how often does that happen?

No, I don't think I really miss doses - I try to stay pretty on top of taking everything as scheduled. I can't think of any particular medication or time of day where I slip up.

✧ new info uncovered

Has anything made it difficult to take your medications, such as your schedule, cost, refills, insurance, transportation, or remembering doses?

No, nothing like that has really been a problem. My schedule, cost, refills, insurance - all of that's been fine, and I haven't had trouble getting to the pharmacy or remembering to take them either.

✧ new info uncovered

The patient reports consistent adherence without a recalled missed dose pattern.

Evidence: Wednesday AI patient interview.

Have you been weighing yourself at home? Tell me what your routine looks like and whether you have noticed any changes

Honestly, I haven't been super consistent with weighing myself at home. I don't have a set routine for it, so I can't really say if there's been much change one way or another.

Have you been checking your blood pressure or pulse at home? What have your readings generally looked like?

No, I haven't really been checking my blood pressure or pulse at home. I don't have a routine set up for that, so I couldn't tell you what my numbers usually look like.

What changes, if any, have you made to the way you eat since your last visit?

I haven't really made any specific changes to my eating since the last visit. I'm still eating pretty much the way I normally do.

✦ new info uncovered

Have you made any other lifestyle changes since the last visit, such as changes in exercise or activity?

No, not really - I haven't made any specific changes to my exercise or activity levels since last time. I'm just keeping up with my normal day-to-day stuff like before.

✦ new info uncovered

What is your understanding of heart failure and what your medications are doing for your heart?

From what I understand, my heart isn't pumping as strong as it should, so these medications are supposed to help it work better and keep me from retaining fluid. My doctor's explained it helps take some strain off my heart, but I'll admit I don't know all the technical details behind each one.

✦ new info uncovered

What questions or concerns do you still have about your heart failure or your medications?

Really my biggest concern right now is this dizziness - I just want to make sure it's not something serious and that there's something we can do about it. Otherwise I feel like I understand my condition and medications pretty well at this point.

✦ new info uncovered

What is the most important health goal you want to work toward over the next few months?

The patient reports no specific diet or exercise changes.

Answer key evidence: Wednesday intended social and lifestyle progress.

Previous: 320 pg/mL

9. Immunizations

No changes since previous visit.

10. Social History

Former smoker

Alcohol:

1–2 drinks on weekends

Exercise:

Walking 20–30 minutes most days

Diet:

Attempting sodium reduction

Reports reading food labels more frequently

11. Family History

No changes

12. Problem List

Active Problems

HFrEF improving on GDMT

Hypertension controlled

Hyperlipidemia

The answer key requires weekend alcohol use, walking most days, and sodium reduction.

Issue 5. Visible Wednesday vital signs and weight differ from the official answer key while the grader accepts the live values

What the student saw or heard: The live EMR shows BP 106/68 mmHg, repeat BP 104/66 mmHg, HR 62 bpm, temperature 98.6 F, SpO2 97%, and weight 101 kg.

What the grader expected: The official Wednesday answer key expects BP 118/72 mmHg, HR 68 bpm, temperature 98.2 F, SpO2 98% on room air, and weight 99 kg. The Wednesday grader nevertheless treats the live low blood pressure and orthostasis state as the correct case.

Why this is a problem: These are not trivial differences. They determine whether GDMT should be titrated, held, or deescalated. The answer key and grader should not point to different objective states.

Suggested fix: Create one Wednesday objective dataset and use it for the answer key, live EMR, and grader. Regression testing should compare the exact values before release.

Evidence: Wednesday live vital signs.

Vitals				
Vital signs recorded at this visit				
VITAL	VALUE	REFERENCE	TREND	FLAG
Office BP 06/23/2026 09:14	106/68	<130/80 mmHg		
Repeat BP 06/23/2026 09:14	104/66	<130/80 mmHg	—	
Heart rate 06/23/2026 09:14	62	60–100 bpm	—	
Respiratory rate 06/23/2026 09:14	16	12–20 /min	—	
Temperature 06/23/2026 09:14	98.6°F	97.0–99.5 °F	—	
SpO ₂ 06/23/2026 09:14	97%	95–100 %	—	
Weight 06/23/2026 09:14	101 kg	—		
Height 06/23/2026 09:14	5'10"	—	—	
BMI 06/23/2026 09:14	31.9	18.5–24.9 kg/m ²	—	
BSA (Mosteller) 06/23/2026 09:14	2.04 m ²	—	—	

The live Wednesday EMR shows the low blood pressure orthostasis state.

Answer key evidence: Wednesday intended vital signs.

4. Past Medical History

Heart Failure with Reduced Ejection Fraction

Hypertension

Hyperlipidemia

Obesity

5. Medication List

Sacubitril/Valsartan 49/51 mg PO BID

Metoprolol Succinate 50 mg PO daily

Empagliflozin 10 mg PO daily

Atorvastatin 40 mg PO daily

6. Allergies

Drug Allergies

NKDA

Food Allergies

None

7. Vitals

Today's Vitals

BP: 118/72 mmHg

HR: 68 bpm

RR: 16

Temp: 98.2°F

The official answer key lists different BP, HR, temperature, and weight values.

Grader evidence: Wednesday AI feedback.

AI Grading Feedback

Week 4 · Michael Thompson · Wednesday · Generated 9/8/2026, 12:03:42 PM

87.5%

✓ STRENGTHS

The student produced a thorough, well-organized SOAP note that captured nearly all subjective details regarding the dizziness (onset, triggers, duration, absence of falls/syncope), accurately documented the full GDMT regimen and vital signs, and correctly identified orthostatic hypotension as the primary medical therapy problem. The assessment appropriately linked HCTZ discontinuation as the key intervention while explicitly preserving all four GDMT pillars, and the plan included appropriate monitoring for renal function/electrolytes, home BP/weight monitoring with clear escalation thresholds, and patient counseling on positional changes. Family history, allergies, social history, and immunization status were all accurately captured.

○ OPPORTUNITIES TO IMPROVE

To strengthen the assessment, explicitly compare current NT-proBNP (165) and weight (101 kg) to baseline values (320 pg/mL and +2 kg) to demonstrate objective evidence of treatment response, and use this trend to support the assessment that HFrEF is improving on GDMT. When discussing the etiology of orthostatic hypotension, acknowledge that carvedilol and sacubitril/valsartan also contribute to the patient's BP lowering in addition to HCTZ, even though the plan correctly prioritizes HCTZ discontinuation. Finally, align the follow-up interval with the expected ~1 month timeframe for reassessment of GDMT titration, though the 1-2 week recheck for BMP/orthostasis is reasonable and could be described as an interim visit prior to the 1-month follow-up.

▮ GOLD STANDARD COMPARISON

—

This is AI-generated formative feedback — a study aid. Use your preceptor's judgment as the final word.

The grader praises the live orthostasis assessment rather than the official answer key state.

Issue 6. The grader requires a 2 kg weight decrease that is supported by neither the live baseline nor the answer key

What the student saw or heard: The live Tuesday weight is 102 kg, and the live Wednesday weight is 101 kg, which represents a 1 kg decrease.

What the grader expected: The Wednesday grader deducts because the SOAP note does not describe 101 kg as a 2 kg decrease from baseline. The official answer key also does not support that value because it lists 102 kg on Tuesday and 99 kg on Wednesday, which would be a 3 kg decrease.

Why this is a problem: The required trend matches neither available data source. This suggests a stale or third baseline value in the grading logic and makes the trend deduction impossible to reproduce.

Suggested fix: Calculate trend values from the same encounter dataset shown to the student. Add automated checks that compare the baseline and follow up values used by the grader before a trend-based deduction is generated.

Evidence: Tuesday live baseline weight.

Vitals				
Vital signs recorded at this visit				
VITAL	VALUE	REFERENCE	TREND	FLAG
Office BP 06/23/2026 09:14	138/86	<130/80 mmHg		
Repeat BP 06/23/2026 09:14	136/84	<130/80 mmHg	—	
Heart rate 06/23/2026 09:14	82	60–100 bpm	—	
Respiratory rate 06/23/2026 09:14	16	12–20 /min	—	
Temperature 06/23/2026 09:14	98.6°F	97.0–99.5 °F	—	
SpO ₂ 06/23/2026 09:14	96%	95–100 %	—	
Weight 06/23/2026 09:14	102 kg	—		
Height 06/23/2026 09:14	5'10"	—	—	
BMI 06/23/2026 09:14	32.2	18.5–24.9 kg/m ²	—	
BSA (Mosteller) 06/23/2026 09:14	2.04 m ²	—	—	

The live Tuesday weight is 102 kg.

Evidence: Wednesday live follow up weight.

Vitals

Vital signs recorded at this visit

VITAL	VALUE	REFERENCE	TREND	FLAG
Office BP 06/23/2026 09:14	106/68	<130/80 mmHg		
Repeat BP 06/23/2026 09:14	104/66	<130/80 mmHg	—	
Heart rate 06/23/2026 09:14	62	60–100 bpm	—	
Respiratory rate 06/23/2026 09:14	16	12–20 /min	—	
Temperature 06/23/2026 09:14	98.6°F	97.0–99.5 °F	—	
SpO ₂ 06/23/2026 09:14	97%	95–100 %	—	
Weight 06/23/2026 09:14	101 kg	—		
Height 06/23/2026 09:14	5'10"	—	—	
BMI 06/23/2026 09:14	31.9	18.5–24.9 kg/m ²	—	
BSA (Mosteller) 06/23/2026 09:14	2.04 m ²	—	—	

The live Wednesday weight is 101 kg.

Grader evidence: Wednesday AI feedback.

AI Grading Feedback

Week 4 · Michael Thompson · Wednesday · Generated 9/8/2026, 12:03:42 PM

87.5%

✓ STRENGTHS

The student produced a thorough, well-organized SOAP note that captured nearly all subjective details regarding the dizziness (onset, triggers, duration, absence of falls/syncope), accurately documented the full GDMT regimen and vital signs, and correctly identified orthostatic hypotension as the primary medical therapy problem. The assessment appropriately linked HCTZ discontinuation as the key intervention while explicitly preserving all four GDMT pillars, and the plan included appropriate monitoring for renal function/electrolytes, home BP/weight monitoring with clear escalation thresholds, and patient counseling on positional changes. Family history, allergies, social history, and immunization status were all accurately captured.

○ OPPORTUNITIES TO IMPROVE

To strengthen the assessment, explicitly compare current NT-proBNP (165) and weight (101 kg) to baseline values (320 pg/mL and +2 kg) to demonstrate objective evidence of treatment response, and use this trend to support the assessment that HFrEF is improving on GDMT. When discussing the etiology of orthostatic hypotension, acknowledge that carvedilol and sacubitril/valsartan also contribute to the patient's BP lowering in addition to HCTZ, even though the plan correctly prioritizes HCTZ discontinuation. Finally, align the follow-up interval with the expected ~1 month timeframe for reassessment of GDMT titration, though the 1-2 week recheck for BMP/orthostasis is reasonable and could be described as an interim visit prior to the 1-month follow-up.

The grader states that 101 kg should be interpreted as a 2 kg decrease.

Issue 7. BNP and NT proBNP naming is inconsistent across the Wednesday sources

What the student saw or heard: The live Wednesday laboratory panel identifies NT proBNP 165 pg/mL, and the grader also refers to NT proBNP 165 pg/mL with baseline NT proBNP 320 pg/mL.

What the grader expected: The official Wednesday answer key uses BNP 165 pg/mL with prior BNP 320 pg/mL, and the Guiding Question asks why BNP trending is useful.

Why this is a problem: BNP and NT proBNP are related but different assays. Reusing the same numeric values while changing the test name creates educational and grading ambiguity.

Suggested fix: Choose one natriuretic peptide assay for the case and use the exact same name in all visits, answer keys, laboratory panels, educational questions, trends, and grader rules.

Evidence: Wednesday live cardiac biomarker panel.

Labs

Results are organized into panels — open the one you need

Lab panel

Cardiac Biomarkers

Cardiac Biomarkers

Collected: 06/09/2026 07:50

TEST	RESULT	REFERENCE RANGE	FLAG
NT-proBNP	165 pg/mL	—	High

The live Wednesday EMR identifies NT proBNP 165 pg/mL.

Evidence: Wednesday Guiding Question.

Guiding Questions

Questions to scaffold your reasoning — they guide without giving away the answer

- 1 What findings demonstrate treatment success?
- 2 Why is MRA therapy appropriate today?
- 3 Should mild orthostatic symptoms change management?
- 4 What monitoring is required after spironolactone initiation?
- 5 Why is BNP trending useful?
- 6 What is the next long-term goal?

The educational prompt asks about BNP trending.

Answer key evidence: Wednesday intended laboratory data.

Weight: 99 kg

Height: 70 inches

BMI: 31.3 kg/m²

SpO₂: 98% on room air

Home Monitoring Log

Average BP:

120/74 mmHg

Average HR:

66 bpm

Daily weights stable

No recent weight gain

8. Laboratory Data

CMP

Na: 138 mEq/L

K: 4.5 mEq/L

BUN: 18 mg/dL

SCr: 1.1 mg/dL

eGFR: 80 mL/min/1.73m²

AST: 22 U/L

ALT: 25 U/L

BNP

Current: 165 pg/mL

The official answer key also uses BNP rather than NT proBNP.

Issue 8. Three month follow up vitals and laboratory results reuse the baseline timestamps

What the student saw or heard: The Wednesday encounter is labeled a 3 month follow up, but the vital sign table still uses 06/23/2026 at 09:14, the same timestamp used on Tuesday. The Wednesday laboratory values also use the same 06/09/2026 at 07:50 collection time as the Tuesday baseline despite different potassium, creatinine, eGFR, and NT proBNP results.

What the grader expected: The answer key treats Wednesday as a true 3 month longitudinal follow up with new objective data and trends.

Why this is a problem: Different measurements cannot all come from the same exact encounter time and the same exact laboratory specimen. The static metadata makes valid longitudinal trends clinically impossible as displayed.

Suggested fix: Assign encounter specific vital sign dates and separate laboratory collection dates for the 3-month visit. Trend logic should reference dated results rather than reusing baseline timestamps.

Evidence: longitudinal vital sign timestamps.

Tuesday initial visit

Vitals

Vital signs recorded at this visit

VITAL	VALUE	REFERENCE	TREND	FLAG
Office BP 06/23/2026 09:14	138/88	<130/80 mmHg		
Repeat BP 06/23/2026 09:14	136/84	<130/80 mmHg	—	
Heart rate 06/23/2026 09:14	82	60–100 bpm	—	
Respiratory rate 06/23/2026 09:14	16	12–20 /min	—	
Temperature 06/23/2026 09:14	98.6°F	97.0–99.5 °F	—	
SpO ₂ 06/23/2026 09:14	96%	95–100 %	—	
Weight 06/23/2026 09:14	102 kg	—		
Height 06/23/2026 09:14	5'10"	—	—	
BMI 06/23/2026 09:14	32.2	18.5–24.9 kg/m ²	—	
BSA (Mosteller) 06/23/2026 09:14	2.04 m ²	—	—	

Wednesday 3 month follow up

Vitals

Vital signs recorded at this visit

VITAL	VALUE	REFERENCE	TREND	FLAG
Office BP 06/23/2026 09:14	106/68	<130/80 mmHg		
Repeat BP 06/23/2026 09:14	104/66	<130/80 mmHg	—	
Heart rate 06/23/2026 09:14	62	60–100 bpm	—	
Respiratory rate 06/23/2026 09:14	16	12–20 /min	—	
Temperature 06/23/2026 09:14	98.6°F	97.0–99.5 °F	—	
SpO ₂ 06/23/2026 09:14	97%	95–100 %	—	
Weight 06/23/2026 09:14	101 kg	—		
Height 06/23/2026 09:14	5'10"	—	—	
BMI 06/23/2026 09:14	31.9	18.5–24.9 kg/m ²	—	
BSA (Mosteller) 06/23/2026 09:14	2.04 m ²	—	—	

Thursday 6 month follow up

Best Practice Advisories

SYSTEM-GENERATED ADVISORIES — REVIEW AND ACT

Key vitals

Office BP 06/23/2026 09:14		122/78
Repeat BP 06/23/2026 09:14		120/76
HR 06/23/2026 09:14		70 bpm
Temp 06/23/2026 09:14		98.6°F
RR 06/23/2026 09:14		16
SpO ₂ 06/23/2026 09:14		97%
Weight 06/23/2026 09:14		101 kg
Height 06/23/2026 09:14		5'10"
Pain 06/23/2026 09:14		0/10
BMI 06/23/2026 09:14		31.9

Active problems

HFrEF
Stable, requires GDMT titration

Key labs

eGFR Ref — 06/09/2026 07:50	75 mL/min/1.73m ²
SCr Ref — 06/09/2026 07:50	1.2 mg/dL
NT-proBNP Ref — 06/09/2026 07:50	92 pg/mL

Tuesday, Wednesday, and Thursday use the same vital sign timestamp despite different values.

Evidence: longitudinal laboratory timestamps.

Tuesday: NT proBNP 320 pg/mL

All Labs		Collected: 06/09/2026 07:50	
TEST	RESULT	REFERENCE RANGE	FLAG
NT-proBNP	320 pg/mL	—	High
K	4.2 mEq/L	—	
SCr	1.0 mg/dL	—	
eGFR	88 mL/min/1.73m ²	—	

Wednesday: NT proBNP 165 pg/mL

Labs

Results are organized into panels — open the one you need

Lab panel Cardiac Biomarkers

Cardiac Biomarkers

Collected: 06/09/2026 07:50

TEST	RESULT	REFERENCE RANGE	FLAG
NT-proBNP	165 pg/mL	—	High

Thursday: NT proBNP 92 pg/mL

Labs

Results are organized into panels — open the one you need

Lab panel Cardiac Biomarkers

Cardiac Biomarkers

Collected: 06/09/2026 07:50

TEST	RESULT	REFERENCE RANGE	FLAG
NT-proBNP	92 pg/mL	—	

The biomarker and renal values change while the collection timestamp remains identical.

Patient A – Michael Thompson – Thursday 6-Month Follow-Up

8 confirmed system/data issues are included for this visit. Items are grouped so one underlying problem is not counted multiple times just because it caused several grader deductions.

#	Issue	Problem type
1	HFimpEF and repeat LVEF improvement required by the grader are not visible in the live Thursday case.	Required information is unavailable
2	Medication agents and doses in the live Thursday EMR do not match the grader case.	EMR and grader disagree
3	Thursday vital signs and weight in the live EMR differ from the values used by the grader.	EMR and grader disagree
4	Daily home monitoring, sodium restriction, and activity required by the grader conflict with the AI patient.	AI patient and grader disagree
5	Surgical, family, smoking, and alcohol history again conflict with the persistent grader case.	AI patient and grader disagree
6	Thursday preventive care updates required by the answer key are not exposed in the live immunization panel.	Required information is unavailable
7	The same Thursday encounter is labeled both a 6 Month Follow Up and an Initial Ambulatory Care Pharmacist Visit.	Internal EMR contradiction
8	Six month follow up vitals and laboratory results continue to reuse baseline timestamps.	Longitudinal data integrity issue

Issue 1. HFimpEF and repeat LVEF improvement required by the grader are not visible in the live Thursday case

What the student saw or heard: The live Thursday problem list states HFimpEF, stable, requires GDMT titration. The live evidence shows NT proBNP 92 pg/mL but does not display a repeat LVEF of 45% or a visible prior to current EF trend. The patient asks whether doses still need to be increased rather than whether medications can be stopped because the EF improved.

What the grader expected: The official answer key and grader require HFimpEF with LVEF improving from 35% to 45%, interpretation of improved systolic function, and counseling that treatment success does not mean cure.

Why this is a problem: HFimpEF requires objective EF improvement. A student should not diagnose HFimpEF or document LVEF 45% when the repeat echocardiogram is not visible in the live chart.

Suggested fix: Expose the repeat echocardiogram and the 35% to 45% EF trend in the Thursday live EMR before grading HFimpEF. If the live HFimpEF titration case is intended instead, update the answer key and grader to that case.

Evidence: Thursday live cardiac data.

Labs

Results are organized into panels — open the one you need

Lab panel

Cardiac Biomarkers

Cardiac Biomarkers

Collected: 06/09/2026 07:50

TEST	RESULT	REFERENCE RANGE	FLAG
NT-proBNP	92 pg/mL	—	

The live Thursday encounter shows NT proBNP 92 pg/mL without the required repeat EF evidence.

Answer key evidence: Thursday intended BNP and repeat echocardiogram data.

ALT: 24 U/L

BNP

Current: 92 pg/mL

Previous: 165 pg/mL

Previous Baseline: 320 pg/mL

Repeat Echocardiogram

LVEF: 45%

Previous: 35%

Interpretation:

Improved systolic function

9. Immunizations

Pneumococcal vaccine received since last visit.

Shingrix dose #1 received.

10. Social History

Former smoker

Alcohol:

1–2 drinks on weekends

Exercise:

Walking 30–40 minutes most days

Diet:

The official answer key contains BNP 92 and repeat LVEF 45% from a previous 35%.

Grader evidence: Thursday grading feedback.

- ☐ [1E.2] -0.4 pts: Sacubitril/valsartan dose documented as 24/26 mg instead of expected 97/103 mg.
- ☐ [1E.4] -0.3 pts: Carvedilol is listed instead of metoprolol succinate/XL.
- ☐ [1E.5] -0.3 pts: Incorrect medication (carvedilol) documented instead of metoprolol succinate 100 mg.
- ☐ [1E.6] -0.2 pts: Carvedilol BID documented instead of metoprolol succinate PO daily.
- ☐ [1E.7] -0.3 pts: Dapagliflozin is listed instead of empagliflozin.
- ☐ [1E.8] -0.3 pts: Incorrect medication (dapagliflozin) documented instead of empagliflozin 10 mg.
- ☐ [1E.9] -0.2 pts: Dapagliflozin PO daily documented instead of empagliflozin PO daily.
- ☐ [1E.18] -0.3 pts: COVID vaccination status not mentioned.
- ☐ [1E.19] -0.3 pts: Pneumococcal vaccination status not mentioned.
- ☐ [1E.20] -0.3 pts: Shingrix vaccination status not mentioned.
- ☐ [1E.21] -0.4 pts: Tdap status/timeframe not mentioned.
- ☐ [1G.2] -0.5 pts: Patient's question is framed around dose increase, not discontinuation of medications.
- ☐ [1G.3] -0.4 pts: No link made between patient's question and EF improvement/symptom resolution; EF not discussed.
- ☐ [1G.4] -0.3 pts: Note explicitly states no major recent lifestyle changes, contradicting engagement in lifestyle modifications.
- ☐ [1G.5] -0.4 pts: Note states no structured exercise routine, not ongoing physical activity as a goal.
- ☐ [2A.1] -0.6 pts: Documented BP 122/78 mmHg does not match expected 112/68 mmHg.
- ☐ [2A.2] -0.4 pts: Documented HR 70 bpm does not match expected 62 bpm.
- ☐ [2A.4] -0.3 pts: Documented temp 98.6°F does not match expected 98.1°F.
- ☐ [2A.5] -0.4 pts: Documented SpO2 97% does not match expected 98% on room air.
- ☐ [2A.6] -0.4 pts: Documented weight 101 kg does not match expected 97 kg.
- ☐ [2A.8] -0.5 pts: Documented BMI 31.9 does not match expected 30.7 kg/m².
- ☐ [2A.9] -0.5 pts: Average home BP not documented.
- ☐ [2A.10] -0.3 pts: Average home HR not documented.
- ☐ [2A.11] -1.0 pts: Note states patient does not weigh himself, contradicting stable daily weight monitoring.
- ☐ [2B.1] -0.3 pts: Sodium value not documented.
- ☐ [2B.3] -0.3 pts: BUN value not documented.
- ☐ [2B.6] -0.3 pts: AST value not documented.
- ☐ [2B.7] -0.3 pts: ALT value not documented.
- ☐ [2B.9] -0.5 pts: Previous BNP value not documented.
- ☐ [2B.10] -0.5 pts: Baseline BNP value not documented.
- ☐ [2B.11] -0.7 pts: Current LVEF value not documented; note states EF data unavailable.
- ☐ [2B.12] -0.4 pts: Previous LVEF value not documented.
- ☐ [2B.13] -0.4 pts: Echocardiogram interpretation of improved LV systolic function not documented.
- ☐ [2C.1] -0.5 pts: BMI calculation based on incorrect weight (101 kg) rather than expected 97 kg, so not reconciled correctly.
- ☐ [2C.2] -0.7 pts: BNP trend across baseline, previous, and current values not identified.
- ☐ [2C.3] -0.6 pts: LVEF improvement trend (35% to 45%) not identified.
- ☐ [2C.4] -0.3 pts: States potassium is stable but does not explicitly contextualize in relation to spironolactone/sacubitril-valsartan therapy.
- ☐ [2C.5] -0.6 pts: eGFR value stated but not explicitly linked to supporting continuation of four-drug GDMT.
- ☐ [2C.6] -0.5 pts: Absence of lipid panel not flagged despite ongoing statin therapy.
- ☐ [2C.7] -0.5 pts: Home monitoring values not documented or compared to office vitals.
- ☐ [3A.1] -0.9 pts: HFimpEF diagnosis not made; assessment explicitly states it cannot be confirmed due to missing EF data.

<https://newvacs.vercel.app/app>

3/6

The grader requires the hidden EF trend and HFimpEF interpretation.

Issue 2. Medication agents and doses in the live Thursday EMR do not match the grader case

What the student saw or heard: The live Thursday medication list shows sacubitril/valsartan 24/26 mg twice daily, carvedilol 3.125 mg twice daily, spironolactone 25 mg daily, dapagliflozin 10 mg daily, and atorvastatin 40 mg daily.

What the grader expected: The official answer key and grader expect sacubitril/valsartan 97/103 mg twice daily, metoprolol succinate 100 mg daily, empagliflozin 10 mg daily, spironolactone 25 mg daily, and atorvastatin 40 mg daily.

Why this is a problem: The student is penalized for accurately documenting the live medication list. The differences are clinically important because they change dose optimization, heart rate control, tolerability, and counseling.

Suggested fix: Make the Thursday EMR and grader use the same maintenance regimen. If the answer key regimen is intended, update the live medication list and AI patient medication memory to those exact agents and doses.

Evidence: Thursday live medication list.

Medications

Documented medication list (as recorded in the chart)

Medications

Current / Home meds (5)Discontinued (0)

✓ Allergy/interaction check active · NKDA — no conflicts

PRESCRIBED / ACTIVE MEDICATIONS

MEDICATION	STATUS	DOSE	ROUTE	FREQ	INDICATION	START	PRESCRIBER	LAST FILLED	REFILLS	PHA
Sacubitril/Valsartan (Entresto) ✓ Sig:	Active	24/26 mg	by mouth	BID	HFrEF			Qty		
Carvedilol ✓ Sig:	Active	3.125 mg	by mouth	BID	HFrEF			Qty		
Spironolactone ✓ Sig:	Active	25 mg	by mouth	daily	HFrEF			Qty		
Dapagliflozin ✓ Sig:	Active	10 mg	by mouth	daily	HFrEF			Qty		
Atorvastatin ✓ Sig:	Active	40 mg	by mouth	daily	HLD			Qty		

The live Thursday EMR displays low dose Entresto, carvedilol, dapagliflozin, spironolactone, and atorvastatin.

Answer key evidence: Thursday intended medication list.

4. Past Medical History

Heart Failure with Reduced Ejection Fraction

Hypertension

Hyperlipidemia

Obesity

5. Medication List

Sacubitril/Valsartan 97/103 mg PO BID

Metoprolol Succinate 100 mg PO daily

Empagliflozin 10 mg PO daily

Spirolactone 25 mg PO daily

Atorvastatin 40 mg PO daily

6. Allergies

Drug Allergies

NKDA

Food Allergies

None

7. Vitals

Today's Vitals

BP: 112/68 mmHg

The official answer key uses high dose Entresto, metoprolol succinate, and empagliflozin.

Grader evidence: Thursday grading feedback.

a controlled problem with specific BP values, preventive care/vaccine gaps such as Shingrix and Tdap, and target-titrated doses of sacubitril/valsartan 97/103mg and metoprolol succinate 100mg) were not addressed. To improve, the student should explicitly document current EF value, compare BNP trends over time, assess and label hypertension status with supporting BP values if present, address preventive care/immunization gaps in the assessment, and align medication continuation with target GDMT dosing when applicable, along with explicit discussion of HFimpEF concepts and guideline-based rationale for continuing therapy despite improvement. To strengthen the note, explicitly discuss the HFimpEF concept and reinforce that clinical improvement reflects treatment success rather than cure, since these are key patient education points for this GDMT-titration visit. Add explicit statements about daily (not just 'regular') weight monitoring, medication adherence monitoring, and repeat echocardiography per cardiology follow-up. Include preventive health maintenance items (Shingrix series completion, Tdap status) since these are part of comprehensive primary/cardiology care. Finally, specify a follow-up timeframe (e.g., 3-6 months) and explicitly state the goal of maintaining maximally tolerated GDMT long term, and reinforce that follow-up remains necessary despite feeling well.

- ☐ [1A.4] -0.2 pts: Encounter setting (ambulatory care cardiology clinic) is not stated anywhere in the note.
- ☐ [1A.7] -0.1 pts: States '6 month heart failure follow up' but does not tie this timeframe specifically to GDMT initiation.
- ☐ [1A.9] -0.5 pts: Spironolactone is listed as a current medication but is not identified as the most recent addition since the prior visit.
- ☐ [1B.1] -0.3 pts: No comparative statement of substantial improvement since diagnosis is documented.
- ☐ [1B.2] -0.3 pts: Improved exercise tolerance is not reported; note states no structured exercise routine.
- ☐ [1B.3] -0.2 pts: Energy described as 'good' currently but not framed as improved from baseline.
- ☐ [1B.4] -0.2 pts: No mention of previously dyspnea-inducing activities now being easier.
- ☐ [1B.5] -0.4 pts: No walking frequency/duration documented; note states no structured exercise routine.
- ☐ [1B.14] -0.3 pts: Note states 'no unusual fatigue' rather than occasional fatigue with strenuous exertion.
- ☐ [1B.16] -0.2 pts: Note explicitly states patient does not routinely weigh himself, contradicting daily weight monitoring.
- ☐ [1B.17] -0.2 pts: Patient cannot report recent weight trends; stable weights not documented.
- ☐ [1B.18] -0.2 pts: No specific sodium reduction efforts documented; patient has no sodium/fluid goal and reports only 'eating reasonably'.
- ☐ [1B.20] -0.5 pts: Patient's question is about increasing doses, not about discontinuing medication due to improvement.
- ☐ [1C.2] -0.2 pts: Hypertension is not listed in PMH.
- ☐ [1C.4] -0.2 pts: Obesity is not listed in PMH despite BMI 31.9 in objective.
- ☐ [1C.5] -0.3 pts: Note states 'no surgeries or procedures recalled,' omitting appendectomy.
- ☐ [1C.6] -0.3 pts: Father's hypertension is not documented; mother is listed with hypertension instead.
- ☐ [1C.7] -0.3 pts: Father's MI is documented but age stated as 64 instead of the expected 62.
- ☐ [1C.8] -0.5 pts: Mother's type 2 diabetes is not documented; hypertension is listed instead.
- ☐ [1C.9] -0.3 pts: Surgical history is documented as none recalled rather than appendectomy.
- ☐ [1C.10] -0.4 pts: Only 2 of 4 PMH conditions (HFrEF, hyperlipidemia) are listed; hypertension and obesity are omitted.
- ☐ [1C.11] -0.6 pts: Family history includes unsupported additional diagnoses (father's CAD and HF) beyond documented facts, and omits/misassigns expected findings.
- ☐ [1D.5] -0.3 pts: Pack-year smoking history is not quantified.
- ☐ [1D.7] -0.4 pts: Alcohol use documented as 'rare, special occasions' rather than the expected 1-2 drinks on weekends.
- ☐ [1D.9] -0.2 pts: Note states no structured exercise routine, contradicting regular physical activity.
- ☐ [1D.10] -0.3 pts: No specific walking routine or physical activity detail is documented.

<https://newvacs.vercel.app/app>

2/6

The grader deducts for the live medication names and doses.

Issue 3. Thursday vital signs and weight in the live EMR differ from the values used by the grader

What the student saw or heard: The live Thursday EMR shows BP 122/78 mmHg, repeat BP 120/76 mmHg, HR 70 bpm, temperature 98.6 F, SpO2 97%, weight 101 kg, and BMI 31.9 kg/m2.

What the grader expected: The answer key and grader expect BP 112/68 mmHg, HR 62 bpm, temperature 98.1 F, SpO2 98% on room air, weight 97 kg, and BMI 30.7 kg/m2.

Why this is a problem: The SOAP note uses the actual visible objective measurements but is penalized for not using hidden values. These differences also affect decisions about further GDMT titration and tolerability.

Suggested fix: Synchronize Thursday vitals and anthropometrics across the live EMR, answer key, and grader. Objective data should be graded against the exact values displayed to the student.

Evidence: Thursday live vital signs.

 Best Practice Advisories

SYSTEM-GENERATED ADVISORIES — REVIEW AND ACT

 Key vitals

Office BP	06/23/2026 09:14	 122/78
Repeat BP	06/23/2026 09:14	120/76
HR	06/23/2026 09:14	70 bpm
Temp	06/23/2026 09:14	98.6°F
RR	06/23/2026 09:14	16
SpO ₂	06/23/2026 09:14	97%
Weight	06/23/2026 09:14	 101 kg
Height	06/23/2026 09:14	5'10"
Pain	06/23/2026 09:14	0/10
BMI	06/23/2026 09:14	31.9

 Active problems

HFrEF

Stable, requires GDMT titration

 Key labs

eGFR	06/09/2026 07:50	75 mL/min/1.73m ²
SCr	06/09/2026 07:50	1.2 mg/dL
NT-proBNP	06/09/2026 07:50	92 pg/mL

The live Thursday EMR shows BP 122/78, HR 70, weight 101 kg, and BMI 31.9.

Answer key evidence: Thursday intended vital signs.

4. Past Medical History

Heart Failure with Reduced Ejection Fraction

Hypertension

Hyperlipidemia

Obesity

5. Medication List

Sacubitril/Valsartan 97/103 mg PO BID

Metoprolol Succinate 100 mg PO daily

Empagliflozin 10 mg PO daily

Spirolactone 25 mg PO daily

Atorvastatin 40 mg PO daily

6. Allergies

Drug Allergies

NKDA

Food Allergies

None

7. Vitals

Today's Vitals

BP: 112/68 mmHg

The official answer key lists a different BP, HR, temperature, and weight state.

Grader evidence: Thursday grading feedback.

- ☐ [1E.2] -0.4 pts: Sacubitril/valsartan dose documented as 24/26 mg instead of expected 97/103 mg.
- ☐ [1E.4] -0.3 pts: Carvedilol is listed instead of metoprolol succinate/XL.
- ☐ [1E.5] -0.3 pts: Incorrect medication (carvedilol) documented instead of metoprolol succinate 100 mg.
- ☐ [1E.6] -0.2 pts: Carvedilol BID documented instead of metoprolol succinate PO daily.
- ☐ [1E.7] -0.3 pts: Dapagliflozin is listed instead of empagliflozin.
- ☐ [1E.8] -0.3 pts: Incorrect medication (dapagliflozin) documented instead of empagliflozin 10 mg.
- ☐ [1E.9] -0.2 pts: Dapagliflozin PO daily documented instead of empagliflozin PO daily.
- ☐ [1E.18] -0.3 pts: COVID vaccination status not mentioned.
- ☐ [1E.19] -0.3 pts: Pneumococcal vaccination status not mentioned.
- ☐ [1E.20] -0.3 pts: Shingrix vaccination status not mentioned.
- ☐ [1E.21] -0.4 pts: Tdap status/timeframe not mentioned.
- ☐ [1G.2] -0.5 pts: Patient's question is framed around dose increase, not discontinuation of medications.
- ☐ [1G.3] -0.4 pts: No link made between patient's question and EF improvement/symptom resolution; EF not discussed.
- ☐ [1G.4] -0.3 pts: Note explicitly states no major recent lifestyle changes, contradicting engagement in lifestyle modifications.
- ☐ [1G.5] -0.4 pts: Note states no structured exercise routine, not ongoing physical activity as a goal.
- ☐ [2A.1] -0.6 pts: Documented BP 122/78 mmHg does not match expected 112/68 mmHg.
- ☐ [2A.2] -0.4 pts: Documented HR 70 bpm does not match expected 62 bpm.
- ☐ [2A.4] -0.3 pts: Documented temp 98.6°F does not match expected 98.1°F.
- ☐ [2A.5] -0.4 pts: Documented SpO2 97% does not match expected 98% on room air.
- ☐ [2A.6] -0.4 pts: Documented weight 101 kg does not match expected 97 kg.
- ☐ [2A.8] -0.5 pts: Documented BMI 31.9 does not match expected 30.7 kg/m².
- ☐ [2A.9] -0.5 pts: Average home BP not documented.
- ☐ [2A.10] -0.3 pts: Average home HR not documented.
- ☐ [2A.11] -1.0 pts: Note states patient does not weigh himself, contradicting stable daily weight monitoring.
- ☐ [2B.1] -0.3 pts: Sodium value not documented.
- ☐ [2B.3] -0.3 pts: BUN value not documented.
- ☐ [2B.6] -0.3 pts: AST value not documented.
- ☐ [2B.7] -0.3 pts: ALT value not documented.
- ☐ [2B.9] -0.5 pts: Previous BNP value not documented.
- ☐ [2B.10] -0.5 pts: Baseline BNP value not documented.
- ☐ [2B.11] -0.7 pts: Current LVEF value not documented; note states EF data unavailable.
- ☐ [2B.12] -0.4 pts: Previous LVEF value not documented.
- ☐ [2B.13] -0.4 pts: Echocardiogram interpretation of improved LV systolic function not documented.
- ☐ [2C.1] -0.5 pts: BMI calculation based on incorrect weight (101 kg) rather than expected 97 kg, so not reconciled correctly.
- ☐ [2C.2] -0.7 pts: BNP trend across baseline, previous, and current values not identified.
- ☐ [2C.3] -0.6 pts: LVEF improvement trend (35% to 45%) not identified.
- ☐ [2C.4] -0.3 pts: States potassium is stable but does not explicitly contextualize in relation to spironolactone/sacubitril-valsartan therapy.
- ☐ [2C.5] -0.6 pts: eGFR value stated but not explicitly linked to supporting continuation of four-drug GDMT.
- ☐ [2C.6] -0.5 pts: Absence of lipid panel not flagged despite ongoing statin therapy.
- ☐ [2C.7] -0.5 pts: Home monitoring values not documented or compared to office vitals.
- ☐ [3A.1] -0.9 pts: HFimpEF diagnosis not made; assessment explicitly states it cannot be confirmed due to missing EF data.

The grader requires the different hidden vital signs and weight.

Issue 4. Daily home monitoring, sodium restriction, and activity required by the grader conflict with the AI patient

What the student saw or heard: Michael states that he does not regularly weigh himself at home, cannot describe day to day weight changes, has not been given a specific sodium or fluid goal, has not made major lifestyle changes, and does not have a structured exercise routine.

What the grader expected: The answer key and grader require stable daily weights, home BP and HR monitoring, active sodium restriction, improved dietary habits, and walking 30 to 40 minutes most days.

Why this is a problem: The student asked about each of these self-management behaviors and documented the responses returned by the AI patient. Adding a daily routine or sodium goal would require inventing data.

Suggested fix: Persist the intended maintenance behaviors in the Thursday AI patient memory and visible chart. If daily weights, sodium restriction, and regular walking are graded, the patient should report them when directly asked.

Evidence: Thursday patient interview: home weight monitoring.

Do you weigh yourself at home? How often, and have your weights been stable or changed recently?

I don't really weigh myself at home regularly, so I don't have a good sense of any day-to-day changes. I just know my weight today at the clinic.

🌟 new info uncovered

Michael states that he does not weigh himself regularly at home.

Evidence: Thursday patient interview: sodium and fluid goals.

What are you doing with salt and fluid intake? Have you been given a daily sodium or fluid goal?

I don't think anyone's given me a specific number for sodium or fluid limits. I try to eat reasonably, but I couldn't tell you an exact goal.

🌟 new info uncovered

Michael states that he has not been given a specific sodium or fluid target.

Evidence: Thursday patient interview: lifestyle change.

12 What lifestyle changes have you made since the last visit, such as cooking at home, eating out less, exercise, or other habits?

I haven't really made any big changes lately - just trying to eat okay and go about my normal routine. Nothing major has changed as far as habits go.

🌟 new info uncovered

Michael reports no major lifestyle change or structured exercise routine.

Answer key evidence: Thursday intended home monitoring and lifestyle state.

HR: 62 bpm
RR: 16
Temp: 98.1°F
Weight: 97 kg
Height: 70 inches
BMI: 30.7 kg/m²
SpO₂: 98% on room air

Home Monitoring Log

Daily weights stable
Average BP:
112–118 / 68–72 mmHg
Average HR:
60–66 bpm
No significant weight fluctuations

8. Laboratory Data

CMP

Na: 137 mEq/L
K: 4.8 mEq/L
BUN: 20 mg/dL
SCr: 1.2 mg/dL
eGFR: 75 mL/min/1.73m²
AST: 23 U/L

The answer key requires daily stable weights and specific home BP and HR values.

Issue 5. Surgical, family, smoking, and alcohol history again conflict with the persistent grader case

What the student saw or heard: The Thursday AI patient again reports no recalled surgery, father with coronary artery disease, heart failure, and myocardial infarction at age 64, mother with hypertension, former smoking without a recalled amount, and rare alcohol limited to special occasions.

What the grader expected: The persistent official case requires appendectomy at age 24, father with hypertension and myocardial infarction at age 62, mother with type 2 diabetes, 20 pack years of smoking, and 1 to 2 alcoholic drinks on weekends. The grader deducts for the live alternate history.

Why this is a problem: These are stable historical facts and should not change by visit. The repeated contradiction confirms that different case versions are persisting across components.

Suggested fix: Use one persistent Patient A history object for all visits. Surgical, family, tobacco, and alcohol history should be loaded from the same source into the AI patient, chart, answer key, and grader.

Evidence: Thursday patient interview: surgery.

Have you ever had any surgeries or procedures? If so, what were they and about when?

I don't recall having any surgeries or procedures - nothing comes to mind.

Michael again denies prior surgery or procedures.

Evidence: Thursday AI patient interview.

 new info uncovered

What medical conditions have you been diagnosed with?

I have heart failure - the doctor's mentioned it's the kind where my heart doesn't pump as strong as it should - and I also have high cholesterol. That's what I'm being treated for.

 new info uncovered

Have you ever had any surgeries or procedures? If so, what were they and about when?

I don't recall having any surgeries or procedures - nothing comes to mind.

Tell me about your family medical history. Has anyone had the same conditions, heart disease, stroke, kidney disease, diabetes, or other major illnesses?

My father had coronary artery disease and heart failure, and he actually had a heart attack when he was 64. My mother has high blood pressure - that's the family history I know of.

 new info uncovered

Please tell me every medication you are taking right now, including the dose, how you take it, and how often.

Sure - I take Entresto, 24/26 milligrams, twice a day. Carvedilol, 3.125 milligrams, twice a day. Spironolactone, 25 milligrams, once a day. Dapagliflozin, 10 milligrams, once a day. And atorvastatin, 40 milligrams, once a day for my cholesterol.

 new info uncovered

How consistently are you taking your medications? About how many doses do you miss, and what usually causes you to miss them?

I take my medications pretty regularly, I don't really miss doses that I can think of. I keep them all set up so I remember to take them on schedule.

 new info uncovered

Do you have any medication or food allergies? What reaction do you have?

No, I don't have any known drug or food allergies.

 new info uncovered

Who do you live with, and what do you do for work or school?

Michael gives the alternate family and medication history.

Evidence: Thursday AI patient interview.

I'm married and live with my wife. I work full-time as an accountant.

🌟 new info uncovered

Do you currently use tobacco or nicotine, or have you in the past? If yes, how much, for how long, and when did you quit if you stopped?

I'm a former smoker - I quit about 5 years ago. I don't really remember the exact amounts from back then.

🌟 new info uncovered

How much alcohol do you drink, and do you use any recreational or illicit drugs?

I rarely drink alcohol, maybe just a glass of wine or beer on special occasions. I don't use any recreational or illicit drugs.

🌟 new info uncovered

What does your usual physical activity look like, and has your condition changed what you are able to do?

I don't really have a set exercise routine, honestly, it's mostly just my regular day-to-day stuff with work. My heart failure hasn't stopped me from doing anything I usually do.

🌟 new info uncovered

What concerns you most right now, and what are your main goals for treatment?

Honestly, my biggest concern is just making sure I stay feeling as good as I do now. My goal is really just to keep things stable and not have any setbacks with my heart.

🌟 new info uncovered

Michael gives the alternate smoking and alcohol history.

Answer key evidence: official persistent history.

Patient A Master Profile

Patient Name

Michael Thompson

Age

58 years old

Sex

Male

Visit Setting

Ambulatory Care Cardiology Clinic

Referral from Primary Care Provider

Main Medical Conditions

- Newly diagnosed Heart Failure with Reduced Ejection Fraction (HFrEF)
- Hypertension
- Hyperlipidemia
- Obesity

Educational Purpose

Patient A serves as the foundational patient for Week 4.

Students will focus on:

- Identifying and classifying heart failure
- Applying heart failure guidelines
- Initiating guideline-directed medical therapy (GDMT)

The Master Profile contains the intended appendectomy and persistent patient history.

Issue 6. Thursday preventive care updates required by the answer key are not exposed in the live immunization panel

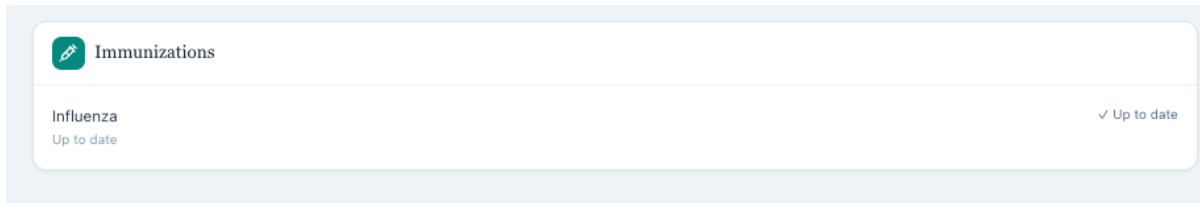
What the student saw or heard: The live Thursday immunization section shows influenza as up to date and does not display the new pneumococcal vaccine or Shingrix dose described in the official follow up case.

What the grader expected: The Thursday answer key states that pneumococcal vaccine was received since the last visit and Shingrix dose number 1 was received. The grader also deducts for missing preventive care items.

Why this is a problem: The student cannot document interval vaccine updates that are not visible in the chart and are not provided by the patient.

Suggested fix: Expose the Thursday preventive care updates in the live immunization panel if they are part of the case. If the panel remains limited to influenza, remove the hidden vaccine requirements from grading.

Evidence: Thursday live immunization panel.



The live Thursday panel displays influenza only.

Answer key evidence: Thursday intended immunization updates.

ALT: 24 U/L

BNP

Current: 92 pg/mL

Previous: 165 pg/mL

Previous Baseline: 320 pg/mL

Repeat Echocardiogram

LVEF: 45%

Previous: 35%

Interpretation:

Improved systolic function

9. Immunizations

Pneumococcal vaccine received since last visit.

Shingrix dose #1 received.

10. Social History

Former smoker

Alcohol:

1–2 drinks on weekends

Exercise:

Walking 30–40 minutes most days

Diet:

The official answer key lists pneumococcal vaccination and Shingrix dose number 1 since the previous visit.

Issue 7. The same Thursday encounter is labeled both a 6 Month Follow Up and an Initial Ambulatory Care Pharmacist Visit

What the student saw or heard: The encounter header identifies Thursday as a 6 Month Follow Up. Inside the Subjective section, the documented history metadata says Initial Ambulatory Care Pharmacist Visit.

What the grader expected: The official answer key defines the encounter as the scheduled 6 month follow up and maintenance phase assessment.

Why this is a problem: The visit stage determines how the student interprets treatment history, longitudinal response, medication titration, and expected follow up data. Two stage labels within the same encounter are internally contradictory.

Suggested fix: Change the Subjective metadata to 6 Month Follow Up or clearly label it as historical information from an earlier visit if that is the intended design.

Evidence: Thursday encounter header.

<

MT

Michael Thompson

58 yo male · White · MRN W4-51004 · DOB 03/12/1972

PCP: Dr. Johnson · Pharmacy: Walgreens · Language: English · Allergies: No known drug allergies (NKDA)

Thursday - 6-Month Follow-up
Ambulatory care clinic

Reset

Exit

CODE STATUS	ISOLATION	COVERAGE	PHARMACIST OF RECORD	CARE MANAGER	LAST SEEN	NEXT APPT
Full Code	None	Aetna — Choice POS II (PPO)	R. Patel, PharmD (Ambulatory Care)	S. Nguyen, RN (Chronic Care Mgmt)	03/18/2026	06/30/2026 10:30

FLAGS

Fall risk

Advance directive

FYI

Case Progress: 67%

The live header identifies Thursday as a 6 Month Follow Up.

Evidence: Thursday Subjective metadata.

Subjective

Documented history plus the findings you gather through the interview

Some subjective information is intentionally missing. Interview the patient to complete this section, then document what you learn below.

Documented (from the chart)

Documented by: Intake RN · 06/23/2026 08:50 · Initial Ambulatory Care Pharmacist Visit

Chief Concern

"I feel great. Do I still need to increase the doses?"

HPI

Dizziness resolved after stopping HCTZ. No HF symptoms. Feels great.

The same encounter labels the documented history as an Initial Ambulatory Care Pharmacist Visit.

Issue 8. Six month follow up vitals and laboratory results continue to reuse baseline timestamps

What the student saw or heard: The Thursday encounter is labeled a 6 month follow up but the vital signs again use 06/23/2026 at 09:14 and the laboratory results again use 06/09/2026 at 07:50, the same timestamps used on earlier encounters despite different values.

What the grader expected: The official answer key presents Thursday as a later maintenance visit with new laboratory and clinical trend data.

Why this is a problem: One recorded time and one laboratory specimen cannot produce multiple different result sets across months. The unchanged timestamps break the longitudinal record and make trend interpretation unreliable.

Suggested fix: Assign encounter specific dates and laboratory collection times for the 6-month visit. All trend logic should use those actual dated results.

Evidence: longitudinal vital sign timestamps.

Tuesday initial visit

Vitals				
Vital signs recorded at this visit				
VITAL	VALUE	REFERENCE	TREND	FLAG
Office BP 06/23/2026 09:14	138/86	<130/80 mmHg		
Repeat BP 06/23/2026 09:14	136/84	<130/80 mmHg	—	
Heart rate 06/23/2026 09:14	82	60–100 bpm	—	
Respiratory rate 06/23/2026 09:14	16	12–20 /min	—	
Temperature 06/23/2026 09:14	98.6°F	97.0–99.5 °F	—	
SpO ₂ 06/23/2026 09:14	96%	95–100 %	—	
Weight 06/23/2026 09:14	102 kg	—		
Height 06/23/2026 09:14	5'10"	—	—	
BMI 06/23/2026 09:14	32.2	18.5–24.9 kg/m²	—	
BSA (Mosteller) 06/23/2026 09:14	2.04 m²	—	—	

Wednesday 3 month follow up

Vitals				
Vital signs recorded at this visit				
VITAL	VALUE	REFERENCE	TREND	FLAG
Office BP 06/23/2026 09:14	106/68	<130/80 mmHg		
Repeat BP 06/23/2026 09:14	104/66	<130/80 mmHg	—	
Heart rate 06/23/2026 09:14	62	60–100 bpm	—	
Respiratory rate 06/23/2026 09:14	16	12–20 /min	—	
Temperature 06/23/2026 09:14	98.6°F	97.0–99.5 °F	—	
SpO ₂ 06/23/2026 09:14	97%	95–100 %	—	
Weight 06/23/2026 09:14	101 kg	—		
Height 06/23/2026 09:14	5'10"	—	—	
BMI 06/23/2026 09:14	31.9	18.5–24.9 kg/m²	—	
BSA (Mosteller) 06/23/2026 09:14	2.04 m²	—	—	

Thursday 6 month follow up

Best Practice Advisories SYSTEM-GENERATED ADVISORIES — REVIEW AND ACT	Active problems HFrEF Stable, requires GDMT titration
Key vitals Office BP 06/23/2026 09:14 122/78 Repeat BP 06/23/2026 09:14 120/76 HR 06/23/2026 09:14 70 bpm Temp 06/23/2026 09:14 98.6°F RR 16	Key labs eGFR 75 mL/min/1.73m² Ref — · 06/09/2026 07:50 SCr 1.2 mg/dL Ref — · 06/09/2026 07:50 NT-proBNP 92 pg/mL Ref — · 06/09/2026 07:50

All three visits use the same vital sign timestamp despite different values.

Evidence: longitudinal laboratory timestamps.

Tuesday: NT proBNP 320 pg/mL

All Labs		Collected: 06/09/2026 07:50	
TEST	RESULT	REFERENCE RANGE	FLAG
NT-proBNP	320 pg/mL	—	• High
K	4.2 mEq/L	—	
SCr	1.0 mg/dL	—	
eGFR	88 mL/min/1.73m ²	—	

Wednesday: NT proBNP 165 pg/mL

Labs

Results are organized into panels — open the one you need

Lab panel

Cardiac Biomarkers

Cardiac Biomarkers

Collected: 06/09/2026 07:50

TEST	RESULT	REFERENCE RANGE	FLAG
NT-proBNP	165 pg/mL	—	High

Thursday: NT proBNP 92 pg/mL

Labs

Results are organized into panels — open the one you need

▼

Lab panel

Cardiac Biomarkers

▼

A

Cardiac Biomarkers

Collected: 06/09/2026 07:50

TEST	RESULT	REFERENCE RANGE	FLAG
NT-proBNP	92 pg/mL	—	

All three visits use the same laboratory collection timestamp despite changing results.

Patient An Overall Pattern

Summary for developer handoff

The official answer key itself presents a coherent intended progression from newly diagnosed HFrEF on Tuesday, to partial GDMT with MRA still missing on Wednesday, to HFimpEF on optimized long-term therapy on Thursday. The live VACS components do not consistently render or grade that progression. Tuesday and Thursday graders largely follow the official answer key while the live EMR and AI patient use a different patient state. Wednesday is different because the live EMR, AI patient, and grader largely agree with one another but use an alternate state that conflicts with the official Wednesday answer key.

Suggested system level fix: Create one canonical longitudinal Patient A state for each encounter and require the EMR renderer, AI patient memory, teaching prompts, answer key, and grader to load that same version. Add automated comparison tests for medications, symptoms, vitals, labs, problem list, preventive care, historical data, and encounter timestamps before release.

Source Materials Reviewed

- Official Patient A Master Profile and official Tuesday, Wednesday, and Thursday answer keys from the uploaded answer key ZIP.
- Tuesday live EMR screenshots, full AI patient interview, SOAP source, and AI grader feedback.
- Wednesday live EMR screenshots, full AI patient interview, SOAP source, and AI grader feedback.
- Thursday live EMR screenshots, full AI patient interview, SOAP source, and AI grader feedback.
- Only confirmed conflicts or unavailable information problems are counted as issues. Routine SOAP writing opportunities are not counted as VACS defects.

WEEK 4

VACS AI Patient & Grading Issue Report

Patient B: Angela Brooks

Clinical and Operational Audit

Purpose

This report lists confirmed or strongly supported VACS system, data synchronization, and grading conflicts for Patient B, Angela Brooks. It separates software or data problems from ordinary SOAP documentation opportunities. The student facing EMR, AI patient interview, rubric or grader reference, and grader feedback are treated as separate evidence sources.

Quick Guide for Non-Clinical Readers

Term	Plain language meaning
AI patient	The simulated patient the student interviews.
EMR	The electronic patient chart shown to the student.
Grader / rubric	The AI scoring system and the facts it expects.
AFib	Atrial fibrillation, an irregular heart rhythm.
HFrEF	Heart failure with reduced ejection fraction.
LVEF	The percentage used to describe how strongly the left ventricle pumps.
DOAC	A direct oral anticoagulant, such as apixaban.
CBC	Complete blood count.
NT proBNP	A blood marker used in heart failure assessment.

Current Draft Review Summary

This working draft contains 26 grouped system and data findings across Angela Brooks' three Week 4 encounters. Repeated conflicts are retained under each visit when they can independently affect the student experience or grading. Ordinary SOAP writing improvements are not counted as system defects.

Encounter	Grouped issues in current draft
Tuesday Initial Visit	6
Wednesday 3 Month Follow Up	10
Thursday 6 Month Follow Up	10

Draft note: the Thursday section can still be updated when the final screenshot set is supplied. The current report is intentionally built from evidence already provided.

Patient B – Angela Brooks – Tuesday Initial Visit

6 grouped system or data issues are included in this working draft for this visit. Items are grouped so one underlying problem is not counted multiple times only because it produced several rubric deductions.

#	Issue	Problem type
1	Patient age conflicts with the displayed date of birth	Internal EMR inconsistency
2	The AI patient and grader use different adherence, cost, palpitation, and medication understanding histories	AI patient and grader disagree
3	Past medical, surgical, and family history returned by the AI patient does not match the grader reference	AI patient and grader disagree
4	Atorvastatin 40 mg daily is required by the grader but is absent from the live medication list and patient reconciliation	EMR and AI patient vs grader
5	The grader requires clinical values and examination findings that are not visible in the student facing EMR	Required information is unavailable
6	Vaccine statuses expected by the grader are not supported by the AI patient or visible EMR	Required information is unavailable

Issue 1. Patient age conflicts with the displayed date of birth

What the student saw or heard: The Tuesday header identifies Angela as 64 years old while also displaying a date of birth of 03/12/1972. The encounter data are dated June 2026.

What the grader or rubric expected: The rubric consistently grades Angela as a 64-year-old woman.

Why this is a problem: A March 1972 date of birth corresponds to age 54 in June 2026, not age 64. The chart therefore contains two incompatible demographic facts. Age can affect clinical risk calculations and medication decisions, so the student should not have to decide which demographic field is authoritative.

Suggested fix: Correct either the age or date of birth and use the same demographic value across every Week 4 encounter and grader reference.

Angela Brooks

64 yo female · Black · MRN W4-22091 · DOB 03/12/1972

PCP: Dr. Johnson · Pharmacy: Walgreens · Language: English · Allergies: No known drug allergies (NKDA)

Tuesday · Initial Ambulatory Care Visit

Ambulatory care clinic

ResetExit

CODE STATUS

Full Code

ISOLATION

None

COVERAGE

Aetna — Choice POS II (PPO)

PHARMACIST OF RECORD

R. Patel, PharmD (Ambulatory Care)

CARE MANAGER

S. Nguyen, RN (Chronic Care Mgmt)

LAST SEEN

03/18/2026

NEXT APPT

06/30/2026 10:30

FLAGS

Fall riskAdvance directiveFYI

Best Practice Advisories

SYSTEM-GENERATED ADVISORIES — REVIEW AND ACT

ALERTOverdue

Evidence – Tuesday EMR header displays both age 64 and DOB 03/12/1972.

Issue 2. The AI patient and grader use different adherence, cost, palpitation, and medication understanding histories

What the student saw or heard: Angela reports that Eliquis costs about \$50 per month, she tried to stretch the supply, then ran out and stopped it completely. She says the other heart failure medicines are taken pretty regularly. She reports occasional fluttering or an irregular beat and clearly states that Eliquis is intended to reduce stroke risk from AFib.

What the grader or rubric expected: The grader expects apixaban and sacubitril/valsartan doses to be missed about 2 to 3 times per week, expects both medications to be expensive, expects denial of significant palpitations, and expects a knowledge gap about apixaban and stroke prevention.

Why this is a problem: The student is penalized for preserving the interview. Complete anticoagulant discontinuation is clinically different from intermittent missed doses, and documented understanding of stroke prevention is the opposite of the expected knowledge deficit.

Suggested fix: Choose one Tuesday history and synchronize the AI patient, EMR, and grader. If the grader version is intended, the AI patient must return those answers when directly asked.

**Angela Brooks**
Standardized patient - Initial Ambulatory Care Visit

Are you taking a blood thinner or anticoagulant? Which one, how do you take it, and have you missed any doses?
Are you taking a blood thinner or anticoagulant? Which one, how do you take it, and have you missed any doses?

That Eliquis is way too expensive. I'm on a fixed income, I can't pay \$50 a month for it. I was trying to stretch it out by skipping doses around refill time, but I just ran out and stopped taking it completely.
 new info uncovered

Have you had any falls, head injuries, or near-falls since the last visit?

No, I haven't had any falls or hit my head or anything like that. Nothing like that's happened.
 new info uncovered

What symptoms do you notice from your atrial fibrillation, and what do you understand about why it is being treated?

Sometimes I feel my heart flutter or beat a little irregular, but nothing too dramatic. I know they want to treat it to lower my risk of having a stroke, since that irregular heartbeat can cause clots.
 new info uncovered

How are you taking your medications? Do you ever miss doses? Do you take anything over the counter?

What are your health goals?

Session expired 

Evidence – Tuesday interview shows Eliquis cost related discontinuation and the patient stating that AFib treatment lowers stroke risk.

Issue 3. Past medical, surgical, and family history returned by the AI patient does not match the grader reference

What the student saw or heard: Angela names atrial fibrillation and heart failure as her known conditions, denies any prior surgeries or procedures, reports that her mother had AFib and a stroke at age 74, and reports that her father had hypertension and type 2 diabetes.

What the grader or rubric expected: The grader expects hypertension, hyperlipidemia, persistent nonvalvular AFib, LVEF 35%, prior cholecystectomy, father with ischemic stroke at age 70, mother with heart failure, and sister with hypertension.

Why this is a problem: These are not small wording differences. The AI patient gives a different medical, surgical, and family history. The student cannot safely insert hidden diagnoses, surgery, or relatives that the patient directly denies or replaces with different information.

Suggested fix: Synchronize the patient history and grader source of truth. Required diagnoses, surgery, LVEF, and family history should be visible in the EMR or reliably returned by the patient.

 **Angela Brooks**
Standardized patient · Initial Ambulatory Care Visit

What medical conditions have you been diagnosed with?

I've got that irregular heartbeat, the atrial fibrillation, and I also have heart failure - they told me my heart doesn't pump as strong as it should. That's really it as far as I know.
 new info uncovered

Have you ever had any surgeries or procedures? If so, what were they and about when?

No, I haven't had any surgeries or procedures that I can think of. Nothing like that's happened to me.

Tell me about your family medical history. Has anyone had the same conditions, heart disease, stroke, kidney disease, diabetes, or other major illnesses?

My mother had that same irregular heartbeat, the atrial fibrillation, and she ended up having a stroke from it when she was 74. My father had high blood pressure and diabetes, the type 2 kind.
 new info uncovered

Please tell me every medication you are taking right now, including the dose,

How are you taking your medications? Do you ever miss doses? Do you take anything over the counter?

What are your health goals?

Session expired 

Evidence – Tuesday AI patient lists AFib and heart failure, denies surgery, and gives a family history different from the grader reference.

Issue 4. Atorvastatin 40 mg daily is required by the grader but is absent from the live medication list and patient reconciliation

What the student saw or heard: The live Tuesday medication list contains sacubitril/valsartan 49/51 mg twice daily, metoprolol succinate 50 mg daily, empagliflozin 10 mg daily, and apixaban 5 mg twice daily. Angela names the same four medicines during medication reconciliation.

What the grader or rubric expected: The grader requires atorvastatin 40 mg by mouth daily and removes points when the drug, dose, and regimen are not documented.

Why this is a problem: A student should not invent an active medication that is absent from both the chart and the patient interview. Hidden medication data can change medication reconciliation, disease assessment, and safety review.

Suggested fix: Either add atorvastatin 40 mg daily to the Tuesday EMR and AI patient consistently or remove it from Tuesday grading requirements.



Angela Brooks

Standardized patient · Initial Ambulatory Care Visit

Please tell me every medication you are taking right now, including the dose, how you take it, and how often.

Let's see... I take the Sacubitril/Valsartan, that's 49/51 milligrams, twice a day. Then there's the Metoprolol, 50 milligrams once a day, and the Empagliflozin, 10 milligrams once a day. And then there's the Eliquis, which is supposed to be 5 milligrams twice a day, but like I told you, I ran out and haven't been taking it.

🌟 new info uncovered

How consistently are you taking your medications? About how many doses do you miss, and what usually causes you to miss them?

For my other medicines, the heart failure ones, I take those pretty regularly, don't miss much there. It's really just been the Eliquis that I've been missing, and that's because of the cost, I couldn't keep up with paying for it.

🌟 new info uncovered

Evidence – Tuesday medication reconciliation names the same four medications shown in the chart and does not include atorvastatin.

Issue 5. The grader requires clinical values and examination findings that are not visible in the student facing EMR

What the student saw or heard: The visible Tuesday EMR provides vital signs, renal and hepatic labs, NT proBNP, WBC, persistent AFib, and HFrEF. It does not display LVEF 35%, echocardiogram interval change, an irregularly irregular rhythm finding, hemoglobin, platelets, a room air qualifier, a cardiology specialty label, or the nonvalvular AFib qualifier.

What the grader or rubric expected: The grader expects those exact objective findings and descriptors, including LVEF 35%, rhythm characterization, hemoglobin, platelet count, and a component level stroke risk interpretation that relies on hypertension.

Why this is a problem: Measured clinical values and examination findings cannot be safely documented when they are not shown. Hidden objective data create a grading mismatch and may alter medication and risk decisions.

Suggested fix: Expose the objective values and qualifiers in the live EMR or remove them from the Tuesday rubric. The problem list and stroke risk components should also use the same diagnoses.

- Height

06/23/2026 09:14

5'5"
- Pain

06/23/2026 09:14

0/10
- BMI

06/23/2026 09:14

30.8

Key labs

- eGFR

Ref -- : 06/09/2026 07:50

72 mL/min/1.73m²
- SCr

Ref -- : 06/09/2026 07:50

1.0 mg/dL
- NT-proBNP ▲

Ref -- : 06/09/2026 07:50

210 pg/mLHigh

Current medications

Med rec: Pending

Last reconciled 03/18/2026 - R. Patel, PharmD

ACTIVE / PRESCRIBED (EHR)

MEDICATION	DOSE	ROUTE	FREQ	START	PRESCRIBER	LA
Sacubitril/Valsartan	49/51 mg	by mouth	BID			
Metoprolol Succinate	50 mg	by mouth	daily			
Empagliflozin	10 mg	by mouth	daily			
Apixaban	5 mg	by mouth	BID			

Evidence – Tuesday EMR displays the visible medication and laboratory content but does not provide the hidden echocardiogram and blood count details expected by the grader.

Issue 6. Vaccine statuses expected by the grader are not supported by the AI patient or visible EMR

What the student saw or heard: Angela says she is not sure which vaccines she has received recently. The visible EMR confirms influenza as up to date but does not provide the complete set of vaccine statuses expected by the grader.

What the grader or rubric expected: The grader expects COVID vaccination up to date, pneumococcal not documented, Shingrix not documented, and Tdap overdue, then grades assessment and plan items based on those exact statuses.

Why this is a problem: The student should preserve patient uncertainty and use available vaccine records. Exact preventive care facts should not be fabricated to satisfy hidden rubric data.

Suggested fix: Display the intended vaccine statuses in the EMR and keep the patient response and grader consistent. Otherwise, the grader should accept uncertainty when the required data are not exposed.

Do you know whether you are up to date on the vaccines recommended for you?
Which vaccines do you remember receiving recently?

Hmm, I'm not really sure about that. I don't recall exactly which ones I've gotten recently, that's not something I keep track of too well.

✦ new info uncovered

Evidence – Tuesday AI patient states that she cannot recall which vaccines she has received recently.

Patient B – Angela Brooks – Wednesday 3 Month Follow Up

10 grouped system or data issues are included in this working draft for this visit. Items are grouped so one underlying problem is not counted multiple times only because it produced several rubric deductions.

#	Issue	Problem type
1	Patient age conflicts with the displayed date of birth	Internal EMR inconsistency
2	The 3 month follow up is labeled as an initial visit and reuses the same visit date as the Tuesday encounter	Longitudinal timeline inconsistency
3	Heart failure symptom details returned by the AI patient do not match the rubric required exertional dyspnea pattern	AI patient and rubric disagree
4	Past medical, surgical, and family history do not match the Wednesday rubric reference	AI patient and rubric disagree
5	The live problem list does not expose the full set of diagnoses and qualifiers required by the rubric	EMR and rubric disagree
6	Atorvastatin is required by the Wednesday rubric but absent from the live medication list and patient reconciliation	EMR and AI patient vs rubric
7	Objective values expected for grading are not visible in the Wednesday EMR	Required information is unavailable
8	Vaccination statuses beyond influenza are required but unavailable from the patient and visible chart	Required information is unavailable
9	Fixed income is required as Wednesday social history but is not returned when access barriers are reassessed	Required information is unavailable
10	Laboratory values change across encounters despite the same collection date and time	Cross visit data inconsistency

Issue 1. Patient age conflicts with the displayed date of birth

What the student saw or heard: The Wednesday header again identifies Angela as 64 years old and shows DOB 03/12/1972.

What the grader or rubric expected: The Wednesday rubric grades the patient as age 64.

Why this is a problem: The demographic fields remain internally incompatible. In June 2026, the displayed date of birth corresponds to age 54.

Suggested fix: Correct the demographic source of truth and propagate the correction to the header, patient model, and grading rubric.

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AB

Angela Brooks

64 yo female · Black · MRN W4-22091 · DOB 03/12/1972

PCP: Dr. Johnson · Pharmacy: Walgreens · Language: English · Allergies: No known drug allergies (NKDA)

Wednesday · 3-Month Follow-up

Reset

Exit

Ambulatory care clinic

CODE STATUS	ISOLATION	COVERAGE	PHARMACIST OF RECORD	CARE MANAGER	LAST SEEN	NEXT APPT
Full Code	None	Aetna — Choice POS II (PPO)	R. Patel, PharmD (Ambulatory Care)	S. Nguyen, RN (Chronic Care Mgmt)	03/18/2026	06/30/2026

FLAGS

Fall risk

Advance directive

FYI

Case Progress: 33%

Best Practice Advisories

SYSTEM-GENERATED ADVISORIES — REVIEW AND ACT

Active problems

AFib

Cost resolved, fear of bleeding

Key vitals

Office BP

06/23/2026 09:14

126/76

Repeat BP

06/23/2026 09:14

124/74

HR

06/23/2026 09:14

84 bpm

Temp

06/23/2026 09:14

98.1°F

RR

06/23/2026 09:14

16

SpO₂

06/23/2026 09:14

98%

Weight

06/23/2026 09:14

84 kg

Height

06/23/2026 09:14

5'5"

Pain

06/23/2026 09:14

0/10

BMI

06/23/2026 09:14

30.8

Evidence – Wednesday header repeats age 64 with DOB 03/12/1972.

Issue 2. The 3 month follow up is labeled as an initial visit and reuses the same visit date as the Tuesday encounter

What the student saw or heard: The Wednesday header identifies a 3 month follow up, but the Subjective documentation line calls the encounter an Initial Ambulatory Care Pharmacist Visit. Vitals are dated 06/23/2026, the same date used for the Tuesday initial visit.

What the grader or rubric expected: The rubric treats Wednesday as a distinct follow up encounter occurring after the initial visit and evaluates longitudinal improvement.

Why this is a problem: A longitudinal follow up cannot reliably represent three months of change if the encounter is internally labeled as initial and shares the same visit date as the baseline encounter. This can confuse both students and logic that depends on prior versus current data.

Suggested fix: Use a consistent follow up label and advance the encounter date and longitudinal data to a true follow up time point.

Key labs

eGFR

Ref — · 06/09/2026 07:50

73 mL/min/1.73m²

SCr

Ref — · 06/09/2026 07:50

1.0 mg/dL

NT-proBNP▲

Ref — · 06/09/2026 07:50

210 pg/mL^{High}

Current medications

Med rec: Pending

Last reconciled 03/18/2026 · R. Patel, PharmD

ACTIVE / PRESCRIBED (EHR)

MEDICATION	DOSE	ROUTE	FRE Q	STAR T	PRESCRIBER	LAST FILLED	REFILLS
Sacubitril/Valsartan	49/51 mg	by mouth	BID				
Metoprolol Succinate	50 mg	by mouth	dail y				
Empagliflozin	10 mg	by mouth	dail y				
Apixaban	5 mg	by mouth	BID				

Subjective

Documented history plus the findings you gather through the interview

Some subjective information is intentionally missing. **Interview the patient** to complete this section, then document what you learn below.

Documented (from the chart)

Documented by: Intake RN · 06/23/2026 08:50 · Initial Ambulatory Care Pharmacist Visit

Chief Concern

"I started the Eliquis, but I stopped because I got bruises and got scared."

HPI

Patient obtained Apixaban via patient assistance program but stopped taking it due to bruising and fear of major bleeding after a friend's anecdote.

Objective

Objective data: vitals, labs, medications, allergies, immunizations

Vitals

Recorded 06/23/2026 09:14

BP

Ref <130/80 mmHg

126/76

Evidence – Wednesday Subjective section says Initial Ambulatory Care Pharmacist Visit while the encounter header identifies a 3 month follow up.

Issue 3. Heart failure symptom details returned by the AI patient do not match the rubric required exertional dyspnea pattern

What the student saw or heard: Angela says her heart failure symptoms have not changed, she does not recall new shortness of breath, her energy is about the same, and she can perform her usual activities without feeling more short of breath or tired than before.

What the grader or rubric expected: The Wednesday rubric specifically expects stable heart failure with mild fatigue and exertional shortness of breath, along with denial of worsening dyspnea.

Why this is a problem: The patient supports stability and denial of worsening but does not clearly provide the positive exertional dyspnea finding the rubric requires. A student should not convert a negative or nonspecific symptom history into a hidden positive symptom.

Suggested fix: Program the AI patient to consistently report the intended stable exertional dyspnea pattern when asked about energy and activity or relax the rubric to accept the actual interview response.

Angela Brooks

Standardized patient · 3-Month Follow-up

What brings you in today, and what has changed since your last visit or since this problem started?

Well, I finally got approved for that patient assistance program so I could get my blood thinner, the apixaban, but honestly I stopped taking it a little while ago. I got some bruises on my arms and legs that scared me, and then a friend told me a scary story about someone bleeding real bad on this same medicine, so I just got too nervous to keep taking it.

✧ new info uncovered

How have your heart-failure symptoms changed, and when did they first start or begin getting worse?

Honestly, I haven't really noticed my heart failure symptoms changing much lately - I don't recall any new shortness of breath or swelling or anything like that. I've mostly been focused on this whole blood thinner situation and the bruising that scared me.

✧ new info uncovered

How has your energy been? When do you feel fatigued, and what brings it on?

My energy's been about the same as usual, I think. I don't really notice feeling more tired than normal or anything that brings it on - nothing's really changed there that I've noticed.

✧ new info uncovered

What activities make you short of breath or tired now, and what activities have become easier or harder?

I don't think anything's really changed for me there. I can still do my usual stuff around the house and getting around without feeling more short of breath or tired than before.

✧ new info uncovered

Have you had swelling in your ankles, legs, or abdomen? When is it worst?

No, I haven't noticed any swelling in my ankles, legs, or belly. Nothing like that's been bothering me.

Do you get short of breath when lying flat, need extra pillows, or wake up at night short of breath?

Evidence – Wednesday interview shows stable symptoms without a clear positive exertional shortness of breath report.

Issue 4. Past medical, surgical, and family history do not match the Wednesday rubric reference

What the student saw or heard: Angela reports AFib and heart failure, says she does not recall any other major diagnoses, denies surgeries or procedures, reports maternal AFib with stroke at age 74, and reports paternal hypertension and type 2 diabetes.

What the grader or rubric expected: The Wednesday rubric expects HFrEF with LVEF 35%, persistent nonvalvular AFib, hypertension, hyperlipidemia, cholecystectomy, father with ischemic stroke at age 70, mother with heart failure, and sister with hypertension.

Why this is a problem: The follow up AI patient continues to carry a different history from the rubric. The student directly asked the required questions and should not substitute hidden diagnoses, surgery, or family history.

Suggested fix: Persist one medical, surgical, and family history across the longitudinal case and make it available in both the AI patient and follow up EMR.

What symptoms do you notice from your atrial fibrillation, and what do you understand about why it is being treated?

Honestly, I don't really notice any symptoms from the AFib itself, like I said I don't feel my heart racing or anything unusual. I know it's something to do with my heart's rhythm and that it can raise my risk of stroke, which is why I was told to take that blood thinner, but I stopped because of the bruising and being scared.

✧ new info uncovered

How long have you had each of your major medical conditions, and when were they diagnosed?

Honestly, I don't remember the exact dates for everything. I know I've had the AFib and heart issues for a while now, but I couldn't tell you exactly when each one was first diagnosed.

✧ new info uncovered

What medical conditions have you been diagnosed with?

I have atrial fibrillation, that's the irregular heartbeat thing, and I also have heart failure - that's why I take those heart medications. I don't recall having anything else major diagnosed.

✧ new info uncovered

Have you ever had any surgeries or procedures? If so, what were they and about when?

I don't recall having any surgeries or procedures done. Nothing comes to mind.

Tell me about your family medical history. Has anyone had the same conditions, heart disease, stroke, kidney disease, diabetes, or other major illnesses?

My mother had atrial fibrillation and actually had a stroke because of it when she was 74. My father had high blood pressure and type 2 diabetes.

✧ new info uncovered

Please tell me every medication you are taking right now, including the dose, how you take it, and how often.

Let's see... I take the sacubitril/valsartan, that's 49/51 milligrams, twice a day. I also take metoprolol succinate, 50 milligrams once a day, and empagliflozin, 10 milligrams once a day. Then there's the apixaban, 5 milligrams twice a day, but like I said, I stopped taking that one recently because of the bruising and being scared.

Evidence – Wednesday AI patient denies surgery and supplies a family history that differs from the rubric reference.

Issue 5. The live problem list does not expose the full set of diagnoses and qualifiers required by the rubric

What the student saw or heard: The Wednesday active problem area shows AFib with the note Cost resolved, fear of bleeding. It does not visibly list HFrEF, hypertension, hyperlipidemia, LVEF 35%, or the persistent nonvalvular AFib qualifiers.

What the grader or rubric expected: The rubric expects all of those diagnoses and qualifiers to be incorporated into the SOAP note and subsequent risk assessment.

Why this is a problem: A follow up rubric should not require hidden problem list content. The absence of visible diagnoses also makes downstream stroke risk and comorbidity reasoning harder to verify.

Suggested fix: Populate the Wednesday problem list with the full active diagnoses and qualifiers used by the rubric or remove hidden diagnoses from grading.

Cardiac Biomarkers

Collected: 06/09/2026 07:50

TEST	RESULT	REFERENCE RANGE	FLAG
NT-proBNP	210 pg/mL	—	High

Active problems on the chart

Problem list reviewed 06/23/2026

AFib

Cost resolved, fear of bleeding

Noted Managed by Dr. Johnson

Active

Guiding Questions

Questions to scaffold your reasoning — they guide without giving away the answer

1

Why is minor bruising managed differently than major bleeding?

2

Why is continuation of apixaban appropriate?

3

What factors contributed to nonadherence?

4

What counseling points should be provided?

5

How does shared decision-making improve outcomes?

6

Counseling

Plain-language scripts you can adapt when counseling the patient

Starting DOACs

"This blood thinner doesn't require routine blood checks like warfarin, but you must take it exactly as prescribed. Missing doses leaves you unprotected."

Evidence – Wednesday active problem list displays AFib only and does not show the full rubric required problem set.

Issue 6. Atorvastatin is required by the Wednesday rubric but absent from the live medication list and patient reconciliation

What the student saw or heard: The Wednesday live medication list contains sacubitril/valsartan, metoprolol succinate, empagliflozin, and apixaban. Angela names the same medications during the interview.

What the grader or rubric expected: The Wednesday rubric requires atorvastatin 40 mg by mouth daily.

Why this is a problem: The student cannot safely document or assess a statin that is not displayed and not reported by the patient.

Suggested fix: Add atorvastatin to the visible medication list and patient model if it is intended therapy or remove the medication from Wednesday grading.

210 pg/mL^{High}
 COMPLETE BLOOD COUNT
 Collected 06/09/2026 07:50

WBC

Ref —

6.5 x10³/mm³

Immunizations

Influenza

Up to date

✓ Up to date

Medications

Documented medication list (as recorded in the chart)

Medications

Current / Home meds (4)Discontinued (0)

✓ Allergy/interaction check active · NKDA — no conflicts

PRESCRIBED / ACTIVE MEDICATIONS

MEDICATION	STAT US	DOS E	ROUT E	FRE Q	INDICATI ON	STA RT	PRESCRIB ER	LAST FILL ED	REFIL LS	PHARM ACY
Sacubitril/ Valsartan✓ Sig:	Activ e	49/5 1 mg	by mou th	BID	HFrEF			Qty		
Metoprolol Succinate✓ Sig:	Activ e	50 mg	by mou th	dai ly	HFrEF/ Rate control			Qty		
Empagliflozin✓ Sig:	Activ e	10 mg	by mou th	dai ly	HFrEF			Qty		
Apixaban✓ Sig:	Activ e	5 mg	by mou th	BID	AFib stroke preventi on			Qty		

Medication reconciliation prompts

- Ask about OTC medications (e.g., pain relievers, supplements)
- Ask about any side effects or new symptoms
- Ask about missed doses and the reasons behind them
- Ask about cost or refill barriers

Missing →The visible list may be incomplete — patients often omit OTC products. Confirm in the interview.

Allergies

Documented allergies and reactions

Drug allergies

SUBSTANCE	REACTION	SEVERITY	REACTION TYPE	DATE NOTED	SOURCE
					/

Evidence – Wednesday live medication table shows four medications and no atorvastatin.

Issue 7. Objective values expected for grading are not visible in the Wednesday EMR

What the student saw or heard: The Wednesday EMR shows standard vital signs, metabolic and renal results, NT proBNP, and a CBC panel with WBC only. It does not display hemoglobin, platelets, LVEF 35%, echocardiogram interval change, or an irregularly irregular rhythm examination finding.

What the grader or rubric expected: The rubric uses these objective values and descriptors in full credit criteria for HFrEF, AFib, anticoagulation safety, and clinical interpretation.

Why this is a problem: The student should not be required to invent an echocardiogram value, blood count results, or physical examination findings that are not student facing.

Suggested fix: Expose all objective data used by the rubric in the Wednesday EMR or remove unavailable values from scoring.

Comprehensive Metabolic Panel (CMP)
Renal / Kidney Function
Lipid Panel
Cardiac Biomarkers
Pulmonary Studies
Complete Blood Count & Immunology
Urine Studies
Missing / Needed Labs

Comprehensive Metabolic Panel (CMP)

Collected: 06/09/2026 07:50

TEST	RESULT	REFERENCE RANGE	FLAG
Na	139 mEq/L	—	
K	4.3 mEq/L	—	
BUN	17 mg/dL	—	
SCr	1.0 mg/dL	—	
eGFR	73 mL/min/1.73m ²	—	

Renal / Kidney Function

Collected: 06/09/2026 07:50

TEST	RESULT	REFERENCE RANGE	FLAG
BUN	17 mg/dL	—	
SCr	1.0 mg/dL	—	
eGFR	73 mL/min/1.73m ²	—	
K	4.3 mEq/L		

Complete Blood Count & Immunology

Collected: 06/09/2026 07:50

TEST	RESULT	REFERENCE RANGE	FLAG
WBC	6.5 x10 ³ /mm ³	—	

Evidence – Wednesday lab panels show WBC as the only CBC value and do not display the hidden echocardiogram or blood count values.

Issue 8. Vaccination statuses beyond influenza are required but unavailable from the patient and visible chart

What the student saw or heard: Angela states that she cannot remember whether she is up to date. The Wednesday EMR provides influenza status but does not clearly expose the complete COVID, pneumococcal, Shingrix, and Tdap statuses used by the rubric.

What the grader or rubric expected: The rubric requires influenza and COVID to be current, pneumococcal and Shingrix to be not documented, and Tdap to be overdue.

Why this is a problem: The student cannot accurately assign exact preventive care statuses that are not available from either source.

Suggested fix: Populate the immunization section with the same statuses used by the rubric or accept documented uncertainty when the chart is incomplete.

✧ new info uncovered

How consistently are you taking your medications? About how many doses do you miss, and what usually causes you to miss them?

For my other medications - the heart failure ones - I take those pretty regularly, I don't think I miss many doses. The only one I've really stopped completely is the apixaban, because of the bruising and being scared after what my friend told me.

✧ new info uncovered

Have cost, insurance, refills, transportation, or access made it harder to take your medications or keep appointments?

Well, transportation can be tough for me sometimes since I live alone and usually have to take the bus or get a ride to get places. As for cost, I actually got the apixaban through a patient assistance program, so that part's been resolved - it's really just the bruising and fear that's kept me from taking it.

✧ new info uncovered

Do you have any medication or food allergies? What reaction do you have?

No, I don't have any known drug or food allergies.

✧ new info uncovered

Do you know whether you are up to date on the vaccines recommended for you? Which vaccines do you remember receiving recently?

I'm not really sure about that one, I don't remember which vaccines I've had recently or if I'm up to date. That's not something that comes to mind right now.

✧ new info uncovered

Who do you live with, and what do you do for work or school?

I live alone, and I'm retired now - I used to work as an administrative assistant.

✧ new info uncovered

Do you currently use tobacco or nicotine, or have you in the past? If yes, how much, for how long, and when did you quit if you stopped?

I'm a former smoker, I quit about 8 years ago. Before that I smoked for a while, I had about a 15 pack-year history.

✧ new info uncovered

Evidence – Wednesday patient explicitly states that she does not remember her recent vaccine history.

Issue 9. Fixed income is required as Wednesday social history but is not returned when access barriers are reassessed

What the student saw or heard: When asked about cost, insurance, refills, transportation, and access, Angela says the assistance program resolved apixaban cost and that transportation can still be difficult because she lives alone. She does not state fixed income in the Wednesday interview.

What the grader or rubric expected: The Wednesday rubric specifically requires fixed income as a social history element.

Why this is a problem: The student asked the appropriate access question, but the required socioeconomic detail was not returned. A follow up patient should preserve important social determinants if they remain graded.

Suggested fix: Carry fixed income forward in the visible social history or have the AI patient disclose it when asked about affordability and access.

✧ new info uncovered

How consistently are you taking your medications? About how many doses do you miss, and what usually causes you to miss them?

For my other medications - the heart failure ones - I take those pretty regularly, I don't think I miss many doses. The only one I've really stopped completely is the apixaban, because of the bruising and being scared after what my friend told me.

✧ new info uncovered

Have cost, insurance, refills, transportation, or access made it harder to take your medications or keep appointments?

Well, transportation can be tough for me sometimes since I live alone and usually have to take the bus or get a ride to get places. As for cost, I actually got the apixaban through a patient assistance program, so that part's been resolved - it's really just the bruising and fear that's kept me from taking it.

✧ new info uncovered

Do you have any medication or food allergies? What reaction do you have?

No, I don't have any known drug or food allergies.

✧ new info uncovered

Do you know whether you are up to date on the vaccines recommended for you? Which vaccines do you remember receiving recently?

I'm not really sure about that one, I don't remember which vaccines I've had recently or if I'm up to date. That's not something that comes to mind right now.

✧ new info uncovered

Who do you live with, and what do you do for work or school?

I live alone, and I'm retired now - I used to work as an administrative assistant.

✧ new info uncovered

Do you currently use tobacco or nicotine, or have you in the past? If yes, how much, for how long, and when did you quit if you stopped?

I'm a former smoker, I quit about 8 years ago. Before that I smoked for a while, I had about a 15 pack-year history.

✧ new info uncovered

Evidence – Wednesday access discussion identifies resolved cost and ongoing transportation difficulty but does not provide the fixed income detail required by the rubric.

Issue 10. Laboratory values change across encounters despite the same collection date and time

What the student saw or heard: Wednesday labs are stamped 06/09/2026 07:50 and show potassium 4.3 mEq/L, BUN 17 mg/dL, eGFR 73 mL/min/1.73m², and WBC 6.5 x10³/mm³. Tuesday uses the same collection timestamp but shows potassium 4.4, BUN 18, eGFR 72, and WBC 6.8.

What the grader or rubric expected: A single laboratory collection date and time should represent one consistent set of results unless the chart clearly distinguishes separate specimens.

Why this is a problem: Changing values under the same timestamp makes longitudinal interpretation unreliable and can create artificial changes in renal function or blood counts.

Suggested fix: Assign unique collection times to genuinely different lab sets or keep the values identical when the timestamp represents the same specimen.

Comprehensive Metabolic Panel (CMP)

Renal / Kidney Function

Lipid Panel

Cardiac Biomarkers

Pulmonary Studies

Complete Blood Count & Immunology

Urine Studies

Missing / Needed Labs

Comprehensive Metabolic Panel (CMP)

Collected: 06/09/2026 07:50

TEST	RESULT	REFERENCE RANGE	FLAG
Na	139 mEq/L	—	
K	4.3 mEq/L	—	
BUN	17 mg/dL	—	
SCr	1.0 mg/dL	—	
eGFR	73 mL/min/1.73m ²	—	

Renal / Kidney Function

Collected: 06/09/2026 07:50

TEST	RESULT	REFERENCE RANGE	FLAG
BUN	17 mg/dL	—	
SCr	1.0 mg/dL	—	
eGFR	73 mL/min/1.73m ²	—	
K	4.3 mEq/L		

Complete Blood Count & Immunology

Collected: 06/09/2026 07:50

TEST	RESULT	REFERENCE RANGE	FLAG
WBC	6.5 x10 ³ /mm ³	—	

Evidence – Wednesday panels display a 06/09/2026 07:50 laboratory set whose values differ from Tuesday despite the identical timestamp.

Patient B – Angela Brooks – Thursday 6 Month Follow Up

10 grouped system or data issues are included in this working draft for this visit. Items are grouped so one underlying problem is not counted multiple times only because it produced several rubric deductions.

#	Issue	Problem type
1	Patient age conflicts with the displayed date of birth	Internal EMR inconsistency
2	The 6 month follow up is internally labeled as an initial visit and does not advance the longitudinal visit date	Longitudinal timeline inconsistency
3	The Thursday problem list omits HFrEF and the AFib qualifiers and comorbidities used by the grader	EMR and grader disagree
4	Atorvastatin is required by the grader but absent from the live medication list	EMR and grader disagree
5	The CBC panel shows WBC only while the grader expects hemoglobin and platelet values	Required information is unavailable
6	Preventive care statuses required by the grader are not visible beyond influenza	Required information is unavailable
7	The AI patient denies meaningful shortness of breath while the grader expects stable exertional dyspnea	AI patient and grader disagree
8	The AI patient denies unusual bruising while the grader expects a history of occasional minor bruising	AI patient and grader disagree
9	Transportation remains difficult in the AI interview while the grader expects the barrier to be resolved through family support	AI patient and grader disagree
10	Thursday laboratory data conflict with earlier encounters despite reused collection timestamps	Cross visit data inconsistency

Issue 1. Patient age conflicts with the displayed date of birth

What the student saw or heard: The Thursday header again displays Angela as 64 years old with DOB 03/12/1972.

What the grader or rubric expected: The Thursday rubric grades the patient as age 64.

Why this is a problem: The age and DOB remain mathematically incompatible in 2026. Repeating the inconsistency across visits makes it a longitudinal chart data problem rather than a single typographical error.

Suggested fix: Correct the demographic record and synchronize it across all three encounters and the grading reference.

<

AB

Angela Brooks
 64 yo female · Black · MRN W4-22091 · DOB 03/12/1972
 PCP: Dr. Johnson · Pharmacy: Walgreens · Language: English · Allergies: No known drug allergies (NKDA)

Thursday · 6-Month Follow-up

Reset

Exit

CODE STATUS

ISOLATION

COVERAGE

PHARMACIST OF RECORD

CARE MANAGER

LAST SEEN

NEXT APPT

Full Code

None

Aetna — Choice POS II (PPO)

R. Patel, PharmD (Ambulatory Care)

S. Nguyen, RN (Chronic Care Mgmt)

03/18/2026

06/30/2026 10:30

FLAGS

Fall risk

Advance directive

FYI

Case Progress: 0%

Evidence – Thursday header shows age 64 and DOB 03/12/1972.

Issue 2. The 6 month follow up is internally labeled as an initial visit and does not advance the longitudinal visit date

What the student saw or heard: The encounter header states Thursday 6 Month Follow Up, while the Subjective section states Initial Ambulatory Care Pharmacist Visit. The displayed vitals remain dated 06/23/2026, the same date used earlier in the longitudinal sequence.

What the grader or rubric expected: The grader treats this encounter as the later maintenance visit after prior interventions and expects longitudinal improvement.

Why this is a problem: The visit label and date structure do not reliably represent a true six-month progression. This can undermine any grading criterion that depends on improvement from prior visits.

Suggested fix: Use a consistent follow up encounter label and a chronologically later visit date for the six-month encounter.

Evidence – Thursday Subjective documentation calls the encounter an Initial Ambulatory Care Pharmacist Visit despite the 6 month follow up header.

Issue 3. The Thursday problem list omits HFrEF and the AFib qualifiers and comorbidities used by the grader

What the student saw or heard: The live Thursday Problem List displays AFib with the note Anticoagulated successfully. It does not show HFrEF, LVEF 35%, persistent nonvalvular classification, hypertension, or hyperlipidemia.

What the grader or rubric expected: The grader expects persistent nonvalvular AFib, HFrEF supported by LVEF 35%, hypertension, and hyperlipidemia to be incorporated into assessment and management.

Why this is a problem: The student facing chart does not expose the same active diagnoses the grader uses. This affects stroke risk interpretation, HFrEF classification, and comorbidity assessment.

Suggested fix: Populate the live problem list with the full intended active diagnoses and qualifiers or remove hidden problem list items from grading.

Evidence – Thursday live Problem List shows AFib only.

What the student saw or heard: The Thursday Complete Blood Count and Immunology panel displays only WBC $6.6 \times 10^3/\text{mm}^3$. Hemoglobin and platelet count are not shown.

What the grader or rubric expected: The grader expects hemoglobin and platelet documentation and uses those values in anticoagulation safety interpretation.

Why this is a problem: A student cannot document or interpret laboratory values that are absent from the visible panel. These are clinically relevant anticoagulation safety data and should not be hidden.

Suggested fix: Display hemoglobin and platelet count in the CBC panel if they are graded or remove those criteria when the results are not available.

Labs
Results are organized into panels — open the one you need

Lab panel Complete Blood Count & Immunology

Complete Blood Count & Immunology Collected: 06/09/2026 07:50

TEST	RESULT	REFERENCE RANGE	FLAG
WBC	$6.6 \times 10^3/\text{mm}^3$	—	

Evidence – Thursday CBC panel contains WBC only.

Issue 6. Preventive care statuses required by the grader are not visible beyond influenza

What the student saw or heard: The Thursday Immunizations section shows influenza as up to date. The patient facing data supplied do not show COVID, pneumococcal, Shingrix, or Tdap status.

What the grader or rubric expected: The grader expects specific COVID, pneumococcal, Shingrix, and Tdap information and scores preventive care assessment and plan items from those statuses.

Why this is a problem: The student cannot determine whether those vaccines are current, missing, initiated, or due from the visible Thursday record.

Suggested fix: Expose all vaccine statuses used by the grader in the live immunization section, or limit grading to vaccine data actually available to the student.

Objective

Objective data: vitals, labs, medications, allergies, immunizations



Vitals

Recorded 06/23/2026 09:14

BP
Ref <130/80 mmHg 124/74

Repeat BP
Ref <130/80 mmHg 122/72

HR
Ref 60–100 bpm 80 bpm

Temp
Ref 97.0–99.5 °F 98.0°F

RR
Ref 12–20 /min 16

SpO₂
Ref 95–100 % 98%

Weight
Ref — 83 kg

Height
Ref — 5'5"

Pain
Ref 0/10 0/10

BMI
Ref 18.5–24.9 kg/m² 30.5



Key labs

BASIC METABOLIC PANEL

Collected 06/09/2026 07:50

Na
Ref — 138 mEq/L

K
Ref — 4.4 mEq/L

BUN
Ref — 18 mg/dL

SCr
Ref — 1.0 mg/dL

eGFR
Ref — 72 mL/min/1.73m²

HEPATIC FUNCTION

Collected 06/09/2026 07:50

AST
Ref — 22 U/L

ALT
Ref — 24 U/L

COMPLETE BLOOD COUNT

Collected 06/09/2026 07:50

WBC
Ref — 6.6 x10³/mm³



Immunizations

Influenza
Up to date Up to date

Evidence – Thursday Objective view shows influenza only in the immunization section.

Issue 7. The AI patient denies meaningful shortness of breath while the grader expects stable exertional dyspnea

What the student saw or heard: Angela says she has no real shortness of breath, cannot identify a specific activity that brings it on, and can complete normal daily activities without getting winded.

What the grader or rubric expected: The grader expects stable mild exertional dyspnea as part of the HFrEF symptom pattern and deducts when dyspnea is documented as denied.

Why this is a problem: The patient and grader describe different current symptom states. A positive exertional symptom should not be invented after the patient explicitly denies it.

Suggested fix: Align the Thursday patient symptom script with the intended rubric or revise the grader to accept the actual negative dyspnea history.

"What brings you in today, and what are you hoping we can help you with?"

I'm just here for my regular checkup for my heart stuff - the irregular heartbeat and the heart failure. I've been feeling pretty okay lately, just want to make sure everything's still on track with my medications.

✧ new info uncovered

Overall, how have you been feeling since your last cardiology visit?

I've been feeling pretty good overall, no chest pain or anything like that. I do get a little tired sometimes, but nothing that's really stopped me from doing my usual things.

✧ new info uncovered

Tell me about any tiredness, shortness of breath, or other symptoms you have been having.

No real shortness of breath that I've noticed, and I haven't had any swelling in my legs or anything like that. Just that occasional tiredness I mentioned, but it doesn't stop me from doing my daily things.

✧ new info uncovered

What activities bring on the shortness of breath or fatigue?

Honestly, I haven't really noticed anything specific that brings it on - it's just sometimes I feel a little more tired than usual, but it's not tied to any particular activity. I can still do my normal day-to-day things without getting winded or anything.

✧ new info uncovered

Have your symptoms been getting better, worse, or staying about the same?

I'd say about the same, really. Nothing's really changed since my last visit that I've noticed.

✧ new info uncovered

Have you noticed your heartbeat feeling fast, irregular, fluttering, or different?

No, I haven't noticed anything like that. It's felt pretty normal to me.

✧ new info uncovered

Have you had any chest pain, dizziness, fainting, recent falls, or significant swelling in your legs or feet?

Evidence – Thursday interview documents no real shortness of breath and no specific activity related dyspnea.

Issue 8. The AI patient denies unusual bruising while the grader expects a history of occasional minor bruising

What the student saw or heard: When directly asked about bleeding or bruising on apixaban, Angela states that she has not noticed bleeding or unusual bruising and that things have been going smoothly.

What the grader or rubric expected: The grader expects a longitudinal history of occasional minor bruising that is no longer concerning and removes credit when the note documents complete denial.

Why this is a problem: A student should document the current patient response. The grader cannot simultaneously require a positive bruising history that the AI patient denies.

Suggested fix: Decide whether Thursday should represent resolved prior bruising with occasional minor bruising still reported or complete absence of bruising, then synchronize the patient and grader.

✧ new info uncovered

What is your understanding of atrial fibrillation?

I understand it's when my heart beats irregularly, kind of out of rhythm, instead of a steady beat. That's why I take the Eliquis - to help prevent blood clots and lower my risk of having a stroke, since my mom actually had a stroke from her AFib too.

✧ new info uncovered

Have you noticed any bleeding or bruising while taking apixaban?

No, I haven't noticed any bleeding or unusual bruising since I've been on it. Things have been going smoothly with that.

✧ new info uncovered

Do you check your blood pressure at home? If so, how often and what have your readings generally been?"

No, I don't have a blood pressure cuff or a scale at home. I just come in for my checkups to have it checked.

✧ new info uncovered

Do you monitor your weight at home? If so, how often, and have you noticed any recent changes?

No, I don't have a scale at home, so I don't really track my weight. I just check it when I come in for my appointments.

✧ new info uncovered

Tell me what a typical day of eating looks like for you. How much attention do you usually pay to sodium or salt?

Honestly, I haven't really paid too much attention to my salt intake. I don't have a specific routine I follow for that, I just eat what I normally eat.

✧ new info uncovered

What kind of physical activity are you able to do right now, and is fatigue or shortness of breath limiting you?

I'm able to do my normal daily things without much trouble. I do get a little tired sometimes, but it's not really stopping me from being active or doing what I need to do.

✧ new info uncovered

Evidence – Thursday AI patient directly denies bleeding or unusual bruising.

Issue 9. Transportation remains difficult in the AI interview while the grader expects the barrier to be resolved through family support

What the student saw or heard: Angela says transportation can still be difficult because she lives alone and usually has to take the bus or get a ride, although she has managed to attend appointments and obtain medications.

What the grader or rubric expected: The grader expects transportation to be resolved, including family support, and deducts when the note describes ongoing transportation difficulty.

Why this is a problem: The live patient describes improvement in medication access without saying the transportation barrier has resolved. These are different social determinant states.

Suggested fix: Use one longitudinal access story. If transportation is intended to be resolved, the patient should clearly state how it was resolved. Otherwise, the grader should accept residual transportation difficulty.

✧ new info uncovered

Have you ever delayed getting a refill, run out early, or tried to make your medications last longer?

No, I haven't had to do that. Since I got on that assistance program for the Eliquis, it's been affordable, and I've been able to keep up with all my refills just fine.

✧ new info uncovered

Do medication costs ever make it difficult for you to take your medicines the way they were prescribed?

No, not anymore. Since I got the Eliquis through that assistance program, it's been affordable, and I haven't had trouble affording any of my other medications either.

✧ new info uncovered

Are there particular medications that are harder for you to afford than others?

No, not really. Since I got the Eliquis through the assistance program, everything's been affordable for me, and I haven't had trouble with any of my other medications either.

✧ new info uncovered

Do you ever have trouble getting to the pharmacy, appointments, or picking up your medications?

Transportation can be a bit difficult for me since I live alone - I usually have to take the bus or get a ride from someone. But I've still managed to get to my appointments and pick up my medications okay.

✧ new info uncovered

How do you feel about the number of medications you are taking?

Honestly, I'm okay with it. I understand why I need each one, and with my pillbox it's not too hard to keep track of everything.

✧ new info uncovered

What is your understanding of heart failure and what it means for your heart?"

From what I understand, my heart doesn't pump as strongly as it should, so it can't get blood around my body as well as it used to. That's why I take those heart medications - to help it work better and keep me from getting too tired or swollen up. I don't know all the medical details, but that's the basic idea I've got.

Evidence – Thursday AI patient states that transportation can still be difficult and that she depends on the bus or rides.

Issue 10. Thursday laboratory data conflict with earlier encounters despite reused collection timestamps

What the student saw or heard: Thursday labs are again stamped 06/09/2026 07:50 but now show sodium 138 mEq/L, potassium 4.4, BUN 18, eGFR 72, AST 22, ALT 24, and WBC 6.6. Wednesday uses the same timestamp with sodium 139, potassium 4.3, BUN 17, eGFR 73, and WBC 6.5. Tuesday also uses the same timestamp with a different combination of values. The Thursday results

summary additionally marks Cardiac Biomarkers as Not ordered even though earlier encounter data show NT proBNP 210 pg/mL with that collection date.

What the grader or rubric expected: Identical collection dates and times should refer to a consistent laboratory dataset unless separate specimens are explicitly identified.

Why this is a problem: The reused timestamp makes the underlying objective record internally inconsistent across the longitudinal case. This can create artificial renal or safety trends and contradict whether a cardiac biomarker exists.

Suggested fix: Either use distinct collection dates for each encounter specific lab set or keep the values and panel availability consistent when the same timestamp is reused.

Labs

Results are organized into panels — open the one you need

Lab panel

Comprehensive Metabolic Panel (CMP)

Comprehensive Metabolic Panel (CMP)

Collected: 06/09/2026 07:50

TEST	RESULT	REFERENCE RANGE	FLAG
Na	138 mEq/L	—	
K	4.4 mEq/L	—	
BUN	18 mg/dL	—	
SCr	1.0 mg/dL	—	
eGFR	72 mL/min/1.73m ²	—	
AST	22 U/L	—	
ALT	24 U/L	—	

Evidence – Thursday CMP is labeled 06/09/2026 07:50 but contains values that differ from the Wednesday and Tuesday panels using the same timestamp.

VACS AI Patient & Grading Issue Report

Patient C – Robert Jenkins

Purpose: This report lists only confirmed system/data problems for Patient C, Robert Jenkins, found by comparing the AI patient interview, the EMR screen shown to the student, and the AI grader/rubric. SOAP writing suggestions and personal review notes have been removed. Repeated problems are listed under each visit because they can affect grading independently on different days.

Quick Guide for Non-Clinical Readers

Term	Plain language meaning
AI patient	The simulated patient the student interviews.
EMR	The electronic patient chart shown to the student.
Grader / rubric	The AI scoring system and the case facts it expects.
HFrEF	Heart failure with reduced ejection fraction.
AFib	Atrial fibrillation, an irregular heart rhythm that increases stroke risk.
CKD	Chronic kidney disease.
eGFR	An estimate of kidney filtration used to stage kidney disease.
DOAC	A direct oral anticoagulant, such as apixaban.
PCI / DES	A heart catheter and stent procedure; DES means drug eluting stent.
LVEF	Left ventricular ejection fraction, a measure of heart pumping function.

Final Review Summary

This final review contains 35 grouped findings across all 3 Patient C Week 4 visits. Only confirmed conflicts, incorrect display states, or missing information problems are included. Repeated issues are kept under each visit when they can affect grading independently.

Encounter	Grouped issues
Patient C – Robert Jenkins – Tuesday Initial Visit	15
Patient C – Robert Jenkins – Wednesday 3 Month Follow Up	10
Patient C – Robert Jenkins – Thursday 6 Month Follow Up	10

Patient C – Robert Jenkins – Tuesday Initial Visit

15 confirmed system/data issues are included for this visit. Items are grouped so one underlying problem is not counted multiple times just because it caused several grader deductions.

#	Issue	Problem type
1	SOAP grader is connected to a different patient rubric	Grader / case mapping
2	Patient age and date of birth are internally inconsistent in the live header	EMR data conflict
3	Expected mild exertional dyspnea directly conflicts with the Tuesday AI patient denial	AI patient and rubric disagree
4	Kidney concern and medication concern storyline does not match the Robert Tuesday rubric	AI patient and rubric disagree
5	Required chronic disease timing and PCI with drug eluting stent history cannot be obtained from the AI patient	AI patient and rubric disagree
6	Home blood pressure and heart rate ranges required by the rubric cannot be obtained from the AI patient	Required information is unavailable
7	COVID, pneumococcal, Shingrix, and Tdap statuses are graded but not reliably available	Required information is unavailable
8	Live Tuesday vital signs do not match the correct Robert Jenkins Tuesday rubric	EMR and rubric disagree
9	Required 12 month and 6-month renal trend values are not visible in the Tuesday EMR	Required information is unavailable
10	Hemoglobin and platelet values are required by the rubric but absent from the visible CBC	Required information is unavailable
11	HbA1c 6.9% is required by the rubric, but the glycemic panel is not ordered in the live EMR	Required information is unavailable
12	Current and prior LVEF values required by the rubric are not visible in the Tuesday encounter	Required information is unavailable
13	Rubric expects BNP trend while the live EMR displays NT proBNP only	EMR and rubric disagree
14	eGFR 38 mL/min/1.73m ² is incorrectly labeled Normal	EMR display error
15	Active problem list is incomplete compared with the rest of the Tuesday case	EMR data conflict

Issue 1. SOAP grader is connected to a different patient rubric

What the student saw or heard: The live Tuesday encounter identifies Robert Jenkins as a 76-year-old male. The interview and SOAP note describe his heart failure, atrial fibrillation, chronic kidney disease, type 2 diabetes, current Robert medication regimen, and concern about renal function and apixaban dosing.

What the grader expected: The AI feedback states that the note appears to describe a different patient and says the expected case is a 64-year-old female with a different medication, adherence, affordability, social, vital sign, and laboratory history.

Why this is a problem: This is not a documentation omission. The scoring engine is evaluating Robert Jenkins against another patient case, so many deductions cannot represent the student performance in the Tuesday Robert encounter.

Suggested fix: Map Robert Jenkins plus Tuesday plus the initial follow up state to the Robert Jenkins Tuesday rubric. Add a validation check that patient identity, day, encounter state, and rubric identifier match before grading.

STUDENT FACING EVIDENCE

What the student received

Live Tuesday header and interview identify Robert Jenkins, 76 year old male.

GRADER / RUBRIC EVIDENCE

- Tuesday AI feedback says it expected a 64 year old female case.

Tuesday Initial Visit • Issue 1 • Evidence excerpt from preserved VACS audit record

🔔 OPPORTUNITIES TO IMPROVE

This note appears to describe a different patient scenario (Robert Jenkins, 76-year-old male with CKD/apixaban dosing concerns) than the one specified by the grading rubric (a 64-year-old female with HFrEF LVEF 35%, medication nonadherence due to cost, living alone on fixed income). Ensure the correct patient case is being documented. Key missing elements per the intended rubric case include: LVEF value (35%), specific comorbidities (HTN, hyperlipidemia), accurate family history (father ischemic stroke age 70, mother HF, sister HTN), social history (lives alone, retired administrative assistant, fixed income, transportation difficulty), and the critical medication adherence/cost narrative (missed sacubitril/valsartan and apixaban doses due to cost, rationing supply, overwhelm with medication regimen, knowledge gap about apixaban's AFib/stroke prevention indication). Also verify exact medication doses (sacubitril/valsartan 49/51mg, metoprolol succinate 50mg) match the intended case. Ensure all vital signs and laboratory values match the source data precisely (BP, HR, rhythm, temperature, SpO2, weight, height, BMI, and full chemistry/hematology panel including AST, ALT, hemoglobin, platelets, BNP, and echocardiogram findings). Include CHA2DS2-VASc scoring with enumerated risk factors, note absence of anticoagulation monitoring labs, and

Evidence – Tuesday Initial Visit: student facing information compared with the grader/rubric expectation.

Issue 2. Patient age and date of birth are internally inconsistent in the live header

What the student saw or heard: The Tuesday header displays Robert as 76 years old while also showing a date of birth of 03/12/1972.

What the grader expected: The Robert Jenkins Tuesday rubric identifies the patient as 76 years old. A date of birth that is consistent with that age should be stored and displayed throughout the chart.

Why this is a problem: Two identity fields in the same header describe different ages. Patient demographics should not require the student to decide which field is authoritative.

Suggested fix: Correct the stored date of birth or the displayed age so the demographic fields agree across the header, patient object, and grader.

STUDENT FACING EVIDENCE

What the student received

Tuesday encounter header displays age 76 and DOB 03/12/1972.

GRADER / RUBRIC EVIDENCE

- The Robert Jenkins Tuesday rubric identifies the patient as 76 years old. A date of birth that is consistent with that age should be stored and displayed throughout the chart.

Tuesday Initial Visit • Issue 2 • Evidence excerpt from preserved VACS audit record

The screenshot displays a patient header for Robert Jenkins. At the top, it shows the patient's name, age (76 yr male), gender (Male), and date of birth (DOB: 03/12/1972). Below this, there are several tabs: 'Vitals', 'Medications', 'Allergies', 'Immunizations', 'Lab Results', 'Diagnoses', 'Procedures', 'Referrals', 'Care Plans', 'Notes', 'Messages', 'Forms', and 'Reports'. The 'Vitals' tab is currently selected, showing a table with columns for 'Vital Signs', 'Date/Time', and 'Value'. The table contains one row of data: 'BP 120/80', '03/12/2024', and '120/80'. The 'Medications' tab is also visible, showing a table with columns for 'Medication', 'Dose', 'Frequency', and 'Status'. The table contains one row of data: 'Aspirin 81mg', '81mg', 'Daily', and 'Active'. The 'Allergies' tab is also visible, showing a table with columns for 'Allergy', 'Severity', and 'Status'. The table contains one row of data: 'Penicillin', 'Severe', and 'Active'. The 'Immunizations' tab is also visible, showing a table with columns for 'Immunization', 'Date', and 'Status'. The table contains one row of data: 'MMR', '03/12/2024', and 'Up to date'. The 'Lab Results' tab is also visible, showing a table with columns for 'Lab Test', 'Date/Time', and 'Result'. The table contains one row of data: 'Hemoglobin A1c', '03/12/2024', and '5.7%'. The 'Diagnoses' tab is also visible, showing a table with columns for 'Diagnosis', 'Date/Time', and 'Status'. The table contains one row of data: 'Hypertension', '03/12/2024', and 'Active'. The 'Procedures' tab is also visible, showing a table with columns for 'Procedure', 'Date/Time', and 'Status'. The table contains one row of data: 'Physical Exam', '03/12/2024', and 'Completed'. The 'Referrals' tab is also visible, showing a table with columns for 'Referral', 'Date/Time', and 'Status'. The table contains one row of data: 'Referral to Cardiology', '03/12/2024', and 'Active'. The 'Care Plans' tab is also visible, showing a table with columns for 'Care Plan', 'Date/Time', and 'Status'. The table contains one row of data: 'Care Plan for Hypertension', '03/12/2024', and 'Active'. The 'Notes' tab is also visible, showing a table with columns for 'Note', 'Date/Time', and 'Status'. The table contains one row of data: 'Initial Visit Note', '03/12/2024', and 'Active'. The 'Messages' tab is also visible, showing a table with columns for 'Message', 'Date/Time', and 'Status'. The table contains one row of data: 'Message from Doctor', '03/12/2024', and 'Active'. The 'Forms' tab is also visible, showing a table with columns for 'Form', 'Date/Time', and 'Status'. The table contains one row of data: 'Form for Hypertension', '03/12/2024', and 'Active'. The 'Reports' tab is also visible, showing a table with columns for 'Report', 'Date/Time', and 'Status'. The table contains one row of data: 'Report for Hypertension', '03/12/2024', and 'Active'. The bottom of the header shows the patient's current status as 'Active' and the date of the last update as '03/12/2024'.

Evidence – Tuesday Initial Visit: student facing information compared with the grader/rubric expectation.

Issue 3. Expected mild exertional dyspnea directly conflicts with the Tuesday AI patient denial

What the student saw or heard: Robert is directly asked what activities make him short of breath or tired. He says he has not noticed shortness of breath or increased tiredness and can perform his usual activities around the house and yard without trouble.

What the grader expected: The Robert Jenkins Tuesday rubric requires mild exertional dyspnea that is unchanged from previous visits.

Why this is a problem: This is a positive versus negative symptom contradiction after a targeted question. The student cannot accurately document mild exertional dyspnea when the AI patient explicitly denies it.

Suggested fix: Synchronize the Tuesday symptom state. If mild exertional dyspnea is an intended case fact, the AI patient should report it when asked directly about exertion and exercise tolerance.

STUDENT FACING EVIDENCE

What the student received

Tuesday patient interview: Robert denies exertional shortness of breath and increased tiredness.

GRADER / RUBRIC EVIDENCE

- Robert Jenkins Tuesday requires mild exertional dyspnea unchanged from prior visits.

Tuesday Initial Visit • Issue 3 • Evidence excerpt from preserved VACS audit record

Evidence – Tuesday Initial Visit: student facing information compared with the grader/rubric expectation.

Issue 4. Kidney concern and medication concern storyline does not match the Robert Tuesday rubric

What the student saw or heard: Robert is worried about his kidney numbers, but his medication specific question is whether creatinine 1.82 mg/dL requires a lower apixaban dose. When asked whether the renal concern made him think about changing or stopping medications, he says no and states that he trusts the care team to guide him.

What the grader expected: The Robert Jenkins Tuesday rubric requires concern that sacubitril/valsartan and spironolactone may be worsening kidney function, independent research about medication related kidney effects, consideration of stopping spironolactone, concern shared by his wife, a goal of avoiding dialysis, and a requested risk benefit discussion about continuing therapy.

Why this is a problem: The core patient concern drives the intended interview and assessment. The AI patient supplies a different medication concern and explicitly denies the medication stopping thought that the rubric requires.

Suggested fix: Load the intended renal concern narrative into the Tuesday AI patient. Required medication concerns, spouse concerns, and dialysis goals should be available when directly explored.

STUDENT FACING EVIDENCE

What the student received

Tuesday interview: the main medication question concerns apixaban dosing, and Robert denies considering stopping medications.

GRADER / RUBRIC EVIDENCE

- Tuesday rubric expects sacubitril/valsartan and spironolactone renal concerns and consideration of stopping spironolactone.

Tuesday Initial Visit • Issue 4 • Evidence excerpt from preserved VACS audit record

Evidence – Tuesday Initial Visit: student facing information compared with the grader/rubric expectation.

Issue 5. Required chronic disease timing and PCI with drug eluting stent history cannot be obtained from the AI patient

What the student saw or heard: When asked how long he has had his major conditions, Robert says he cannot remember the exact years. When specifically asked about surgeries or procedures, he says he does not recall having any surgeries or procedures.

What the grader expected: The Tuesday rubric expects HFrEF diagnosed 4 years ago with LVEF 30% at diagnosis, NSTEMI 3 years ago, coronary artery disease, and PCI with drug eluting stent placement listed in history.

Why this is a problem: The student asked the relevant history questions, but the AI patient cannot provide the required timing and directly denies a prior procedure. Those details should not be invented.

Suggested fix: Persist the longitudinal cardiac and procedural history in the AI patient and expose the exact diagnosis timing when it is required by the rubric.

STUDENT FACING EVIDENCE

What the student received

Tuesday interview: Robert cannot provide diagnosis years and says no prior surgery or procedure comes to mind.

GRADER / RUBRIC EVIDENCE

- Tuesday rubric requires HFrEF timing, NSTEMI timing, and PCI with drug eluting stent history.

Tuesday Initial Visit • Issue 5 • Evidence excerpt from preserved VACS audit record

Evidence – Tuesday Initial Visit: student facing information compared with the grader/rubric expectation.

Issue 6. Home blood pressure and heart rate ranges required by the rubric cannot be obtained from the AI patient

What the student saw or heard: Robert reports checking blood pressure and heart rate every day. When asked what the readings usually are, he says they are steady, normal, and similar to the clinic values, but he does not provide numeric ranges.

What the grader expected: The Tuesday rubric requires a home blood pressure range of 110 to 118 over 64 to 70 mmHg and a home heart rate range of 68 to 76 bpm.

Why this is a problem: The student directly requested the home readings, but the AI patient did not return the graded numbers. A precise range should not be created from a general statement that readings are normal.

Suggested fix: Program the AI patient to provide the intended ranges when asked for usual home readings or display the home log in the visible chart.

STUDENT FACING EVIDENCE

What the student received

Tuesday interview: home values are described only as steady and normal.

GRADER / RUBRIC EVIDENCE

- Tuesday rubric requires exact home BP and HR ranges.

Tuesday Initial Visit • Issue 6 • Evidence excerpt from preserved VACS audit record

Evidence – Tuesday Initial Visit: student facing information compared with the grader/rubric expectation.

Issue 7. COVID, pneumococcal, Shingrix, and Tdap statuses are graded but not reliably available

What the student saw or heard: The visible Tuesday immunization area confirms influenza is up to date. Robert says he is unsure of his complete vaccine history and cannot recall which vaccines he has received recently.

What the grader expected: The Tuesday rubric requires COVID vaccination up to date, pneumococcal vaccination completed, Shingrix completed, and Tdap up to date in addition to influenza.

Why this is a problem: The student cannot reliably document the exact preventive care statuses when the chart does not display them, and the patient cannot recall them.

Suggested fix: Populate all graded vaccine statuses in the visible immunization record or make them consistently available through the AI patient.

STUDENT FACING EVIDENCE

What the student received

Tuesday EMR shows influenza up to date; other required statuses are not exposed. Tuesday interview: Robert is unsure of recent vaccine history.

GRADER / RUBRIC EVIDENCE

- The Tuesday rubric requires COVID vaccination up to date, pneumococcal vaccination completed, Shingrix completed, and Tdap up to date in addition to influenza.

Tuesday Initial Visit • Issue 7 • Evidence excerpt from preserved VACS audit record

Evidence – Tuesday Initial Visit: student facing information compared with the grader/rubric expectation.

Issue 8. Live Tuesday vital signs do not match the correct Robert Jenkins Tuesday rubric

What the student saw or heard: The live Tuesday EMR shows BP 120/70 mmHg with repeat 118/68, HR 68 bpm, temperature 98.6°F, SpO2 97%, weight 82 kg, and BMI 28.3.

What the grader expected: The Robert Jenkins Tuesday rubric expects BP 112/68 mmHg, HR 72 bpm, temperature 98.1°F, SpO2 98% on room air, weight 82 kg, and BMI 27.8 kg/m².

Why this is a problem: These are objective values. A student should copy the values displayed in the live EMR and should not be penalized for failing to use hidden rubric values.

Suggested fix: Use one encounter data source for the EMR renderer and Robert Tuesday rubric. Synchronize all vital sign fields before the case is released.

STUDENT FACING EVIDENCE

What the student received

Tuesday live vitals display 120/70, HR 68, temperature 98.6°F, SpO2 97%, weight 82 kg, BMI 28.3.

GRADER / RUBRIC EVIDENCE

- correct Robert Tuesday rubric contains different BP, HR, temperature, SpO2, and BMI values.

Tuesday Initial Visit • Issue 8 • Evidence excerpt from preserved VACS audit record

Evidence – Tuesday Initial Visit: student facing information compared with the grader/rubric expectation.

Issue 9. Required 12 month and 6-month renal trend values are not visible in the Tuesday EMR

What the student saw or heard: The live renal panel displays the current BUN 34 mg/dL, serum creatinine 1.82 mg/dL, and eGFR 38 mL/min/1.73m². Prior renal values are not displayed in the provided Tuesday laboratory panels.

What the grader expected: The Tuesday rubric requires 12-month serum creatinine 1.42 mg/dL and eGFR 52, plus 6-month serum creatinine 1.58 mg/dL and eGFR 46 and requires interpretation of the 12-month decline.

Why this is a problem: The rubric requires a longitudinal trend that the student cannot reconstruct from the current value alone.

Suggested fix: Display the prior renal values and dates when trend interpretation is graded or remove the exact historical trend from scoring if it is intentionally unavailable.

STUDENT FACING EVIDENCE

What the student received

Tuesday renal panel shows current values only.

GRADER / RUBRIC EVIDENCE

- Tuesday rubric requires 12 month and 6 month creatinine and eGFR values.

Tuesday Initial Visit • Issue 9 • Evidence excerpt from preserved VACS audit record

Evidence – Tuesday Initial Visit: student facing information compared with the grader/rubric expectation.

Issue 10. Hemoglobin and platelet values are required by the rubric but absent from the visible CBC

What the student saw or heard: The Tuesday CBC panel displays WBC $6.4 \times 10^3/\text{mm}^3$ only. Hemoglobin and platelets are not shown.

What the grader expected: The Tuesday rubric requires hemoglobin 13.5 g/dL and platelets $228 \times 10^3/\text{mm}^3$.

Why this is a problem: The student cannot report exact CBC values that the live laboratory panel does not contain.

Suggested fix: Add hemoglobin and platelets to the Tuesday CBC panel when they are grading elements or mark those criteria as unavailable and not gradable.

STUDENT FACING EVIDENCE

What the student received

Tuesday CBC panel contains WBC only.

GRADER / RUBRIC EVIDENCE

- Tuesday rubric requires exact hemoglobin and platelet values.

Tuesday Initial Visit • Issue 10 • Evidence excerpt from preserved VACS audit record

Evidence – Tuesday Initial Visit: student facing information compared with the grader/rubric expectation.

Issue 11. HbA1c 6.9% is required by the rubric, but the glycemic panel is not ordered in the live EMR

What the student saw or heard: The Tuesday laboratory overview marks Diabetes / Glycemic Control as Not ordered. No HbA1c result is visible.

What the grader expected: The Tuesday rubric requires HbA1c 6.9% and interpretation that it is near target or controlled.

Why this is a problem: A numeric HbA1c cannot be documented from a panel that the system identifies as not ordered.

Suggested fix: Display the intended HbA1c result if it is required for grading or remove the value and interpretation criteria from the Tuesday score.

STUDENT FACING EVIDENCE

What the student received

Tuesday lab overview marks glycemic control testing as Not ordered.

GRADER / RUBRIC EVIDENCE

- Tuesday rubric requires HbA1c 6.9%.

Tuesday Initial Visit • Issue 11 • Evidence excerpt from preserved VACS audit record

Evidence – Tuesday Initial Visit: student facing information compared with the grader/rubric expectation.

Issue 12. Current and prior LVEF values required by the rubric are not visible in the Tuesday encounter

What the student saw or heard: The live Tuesday EMR provided to the student does not display an echocardiogram result or LVEF value.

What the grader expected: The Tuesday rubric requires current LVEF 32%, baseline LVEF 30%, and a statement that the echocardiographic finding is stable or slightly improved.

Why this is a problem: The student would have to invent an objective cardiac imaging value and trend to satisfy the rubric.

Suggested fix: Expose the current and prior echocardiogram results in the Tuesday EMR when LVEF is a scored element or make the rubric field not gradable when imaging is unavailable.

STUDENT FACING EVIDENCE

What the student received

No echocardiogram or LVEF result is visible in the Tuesday EMR screenshots.

GRADER / RUBRIC EVIDENCE

- Tuesday rubric requires LVEF 32% with prior 30%.

Tuesday Initial Visit • Issue 12 • Evidence excerpt from preserved VACS audit record

Evidence – Tuesday Initial Visit: student facing information compared with the grader/rubric expectation.

Issue 13. Rubric expects BNP trend while the live EMR displays NT proBNP only

What the student saw or heard: The Tuesday live EMR displays NT proBNP 165 pg/mL. No prior cardiac biomarker value is shown.

What the grader expected: The Tuesday rubric requires current BNP 165 pg/mL, previous BNP 210 pg/mL, and interpretation that BNP improved from 210 to 165.

Why this is a problem: BNP and NT proBNP are separate laboratory assays, and the required prior value is not exposed. The student should not relabel the test or invent a trend.

Suggested fix: Use the same biomarker name and longitudinal values across the live EMR and rubric. If NT proBNP is the intended test, the rubric should use NT proBNP and show the prior value.

STUDENT FACING EVIDENCE

What the student received

Tuesday EMR labels the biomarker NT proBNP 165 pg/mL.

GRADER / RUBRIC EVIDENCE

- Tuesday rubric labels the test BNP and requires a previous value of 210 pg/mL.

Tuesday Initial Visit • Issue 13 • Evidence excerpt from preserved VACS audit record

Evidence – Tuesday Initial Visit: student facing information compared with the grader/rubric expectation.

Issue 14. eGFR 38 mL/min/1.73m² is incorrectly labeled Normal

What the student saw or heard: The Tuesday renal panel shows eGFR 38 mL/min/1.73m² with a blue Normal status badge, while serum creatinine is flagged high and CKD is an active problem.

What the grader expected: The same Tuesday rubric classifies eGFR 38 as CKD Stage 3b and approaching the Stage 4 threshold.

Why this is a problem: The numeric result and status badge communicate conflicting interpretations of kidney function and can mislead the learner.

Suggested fix: Correct the eGFR status logic so the badge is calculated from the numeric result and intended CKD classification.

STUDENT FACING EVIDENCE

What the student received

Tuesday renal panel displays eGFR 38 with a Normal badge.

GRADER / RUBRIC EVIDENCE

- Tuesday rubric classifies eGFR 38 as CKD Stage 3b.

Tuesday Initial Visit • Issue 14 • Evidence excerpt from preserved VACS audit record

Evidence – Tuesday Initial Visit: student facing information compared with the grader/rubric expectation.

Issue 15. Active problem list is incomplete compared with the rest of the Tuesday case

What the student saw or heard: The visible Tuesday Problem List contains atrial fibrillation and chronic kidney disease, while the medication indications and AI patient history also support heart failure and type 2 diabetes, and the rubric includes coronary artery disease, hypertension, and hyperlipidemia.

What the grader expected: The Robert Tuesday rubric expects HFrEF, persistent nonvalvular AFib, CAD, CKD Stage 3b, type 2 diabetes, hypertension, and hyperlipidemia as the longitudinal medical history.

Why this is a problem: A student relying on the active Problem List receives an incomplete disease list that does not match other sections of the same encounter.

Suggested fix: Populate the active Problem List from the same longitudinal diagnosis source used by the AI patient, medications, and grader.

STUDENT FACING EVIDENCE

What the student received

Tuesday Problem List displays only AFib and CKD.

GRADER / RUBRIC EVIDENCE

- Tuesday Robert history contains several additional active chronic conditions.

Tuesday Initial Visit • Issue 15 • Evidence excerpt from preserved VACS audit record

Evidence – Tuesday Initial Visit: student facing information compared with the grader/rubric expectation.

Patient C – Robert Jenkins – Wednesday 3 Month Follow Up

10 confirmed system/data issues are included for this visit. Items are grouped so one underlying problem is not counted multiple times just because it caused several grader deductions.

Issue	Problem type
Wednesday encounter loads an epistaxis story instead of the required colonoscopy planning scenario	AI patient and encounter mapping disagreement
Patient age and date of birth remain internally inconsistent in the Wednesday header	EMR data conflict
Medical history returned by the AI patient is incomplete when directly requested	Required information is unavailable
Prior PCI with drug eluting stent history is directly denied by the Wednesday AI patient	AI patient and rubric disagree
Preventive care statuses required by the rubric are not available from the Wednesday patient or visible record	Required information is unavailable
Live Wednesday vital signs do not match the correct Robert Jenkins Wednesday rubric	EMR and rubric disagree
Hemoglobin and platelet values are required but the Wednesday CBC panel shows only WBC	Required information is unavailable
eGFR 39 mL/min/1.73m ² is incorrectly labeled Normal	EMR display error
Wednesday active Problem List is incomplete and reflects the incorrect epistaxis encounter state	EMR data conflict
Counseling content uses warfarin INR language for a patient receiving apixaban	Medication counseling content error

Issue 1. Wednesday encounter loads an epistaxis story instead of the required colonoscopy planning scenario

What the student saw or heard: The live Wednesday Subjective section and AI patient present recent nosebleeds as the reason for the visit. Robert reports two nosebleeds in the past month, asks whether a safer blood thinner exists, and repeatedly states that no colonoscopy, gastroenterology visit, or upcoming procedure has been discussed.

What the grader expected: The Robert Jenkins Wednesday rubric requires an elective screening colonoscopy in approximately three weeks, gastroenterology advice that apixaban interruption may be necessary, no recent bleeding events, concern about stroke risk during interruption, questions about bridging, and written stop and restart instructions.

Why this is a problem: The encounter is running the wrong clinical state. The student cannot complete the intended periprocedural anticoagulation interview when the patient denies the procedure that defines the visit.

Suggested fix: Map Robert Jenkins Wednesday to the colonoscopy planning state. Lock the colonoscopy, gastroenterology, and no recent bleeding facts so generated responses cannot contradict them.

STUDENT FACING EVIDENCE

What the student received

Wednesday interview opens with two recent nosebleeds and a request for a safer blood thinner. Robert repeatedly denies a scheduled colonoscopy or gastroenterology discussion.

GRADER / RUBRIC EVIDENCE

- Wednesday requires colonoscopy planning, no bleeding events, bridging questions, and stop / restart education.

Wednesday 3 Month Follow Up • Issue 1 • Evidence excerpt from preserved VACS audit record

Evidence – Wednesday 3 Month Follow Up: student facing information compared with the grader/rubric expectation.

Issue 2. Patient age and date of birth remain internally inconsistent in the Wednesday header

What the student saw or heard: The Wednesday header displays Robert as 76 years old and also displays DOB 03/12/1972.

What the grader expected: The Wednesday Robert rubric identifies the patient as 76 years old, so all stored identity fields should be consistent with that age.

Why this is a problem: The same patient record contains conflicting demographic information before the clinical encounter begins.

Suggested fix: Correct the date of birth or age in the longitudinal patient record and use the corrected value in everyday header and grading context.

STUDENT FACING EVIDENCE

What the student received

Wednesday header shows age 76 with DOB 03/12/1972.

GRADER / RUBRIC EVIDENCE

- The Wednesday Robert rubric identifies the patient as 76 years old, so all stored identity fields should be consistent with that age.

Wednesday 3 Month Follow Up • Issue 2 • Evidence excerpt from preserved VACS audit record

Evidence – Wednesday 3 Month Follow Up: student facing information compared with the grader/rubric expectation.

Issue 3. Medical history returned by the AI patient is incomplete when directly requested

What the student saw or heard: When asked what medical conditions he has, Robert reports AFib, heart failure, some heart disease, and diabetes. He does not clearly identify CKD Stage 3b, hypertension, or hyperlipidemia.

What the grader expected: The Wednesday rubric requires HFrEF, persistent nonvalvular AFib, CAD, CKD Stage 3b, type 2 diabetes, hypertension, and hyperlipidemia.

Why this is a problem: The student directly asked for the medical history, but the AI patient returned an incomplete disease list. The grader should not require conditions the patient could not supply unless they are visible elsewhere in the chart.

Suggested fix: Use the same longitudinal diagnosis array for the AI patient, Problem List, and rubric so a direct medical history question returns the complete intended list.

STUDENT FACING EVIDENCE

What the student received

Wednesday interview: Robert lists AFib, heart failure, heart disease, and diabetes only.

GRADER / RUBRIC EVIDENCE

- Wednesday rubric includes CKD Stage 3b, hypertension, and hyperlipidemia in addition to the listed conditions.

Wednesday 3 Month Follow Up • Issue 3 • Evidence excerpt from preserved VACS audit record

Evidence – Wednesday 3 Month Follow Up: student facing information compared with the grader/rubric expectation.

Issue 4. Prior PCI with drug eluting stent history is directly denied by the Wednesday AI patient

What the student saw or heard: When asked whether he has ever had surgeries or procedures, Robert says he does not recall having any surgeries or procedures.

What the grader expected: The Wednesday rubric requires prior PCI with drug eluting stent placement in both past medical and past surgical history.

Why this is a problem: This is a direct contradiction after a targeted procedure question. The student cannot safely document PCI with drug eluting stent when the simulated patient denies prior procedures.

Suggested fix: Persist PCI with drug eluting stent as a locked longitudinal history fact and return it when the student asks about prior procedures.

STUDENT FACING EVIDENCE

What the student received

Wednesday interview: Robert says no surgery or procedure comes to mind.

GRADER / RUBRIC EVIDENCE

- Wednesday rubric requires PCI with drug eluting stent placement.

Wednesday 3 Month Follow Up • Issue 4 • Evidence excerpt from preserved VACS audit record

Evidence – Wednesday 3 Month Follow Up: student facing information compared with the grader/rubric expectation.

Issue 5. Preventive care statuses required by the rubric are not available from the Wednesday patient or visible record

What the student saw or heard: Robert says he does not remember which vaccines he has received recently. The Wednesday EMR clearly confirms influenza, but the provided student facing information does not reliably expose the other exact statuses.

What the grader expected: The Wednesday rubric requires COVID up to date, pneumococcal completed, Shingrix completed, and Tdap up to date.

Why this is a problem: The student cannot convert patient uncertainty into confirmed completed vaccination status.

Suggested fix: Populate all graded immunization statuses in the visible record or make the AI patient reliably state them when asked.

STUDENT FACING EVIDENCE

What the student received

Wednesday interview: Robert is unsure of recent vaccines.

GRADER / RUBRIC EVIDENCE

- Wednesday rubric grades exact completed / current statuses for multiple vaccines.

Wednesday 3 Month Follow Up • Issue 5 • Evidence excerpt from preserved VACS audit record

Evidence – Wednesday 3 Month Follow Up: student facing information compared with the grader/rubric expectation.

Issue 6. Live Wednesday vital signs do not match the correct Robert Jenkins

Wednesday rubric

What the student saw or heard: The Wednesday EMR shows BP 120/70 mmHg with repeat 118/68, HR 68 bpm, temperature 98.6°F, SpO2 97%, weight 82 kg, and BMI 28.3.

What the grader expected: The correct Robert Wednesday rubric expects BP 114/68 mmHg, HR 70 bpm, temperature 98.0°F, SpO2 98% on room air, weight 82 kg, and BMI 27.8 kg/m².

Why this is a problem: The student should document the objective values displayed in the live chart, not hidden values from a different data version.

Suggested fix: Synchronize the Wednesday vital sign record used by the EMR and the Robert Wednesday rubric.

STUDENT FACING EVIDENCE

What the student received

Wednesday live vitals display 120/70, HR 68, temperature 98.6°F, SpO2 97%, weight 82 kg, BMI 28.3.

GRADER / RUBRIC EVIDENCE

- Wednesday master rubric contains different BP, HR, temperature, SpO2, and BMI values.

Wednesday 3 Month Follow Up • Issue 6 • Evidence excerpt from preserved VACS audit record

Evidence – Wednesday 3 Month Follow Up: student facing information compared with the grader/rubric expectation.

Issue 7. Hemoglobin and platelet values are required but the Wednesday CBC panel shows only WBC

What the student saw or heard: The Wednesday CBC panel provides WBC $6.5 \times 10^3/\text{mm}^3$ but does not show hemoglobin or platelet count.

What the grader expected: The Wednesday rubric requires hemoglobin 13.4 g/dL and platelets $226 \times 10^3/\text{mm}^3$ and requires interpretation of anemia and thrombocytopenia risk.

Why this is a problem: The student cannot document or interpret laboratory values that are absent from the live panel.

Suggested fix: Display hemoglobin and platelets when they are required for grading or mark those rubric items unavailable and not gradable.

STUDENT FACING EVIDENCE

What the student received

Wednesday CBC panel displays WBC only.

GRADER / RUBRIC EVIDENCE

- Wednesday rubric requires hemoglobin and platelet values.

Wednesday 3 Month Follow Up • Issue 7 • Evidence excerpt from preserved VACS audit record

Evidence – Wednesday 3 Month Follow Up: student facing information compared with the grader/rubric expectation.

Issue 8. eGFR 39 mL/min/1.73m² is incorrectly labeled Normal

What the student saw or heard: The Wednesday CMP and renal panels display eGFR 39 mL/min/1.73m² with a normal badge while the overall renal panel is marked abnormal.

What the grader expected: The Wednesday rubric identifies eGFR 39 as CKD Stage 3b and uses the value in anticoagulation planning.

Why this is a problem: The status badge contradicts both the numeric value and the intended CKD classification.

Suggested fix: Correct the eGFR flagging threshold and generate the status badge directly from the numeric result.

STUDENT FACING EVIDENCE

What the student received

Wednesday renal panel shows eGFR 39 with a Normal badge.

GRADER / RUBRIC EVIDENCE

- Wednesday rubric classifies eGFR 39 as CKD Stage 3b.

Wednesday 3 Month Follow Up • Issue 8 • Evidence excerpt from preserved VACS audit record

Evidence – Wednesday 3 Month Follow Up: student facing information compared with the grader/rubric expectation.

Issue 9. Wednesday active Problem List is incomplete and reflects the incorrect epistaxis encounter state

What the student saw or heard: The Wednesday Problem List shows AFib with minor epistaxis on DOAC. It does not display the complete chronic problem set used elsewhere in the case.

What the grader expected: The Wednesday rubric uses HFrEF, persistent nonvalvular AFib, CAD, CKD Stage 3b, type 2 diabetes, hypertension, and hyperlipidemia, and the intended encounter does not use recent epistaxis as a positive case fact.

Why this is a problem: The Problem List both omits major chronic conditions and reinforces the wrong Wednesday symptom story.

Suggested fix: Populate the Problem List from the longitudinal diagnosis source and remove the epistaxis encounter fact when the correct Wednesday colonoscopy planning state is loaded.

STUDENT FACING EVIDENCE

What the student received

Wednesday Problem List displays AFib with minor epistaxis on DOAC.

GRADER / RUBRIC EVIDENCE

- Wednesday encounter requires a broader chronic problem history and denial of recent bleeding.

Wednesday 3 Month Follow Up • Issue 9 • Evidence excerpt from preserved VACS audit record

Evidence – Wednesday 3 Month Follow Up: student facing information compared with the grader/rubric expectation.

Issue 10. Counseling content uses warfarin INR language for a patient receiving apixaban

What the student saw or heard: The Wednesday counseling card warns that antibiotics and over the counter pain relievers can change the patient INR. Robert is taking apixaban 5 mg twice daily, not warfarin.

What the grader expected: The Wednesday rubric specifically notes that routine INR monitoring is not required for apixaban and also states that routine anti Xa monitoring is not required.

Why this is a problem: The system is displaying anticoagulation counseling that belongs to a different medication monitoring framework and can teach an incorrect monitoring concept.

Suggested fix: Make anticoagulation counseling medication specific. For apixaban, remove warfarin INR language and use DOAC appropriate bleeding, interaction, adherence, and procedure counseling.

STUDENT FACING EVIDENCE

What the student received

Wednesday counseling content references changing INR. Wednesday medication list and interview identify apixaban 5 mg BID.

GRADER / RUBRIC EVIDENCE

- Wednesday rubric states routine INR monitoring is not required for apixaban.

Wednesday 3 Month Follow Up • Issue 10 • Evidence excerpt from preserved VACS audit record

Evidence – Wednesday 3 Month Follow Up: student facing information compared with the grader/rubric expectation.

Patient C – Robert Jenkins – Thursday 6 Month Follow Up

10 confirmed system/data issues are included for this visit. Items are grouped so one underlying problem is not counted multiple times just because it caused several grader deductions.

Issue	Problem type
SOAP grader is connected to a different 64-year-old female case	Grader / case mapping
Patient age and date of birth remain internally inconsistent in the Thursday header	EMR data conflict
Live Thursday vital signs do not match the correct Robert Jenkins Thursday rubric	EMR and rubric disagree
Hemoglobin trend and platelet count required by the rubric are missing from the Thursday CBC	Required information is unavailable
Gastroenterology report is marked missing even though the rubric grades report sourced objective findings	Data / grading mismatch
eGFR 40 mL/min/1.73m ² is incorrectly labeled Normal	EMR display error
Thursday active Problem List contains only AFib despite multiple active chronic conditions	EMR data conflict
COVID, pneumococcal, Shingrix, and Tdap statuses are graded but not available from the Thursday patient or visible immunization record	Required information is unavailable
Required longitudinal cardiac history and disease details remain incomplete in the Thursday AI patient	Required information is unavailable
Expected interest in kidney function monitoring conflicts with the Thursday AI patient response	AI patient and rubric disagree

Issue 1. SOAP grader is connected to a different 64-year-old female case

What the student saw or heard: The live Thursday encounter identifies Robert Jenkins as a 76-year-old male at the 6 month follow up after colonoscopy. The interview and SOAP note document successful periprocedural apixaban interruption and resumption, stable HFrEF symptoms, and the Robert Jenkins medication regimen.

What the grader expected: The AI feedback explicitly states that the rubric expected a 64-year-old female with a different history of adherence, affordability, social barriers, medication doses, vital signs, and laboratory values.

Why this is a problem: The 44.2% score is not a valid assessment of the Robert Jenkins Thursday note because the scoring engine is using another patient case.

Suggested fix: Map Robert Jenkins plus Thursday plus the 6 month follow up state to the Robert Jenkins Thursday rubric and validate patient ID, day, and rubric ID before grading.

STUDENT FACING EVIDENCE

What the student received

Thursday live encounter header identifies Robert Jenkins, 76 year old male.

GRADER / RUBRIC EVIDENCE

- Thursday AI feedback states that it expected a 64 year old female case.

Thursday 6 Month Follow Up • Issue 1 • Evidence excerpt from preserved VACS audit record

Evidence – Thursday 6 Month Follow Up: student facing information compared with the grader/rubric expectation.

Issue 2. Patient age and date of birth remain internally inconsistent in the Thursday header

What the student saw or heard: The Thursday header displays Robert as 76 years old while showing DOB 03/12/1972.

What the grader expected: The correct Robert Thursday rubric identifies the patient as 76 years old.

Why this is a problem: The same chart contains conflicting demographic fields, and the discrepancy persists across the longitudinal encounters.

Suggested fix: Correct the longitudinal demographic record so age and date of birth agree on every visit.

STUDENT FACING EVIDENCE

What the student received

Thursday header displays age 76 and DOB 03/12/1972.

GRADER / RUBRIC EVIDENCE

- The correct Robert Thursday rubric identifies the patient as 76 years old.

Thursday 6 Month Follow Up • Issue 2 • Evidence excerpt from preserved VACS audit record

Evidence – Thursday 6 Month Follow Up: student facing information compared with the grader/rubric expectation.

Issue 3. Live Thursday vital signs do not match the correct Robert Jenkins Thursday rubric

What the student saw or heard: The live Thursday EMR shows BP 120/70 mmHg with repeat 118/68, HR 68 bpm, temperature 98.6°F, SpO2 97%, weight 82 kg, and BMI 28.3.

What the grader expected: The correct Robert Thursday rubric expects BP 112/66 mmHg, HR 72 bpm, temperature 98.0°F, SpO2 98% on room air, weight 81.5 kg, and BMI 27.6 kg/m².

Why this is a problem: Even after the wrong patient grader mapping is corrected, the live objective values still do not match the correct Robert Thursday reference data.

Suggested fix: Use one Robert Thursday encounter data object for the EMR and rubric and validate each vital sign field before release.

STUDENT FACING EVIDENCE

What the student received

Thursday live vitals display 120/70, HR 68, temperature 98.6°F, SpO2 97%, weight 82 kg, BMI 28.3.

GRADER / RUBRIC EVIDENCE

- correct Robert Thursday rubric contains different BP, HR, temperature, SpO2, weight, and BMI values.

Thursday 6 Month Follow Up • Issue 3 • Evidence excerpt from preserved VACS audit record

Evidence – Thursday 6 Month Follow Up: student facing information compared with the grader/rubric expectation.

Issue 4. Hemoglobin trend and platelet count required by the rubric are missing from the Thursday CBC

What the student saw or heard: The Thursday CBC panel displays WBC $6.3 \times 10^3/\text{mm}^3$ only. Current hemoglobin, previous hemoglobin, and platelet count are not shown.

What the grader expected: The correct Thursday rubric requires current hemoglobin 13.3 g/dL, previous hemoglobin 13.4 g/dL, platelets $229 \times 10^3/\text{mm}^3$, and interpretation of the stable hemoglobin trend.

Why this is a problem: The student cannot report or interpret a bleeding safety trend that is absent from the visible laboratory panel.

Suggested fix: Display the required hemoglobin and platelet data in the Thursday CBC, including the prior hemoglobin value, or mark those items unavailable and not gradable.

STUDENT FACING EVIDENCE

What the student received

Thursday CBC panel shows WBC only.

GRADER / RUBRIC EVIDENCE

- Thursday rubric requires current and prior hemoglobin plus platelets.

Thursday 6 Month Follow Up • Issue 4 • Evidence excerpt from preserved VACS audit record

Evidence – Thursday 6 Month Follow Up: student facing information compared with the grader/rubric expectation.

Issue 5. Gastroenterology report is marked missing even though the rubric grades report sourced objective findings

What the student saw or heard: The Thursday Objective screen labels the gastroenterology report as Missing. A brief missing data banner summarizes that colonoscopy occurred 10 days ago, two small tubular adenomas were removed, there were no complications or post procedure bleeding, and routine surveillance was recommended.

What the grader expected: The correct Thursday rubric grades the colonoscopy report as an objective source and requires the two tubular adenomas, no complications, no bleeding, routine gastroenterology surveillance, and documentation that gastroenterology is the source.

Why this is a problem: The system tells the student that the source report is missing while simultaneously requiring report level documentation. A summary banner is not the same as an accessible source report.

Suggested fix: Either provide the gastroenterology report as an accessible objective record or clearly designate the banner as the authoritative report extract and grade only the information exposed there.

STUDENT FACING EVIDENCE

What the student received

Thursday Objective section explicitly labels the gastroenterology report Missing.

GRADER / RUBRIC EVIDENCE

- Thursday rubric grades multiple objective findings from the gastroenterology report.

Thursday 6 Month Follow Up • Issue 5 • Evidence excerpt from preserved VACS audit record

Evidence – Thursday 6 Month Follow Up: student facing information compared with the grader/rubric expectation.

Issue 6. eGFR 40 mL/min/1.73m² is incorrectly labeled Normal

What the student saw or heard: The Thursday renal panel and snapshot display eGFR 40 mL/min/1.73m² with a normal badge while serum creatinine 1.76 mg/dL is flagged high.

What the grader expected: The correct Thursday rubric identifies eGFR 40 as CKD Stage 3b and uses it to support ongoing DOAC and GDMT management.

Why this is a problem: The status badge contradicts the intended renal classification and can mislead the student during medication safety assessment.

Suggested fix: Correct the eGFR status logic so the badge reflects the numeric result and CKD stage.

STUDENT FACING EVIDENCE

What the student received

Thursday eGFR 40 is displayed with a Normal badge.

GRADER / RUBRIC EVIDENCE

- Thursday rubric classifies eGFR 40 as CKD Stage 3b.

Thursday 6 Month Follow Up • Issue 6 • Evidence excerpt from preserved VACS audit record

Evidence – Thursday 6 Month Follow Up: student facing information compared with the grader/rubric expectation.

Issue 7. Thursday active Problem List contains only AFib despite multiple active chronic conditions

What the student saw or heard: The Thursday Problem List contains AFib only. The medication list itself identifies HFrEF, HFrEF / T2DM, and CAD indications, and the interview also references heart failure, kidney disease, diabetes, and high cholesterol.

What the grader expected: The correct Thursday rubric includes HFrEF, persistent nonvalvular AFib, CAD, CKD Stage 3b, type 2 diabetes, hypertension, and hyperlipidemia.

Why this is a problem: The active Problem List is not synchronized with the rest of the same patient record.

Suggested fix: Populate the Problem List from the longitudinal diagnosis source used by medications, interview logic, and the correct Robert rubric.

STUDENT FACING EVIDENCE

What the student received

Thursday Problem List displays AFib only. Thursday medication indications and interview identify additional chronic diseases.

GRADER / RUBRIC EVIDENCE

- The correct Thursday rubric includes HFrEF, persistent nonvalvular AFib, CAD, CKD Stage 3b, type 2 diabetes, hypertension, and hyperlipidemia.

Thursday 6 Month Follow Up • Issue 7 • Evidence excerpt from preserved VACS audit record

Evidence – Thursday 6 Month Follow Up: student facing information compared with the grader/rubric expectation.

Issue 8. COVID, pneumococcal, Shingrix, and Tdap statuses are graded but not available from the Thursday patient or visible immunization record

What the student saw or heard: The Thursday EMR confirms influenza up to date. When directly asked about COVID, pneumonia, shingles, and Tdap vaccines, Robert says he is honestly not sure and does not track them closely.

What the grader expected: The correct Thursday rubric requires COVID up to date, pneumococcal completed, Shingrix completed, and Tdap up to date.

Why this is a problem: The student cannot document exact completed vaccine statuses when the visible chart does not show them and the patient is uncertain.

Suggested fix: Expose all graded vaccine statuses in the Thursday immunization record or make them reliably available through the AI patient.

STUDENT FACING EVIDENCE

What the student received

Thursday EMR shows influenza up to date only. Thursday interview: Robert is unsure of COVID, pneumonia, shingles, and Tdap status.

GRADER / RUBRIC EVIDENCE

- The correct Thursday rubric requires COVID up to date, pneumococcal completed, Shingrix completed, and Tdap up to date.

Thursday 6 Month Follow Up • Issue 8 • Evidence excerpt from preserved VACS audit record

Evidence – Thursday 6 Month Follow Up: student facing information compared with the grader/rubric expectation.

Issue 9. Required longitudinal cardiac history and disease details remain incomplete in the Thursday AI patient

What the student saw or heard: When asked what conditions he has, Robert reports HFrEF, AFib, kidney issues, diabetes, and high cholesterol. He does not provide HFrEF duration, LVEF, persistent nonvalvular AFib qualifiers, CAD with prior NSTEMI, PCI with drug eluting stent, hypertension, or CKD Stage 3b details.

What the grader expected: The correct Thursday rubric requires HFrEF diagnosed 4 years ago, LVEF 30%, persistent nonvalvular AFib, CAD, NSTEMI 3 years ago, PCI with drug eluting stent, CKD Stage 3b, hypertension, and hyperlipidemia.

Why this is a problem: The rubric expects a detailed longitudinal history that is not reproducible from the AI patient response in this follow up encounter.

Suggested fix: Persist the complete longitudinal history and return the required details when the student asks for the medical history or expose them in a visible history section.

STUDENT FACING EVIDENCE

What the student received

Thursday interview: Robert provides only a partial condition list.

GRADER / RUBRIC EVIDENCE

- Thursday rubric requires detailed HFrEF, CAD, PCI, CKD, and hypertension history.

Thursday 6 Month Follow Up • Issue 9 • Evidence excerpt from preserved VACS audit record

Evidence – Thursday 6 Month Follow Up: student facing information compared with the grader/rubric expectation.

Issue 10. Expected interest in kidney function monitoring conflicts with the Thursday AI patient response

What the student saw or heard: When asked whether he has ongoing concerns about kidney function or how closely it should be monitored, Robert says nobody has talked to him much about it and he does not have any specific concerns at the moment.

What the grader expected: The correct Thursday rubric requires interest in ongoing kidney function monitoring as a subjective patient centered element.

Why this is a problem: The student directly explores the criterion and receives the opposite response. Documenting interest in kidney monitoring would contradict the AI patient.

Suggested fix: Synchronize the Thursday patient goals with the rubric. If ongoing kidney monitoring interest is required, the AI patient should express it when asked directly.

STUDENT FACING EVIDENCE

What the student received

Thursday interview: Robert reports no specific kidney monitoring concern.

GRADER / RUBRIC EVIDENCE

- The correct Thursday rubric requires interest in ongoing kidney function monitoring as a subjective patient centered element.

Thursday 6 Month Follow Up • Issue 10 • Evidence excerpt from preserved VACS audit record

Evidence – Thursday 6 Month Follow Up: student facing information compared with the grader/rubric expectation.